Overview

HEDIS MY 2023

The Healthcare Effectiveness Data and Information Set (HEDIS) is one of the most widely used sets of health care performance measures in the United States. The term "HEDIS" originated in the late 1980s as the product of a group of forward-thinking employers and quality experts, and was entrusted to NCQA in the early 1990s. NCQA has expanded the size and scope of HEDIS to include measures for Accountable Care Organizations and other organizations. HEDIS is published across several volumes. HEDIS MY 2023 includes 95 measures across 6 domains:

- Effectiveness of Care.
- Access/Availability of Care.
- Experience of Care.
- Utilization and Risk Adjusted Utilization.
- Health Plan Descriptive Information.
- Measures Reported Using Electronic Clinical Data Systems.

***Volume 1:* Narrative**	A general overview of the HEDIS measurement set and how the data are used.
***Volume 2:* Technical Specifications for Health Plans**	The technical specifications for the HEDIS measures for organizations, including instructions on data collection for each measure and general guidelines for calculations and sampling.
Technical Specifications for ACO Measurement	The technical specifications for the HEDIS quality measures for accountable care organizations.
***Volume 3:* Specifications for Survey Measures**	The technical specifications for HEDIS survey measures and standardized surveys from the Consumer Assessment of Healthcare Providers and Systems (CAHPS) program.
***Volume 5:* HEDIS Compliance Audit: Standards, Policies and Procedures**	The accepted method for auditing the HEDIS production process, including an information systems capabilities assessment and an evaluation of compliance with HEDIS specifications. Standards that Certified HEDIS Compliance Auditors must use when conducting a HEDIS audit.
***Volume 6:* Specifications for the Medicare Health Outcomes Survey**	The technical specifications for the Health Outcomes Survey (HOS).
HEDIS Technical Specifications for Long-Term Services and Support Measures	The technical specifications for organizations providing Medicaid long-term services and supports (LTSS).

How HEDIS Is Developed

NCQA's Committee on Performance Measurement (CPM), which includes representation from purchasers, consumers, health plans, clinicians and policy makers, oversees the evolution of the measurement set. Multiple Measurement Advisory Panels (MAP) provide clinical and technical knowledge required to develop the measures. Additional HEDIS Expert Panels and the Technical Measurement Advisory Panel (TMAP) provide invaluable assistance by identifying methodological issues and providing feedback on new and existing measures.

What's New in Volume 2?

New measures

- Topical Fluoride for Children (TFC).
- Oral Evaluation, Dental Services (OED).
- Deprescribing of Benzodiazepines in Older Adults (DBO).
- Emergency Department Visits for Hypoglycemia in Older Adults with Diabetes (EDH).
- Cervical Cancer Screening (CCS-E).
- Social Need Screening and Intervention (SNS-E).

Retired measures

- Breast Cancer Screening (BCS)*.
- Annual Dental Visit (ADV).
- Frequency of Selected Procedures (FSP).
- Flu Vaccinations for Adults Ages 18–64 (FVA).
- Flu Vaccinations for Adults Ages 65 and Older (FVO).
- Pneumococcal Vaccination Status for Older Adults (PNU).

Only the BCS-E measure will be reported.

Revised measures

For specific revisions, refer to each measure's *Summary of Changes* or to Appendix 1 for a complete summary.

Overall changes

- Moved all optional exclusions to required exclusions.
- Removed the definition of "primary care physician" from *Appendix 3: Practitioner Types.*

Technical specification updates—HEDIS MY 2023

NCQA will freeze the specifications for MY 2023 on March 31, 2023, with the *HEDIS MY 2023 Volume 2 Technical Update:*

- *HEDIS MY 2023 Volume 2 Technical Update memo (3/31/2023 release)* will be posted to the NCQA website (https://my.ncqa.org).
- *HEDIS MY 2023 Volume 2 Value Set Directory (3/31/2023 Release)* will be posted in the My Downloads section of My NCQA (https://my.ncqa.org/Downloads).

Organizations are accountable for all changes included in the *Technical Update*.

Medication List Directory—HEDIS MY 2023

The Medication List Directory (MLD) is available for free order in the NCQA Store (https://store.ncqa.org/hedis-my-2023-medication-list-directory.html).

Once ordered, the Medication List Directory for HEDIS MY 2023 will be available in the My Downloads section of My NCQA (https://my.ncqa.org/Downloads) by March 31, 2023.

Additional medication list changes, if needed, will be communicated in the *HEDIS MY 2023 Volume 2 Technical Update memo (3/31/2023 release).*

HCC Risk Adjustment tables—HEDIS MY 2023	The HCC Risk Adjustment tables are available for free order in the NCQA Store (https://store.ncqa.org/hedis-my-2023-risk-adjustment-tables.html). Once ordered, the HCC Risk Adjustment tables for HEDIS MY 2023 will be made available in the My Downloads section of My NCQA (https://my.ncqa.org/Downloads) by March 31, 2023.
HEDIS MY 2023 first-year measure evaluation	At the conclusion of the HEDIS MY 2023 data collection period, NCQA will evaluate first-year status measures/indicators to determine if they will be publicly reported. Any changes to the HEDIS MY 2023 first-year status measures/indicators will be communicated in a public reporting memo released on October 2, 2024.

Referring to HEDIS Measures and Rates

HEDIS measures and resulting rates must always retain the HEDIS name. Specifically, for unadjusted measures:

- Refer to all *unadjusted* HEDIS measures as **"HEDIS Health Plan measures."**
- Calculated measure rates that are based on *unadjusted* HEDIS specifications and *have not* been certified through NCQA's Measure Certification Program may not be called "Health Plan HEDIS rates" until they are audited and designated reportable by an NCQA-Certified HEDIS Compliance Auditor. Refer to these rates as **"Uncertified, Unaudited Health Plan HEDIS Rates."** Such uncertified rates *may only be used for internal, quality improvement purposes* (e.g., trend analysis) and no incentive payments may be made on such rates.
- Calculated measure rates that are based on *unadjusted* HEDIS specifications and have been certified through NCQA's Measure Certification Program may not be called "Health Plan HEDIS rates" until they are audited and designated reportable by an NCQA-Certified Auditor. Refer to these rates as **"Unaudited Health Plan HEDIS rates."**

Organizations that need assistance in determining the correct naming convention for HEDIS measures/ rates should contact NCQA through My NCQA at https://my.ncqa.org.

Specifically, for *adjusted* measures pursuant to NCQA's Rules for Allowable Adjustment of HEDIS:

- Refer to all *adjusted* HEDIS measures as **"Adjusted HEDIS measures."**
- Calculated measure rates that are based on *adjusted* HEDIS specifications and *have not* been certified through NCQA's Measure Certification Program may not be called "adjusted HEDIS rates" until they are audited and designated reportable by an NCQA-Certified HEDIS Compliance Auditor. Refer to these rates as **"Adjusted, Uncertified, Unaudited HEDIS rates."** Such **uncertified** rates *may only be used for internal, quality improvement purposes* (e.g., trend analysis) and no incentive payments may be made on such rates.
- Calculated measure rates that are based on *adjusted* HEDIS specifications and *have* been certified through NCQA's Measure Certification Program may not be called "adjusted HEDIS rates" until they are audited and designated reportable by an NCQA-Certified Auditor. Refer to these rates as **"Adjusted, Unaudited HEDIS rates."**

If You Have Questions About the Specifications

Policy Clarification Support

NCQA provides different types of policy support to customers, including a function that allows customers to submit specific policy interpretation questions to NCQA staff through My NCQA at https://my.ncqa.org.

FAQs and Policy Updates

The FAQs and Policy Updates clarify HEDIS uses and specifications, and are posted to the NCQA website (https://www.ncqa.org/) on the 15th of each month.

Additional Resources

In addition to the specification volumes, NCQA provides a variety of resources to help organizations understand measure specifications, collect data and report audited HEDIS results:

- Each organization implementing HEDIS is strongly encouraged to join NCQA's HEDIS Users Group (HUG) for technical assistance and guidance on interpreting the specifications. Membership benefits include NCQA HEDIS and Accreditation publications, newsletters, online seminars and discount vouchers for HEDIS conferences and publications. For more information, email hug@ncqa.org.
- HEDIS publications are made available only in electronic publication ("e-pub") format and are sold by user license tiers in the NCQA Store at https://store.ncqa.org/. E-pubs are protected Microsoft Word and Excel files displaying the publication's content and can be downloaded easily.
- NCQA produces HEDIS digital measure specifications that are expressed using FHIR and the CQL standard for representing a clinical quality measure as an electronic document. HEDIS digital measures are published in a format that allows them to be directly consumed by the reporting system. Each digital measure includes both the human-readable documentation and the machine-readable files necessary for implementation. The HEDIS digital measures are sold in the NCQA Store at https://store.ncqa.org/.
- NCQA produces many publications that are relevant to organizations and physicians interested in improving the quality of health care. To obtain a list or to order publications, go to the NCQA Store at https://store.ncqa.org/ or call Customer Support at 888-275-7585.
- An important part of quality improvement is keeping up with the latest in health care and learning best practices. NCQA provides up to date education and training opportunities to help you reach your quality improvement goals. Different learning formats are available to suit your needs. Visit https://www.ncqa.org/education-training/ or call NCQA Customer Support at 888-275-7585.

Reporting Hotline for Fraud and Misconduct

NCQA does not tolerate submission of fraudulent, misleading or improper information by organizations as part of their survey process or for any NCQA program. NCQA reserves the right to publicly display the information on the reported issue, including, but not limited to, the affected organization, the specific data error and the associated resolution. We will display the information on a dedicated public-facing webpage on the NCQA website (https://www.ncqa.org/). NCQA may also revise affected products.

NCQA has created a confidential and anonymous Reporting Hotline to provide a secure method for reporting perceived fraud or misconduct, including submission of falsified documents or fraudulent information to NCQA that could affect NCQA-related operations (including, but not limited to, the survey process, the HEDIS measures and determination of NCQA status and level).

How to Report

- **Toll-Free Telephone:**
  - English-speaking USA and Canada: 844-440-0077 (not available from Mexico).
  - Spanish-speaking North America: 800-216-1288 (from Mexico, user must dial 001-800-216-1288).
- **Website:** https://www.lighthouse-services.com/ncqa.
- **Email:** reports@lighthouse-services.com (must include NCQA's name with the report).
- **Fax:** 215-689-3885 (must include NCQA's name with the report).

Reporting Data Errors to NCQA

Because audited HEDIS data are used to establish plans' Accreditation status and in many NCQA programs and products, NCQA must be made aware of data problems in any previously reported rate.

Organizations must immediately report any error in a measure rate or in its component (in any previous submission, regardless of timing) that is >5% higher or lower than what was reported originally. These should be reported to NCQA through PCS via My NCQA (https://my.ncqa.org) by selecting Product/ Program Type as "HEDIS Audit" and General Content Area as "Data Errors." The report to NCQA must include:

- A description of the issue that includes:
  - The correct rate.
  - The error's cause.
  - How the error was discovered.
  - How the error was corrected.
- The HEDIS measure year and the measures affected.
- The submissions affected.
- The impact on reported rates.

Auditors must document all findings for the year in question and the current year's corrections. Findings must be included in the work papers and must be noted in detail in the organization's Final Audit Report.

General Guidelines for Data Collection and Reporting

SUMMARY OF CHANGES TO HEDIS MY 2023

- Removed *General Guideline 5: Model Type/Mixed Model Type* and renumbered subsequent guidelines.
- Removed *General Guideline 14: Employer Specific Reports* and renumbered subsequent guidelines.
- Updated *General Guideline 16: Deceased Members* (formerly General Guideline 18).
- Updated *General Guideline 31: Race and Ethnicity Stratification* (formerly General Guideline 33).
- Added *General Guideline 32: Medicare Socioeconomic Status Stratification* (formerly *Effectiveness of Care Guideline*).
- Revised *General Guideline 44: Visits that Result in an Inpatient Stay* to clarify that visits resulting in an inpatient stay are now identified based on visit date of service and inpatient stay dates of service (formerly General Guideline 45).
- Removed references to optional exclusions.

HEDIS Reporting

1. Product-Line Reporting

Audited HEDIS results are collected and reported separately for populations covered by commercial insurance, Medicaid and Medicare.

Note: *A subset of HEDIS measures is collected and reported for the Exchange product line. For reporting requirements and measure specifications for Exchange reporting, refer to the Quality Rating System (QRS) Measure Technical Specifications on the CMS website. These reporting requirements must be used for only QRS reporting; Volume 2 may not be used for QRS-specific reporting.*

2. Product-Specific Reporting

Audited HEDIS results may be reported separately by product (HMO, POS, PPO, EPO) or combined (HMO/POS or PPO/EPO). To report a PPO or EPO product combined with an HMO or POS product, submit a written request for approval to PCS via My NCQA (https://my.ncqa.org). The request must address all the elements contained in *General Guideline 3: HEDIS Submission for Organizations Seeking Accreditation*.

The organization must submit data for all members for an entire product. Refer to *General Guideline 14: Reporting ASO and Self-Insured Members* for more information on allowable population exclusions.

Definitions

HMO Health maintenance organization. An organized health care system that is accountable for both the financing and delivery of a broad range of comprehensive health services to an enrolled population. An HMO is accountable for assessing access and ensuring quality and appropriate care. Practitioners affiliated with the health care system render health care services. In this type of organization, members must obtain all services from affiliated practitioners and must usually comply with a predefined authorization system to receive reimbursement.

A **practitioner** is a professional who provides health care services and is usually required to be licensed as defined by law.

POS Point of service. An HMO with an opt-out option. In this type of organization, members may choose to receive services either within the organization's health care system (e.g., an in-network practitioner) or outside the organization's health care delivery system (e.g., an out-of-network practitioner).

The level of benefits or reimbursement is generally determined by whether the member uses in-network or out-of-network services. Common uses of "POS" include references to products that enroll each member in both an HMO (or HMO-like) system and in an indemnity product. A POS product is also referred to as an "HMO swing-out organization," an "out-of-organization benefits rider to an HMO" or an "open-ended HMO."

PPO Preferred provider organization. PPOs are responsible for providing health benefits-related services to covered individuals and for managing a practitioner network. They may administer health benefits programs for employers by assuming insurance risk or by providing only administrative services.

EPO Exclusive provider organization. A type of health insurance product that usually limits coverage to care from providers, or groups of providers, who have contracts with the health insurance issuer to be part of a network of participating providers. EPO members will generally not be reimbursed or receive benefits for out-of-network services; however, some EPOs will provide partial reimbursement for emergency situations.

3. HEDIS Submission for Organizations Seeking Accreditation

Audited HEDIS results must correspond with the product line/product combination for which the organization seeks Accreditation. NCQA defines the organization for Accreditation and HEDIS reporting as part of the Accreditation application process. Members residing in a state where an organization is not licensed to operate are included in the organization's "home" state, where it is licensed to operate and where its main membership resides.

How NCQA Defines an Organization for Accreditation

NCQA's definition of the Accreditable organization (the "Accreditable entity") is based on the legal entity and management structure and delivery system that support the product lines/products NCQA Accredits. NCQA's goal is to arrive at Accreditation decisions that reflect the organization that is legally accountable for services provided to its members and represents an organizational and delivery structure that is meaningful to members.

1. Legal entity NCQA's goal is to identify the legal entity that issues a contract for insurance for a defined population or contracts with an employer to provide managed care services to a self-insured population.

If the legal entity operates in multiple states but otherwise operates as a centralized organization (same oversight and management structure; same staff, same policies and procedures, all functions addressed in standards performed under a corporate or central structure), NCQA conducts one survey for the legal entity; however, the legal entity is required to report HEDIS/ CAHPS by geographic unit, as defined below. For multiple legal entities, NCQA may consider each legal entity as an Accreditable entity. For a legal entity without centralized operations, NCQA may determine that distinct operating units or service areas are the Accreditable entity.

If the organization consists of multiple legal entities within a state, but otherwise operates as a single statewide organization (same management structure; a single practitioner/provider network for the entire state; centralized key functions, including quality improvement, credentialing and utilization management), NCQA awards Accreditation status to each legal entity. With NCQA approval, the organization may submit one statewide HEDIS submission that is applied to each legal entity.

NCQA reserves the right to determine, at its sole discretion, an entity or organization's eligibility for Accreditation, and must approve an organization to be surveyed as one legal entity or multiple legal entities.

2. Practitioner and provider network

The organization must have a single practitioner or provider network. If there are separate and distinct practitioner or provider networks, NCQA may consider each network (along with the accompanying management structure) to be a different Accreditable entity.

NCQA recognizes that organizations sometimes market individual products with practitioner or provider networks that are subsets of a larger network. In this case, NCQA may define the Accreditable entity at the level of the broader network.

3. Centralization

NCQA considers the degree of centralization of key functions assessed by the Accreditation standards. The organization has a single QI program and a single set of policies and procedures for the functions evaluated by the standards, including population health management utilization management credentialing managing member complaints and appeals and developing member materials. If key functions are decentralized, with distinct policies and procedures, NCQA may determine that there is more than one Accreditable entity.

4. Licensure

The organization may have multiple licenses, especially if its service area crosses state lines.

5. HEDIS/CAHPS reporting unit

In general, because evaluation of HEDIS/CAHPS results is required for Accreditation and NCQA issues a unique status for each HEDIS/CAHPS reporting unit, the Accreditable entity is the same as the HEDIS/CAHPS reporting unit.

6. Product/ product line

Product lines reflect the different populations an organization serves. HEDIS/CAHPS results must be collected and reported separately for populations covered by the following product lines:

- Commercial.
- Exchange.
- Medicaid.
- Medicare.

Off-Exchange products must be included in the commercial product line, not in the Exchange product line.

An organization that is responsible for both the Medicare and Medicaid components for dual-eligible members (including members in a Medicare-Medicaid Plan [MMP] benefit package) can select Medicare or Medicaid (or

both) for Accreditation purposes. Dual-eligible members must be included in the product lines selected. An organization that manages Medicaid fee-for-service members may exclude those members from its Medicaid product line.

Products reflect the organizational structure (e.g., network), services and benefits offered by the organization and include HMO, POS, PPO and EPO products.

Because HEDIS reporting must match the product line for which an organization seeks Accreditation, an organization with a Children's Health Insurance Programs (CHIP) population must include those members in its Medicaid product line, using Medicaid measure specifications, even if it needs separate HEDIS submissions for other purposes such as state reporting.

If the state requires the organization to report CHIP members separately, a second, state-specific CHIP-only HEDIS/CAHPS submission is required. The organization must note this state-specific requirement in the Health Organization Questionnaire (HOQ).

The organization must exclude CHIP products from its commercial product line to avoid affecting organization-to-organization comparison.

7. Geographic unit

HEDIS performance varies geographically throughout the United States. To be meaningful to consumers and purchasers, results must reflect geographic variation.

For organizations with HMO, POS and EPO products—which are generally incorporated locally and regulated individually by states—the size of the geographic unit is determined by the legal entity.

For PPO products—which may have a service area larger than a single state—the organization is required to report HEDIS/CAHPS results for geographic regions no larger than a state.

8. Limiting states included in NCQA Accreditation

If an organization licensed to operate in multiple states seeks Accreditation for a limited geographic area, it may not represent that an excluded state is covered by the Accreditation. For example, an organization may have most of its membership in New York and the rest in a national account in Texas. If the organization is Accredited only for New York, it may not state that Texas is also Accredited.

An organization with membership in a state where it is not licensed to operate includes those members in the "home" state where it is licensed to operate and has its main membership.

Health Plan Ratings and Accreditation

Evaluation of HEDIS/CAHPS performance is separate from standards scoring. In addition to Accreditation status, Health Plan Ratings will be displayed on the NCQA Report Card as the indicator of HEDIS/CAHPS performance.

Based on the updated methodology, organizations earn a star rating of 0–5 stars (in half-star increments) for the HEDIS/CAHPS portion of Accreditation. The methodology includes a distinct set of measures for each product line. Each measure is classified in one of three categories:

1. Process measures, which have a weight of 1.
2. Outcome measures, which have a weight of 3.
3. Patient Experience measures, which have a weight of 1.5.

The overall rating is the weighted average of an organization's HEDIS and CAHPS measure ratings, plus Accreditation bonus points (if the organization has NCQA Accreditation), rounded to the nearest half point.

Overall performance is measured in three subcategories (scored 0–5 in half point increments and displayed as stars):

- **Patient Experience.** Patient-reported experience of care, including experience with doctors, services and customer service (measures in the Patient Experience category).
- **Rates for Clinical Measures.** The proportion of eligible members who received preventive services (prevention measures) and the proportion of eligible members who received recommended care for certain conditions (treatment measures).
- **NCQA Accreditation Standards Score.** For an organization with an Accredited or Provisional status, 0.5 bonus points (displayed as stars) are added to the overall rating before rounding to the nearest half point (displayed as stars). An organization with an Interim status receives 0.15 bonus points before rounding to the nearest half point (displayed as stars).

Note: *If an Accredited organization chooses to publicly report performance data on the HEDIS Attestation, it is scored on the data submitted and receives the Accreditation bonus points (displayed as stars). If an organization Accredited on standards only chooses not to publicly report performance data, it is not scored based on performance measurement results and is not awarded Accreditation bonus points.*

If an organization's membership for a reporting unit is less than 15,000, the organization may be scored on standards only, may combine its membership with another HEDIS/CAHPS reporting unit to achieve the minimum reporting threshold or may choose to be scored on HEDIS/CAHPS without meeting the minimum reporting threshold.

Refer to https://www.ncqa.org/hedis/health-plan-ratings/ for the list of required measures and detailed HPR methodology, which includes policies for combining entities.

Annual Reevaluation

For organizations required to submit HEDIS/CAHPS results, NCQA recalculates Ratings annually by product/product line, on the set of measures identified for each reporting year. Organizations must submit audited HEDIS results to NCQA each year by the reporting date specified in *General Guideline 26: HEDIS Reporting Date.* Refer to the *Reports* section at https://www.ncqa.org/hedis/reports-and-research/ for the list of required measures.

4. HEDIS Submission for Organizations Not Seeking Accreditation

To determine how many HEDIS reports to produce, an organization defines itself using the criteria described above. If an organization cannot determine the HEDIS reporting entity, it must submit written documentation regarding the criteria described in *General Guideline 3: HEDIS Submission for Organizations Seeking Accreditation,* to the NCQA Policy Department via PCS at My NCQA (https://my.ncqa.org). NCQA staff review the organization's structure and make a determination.

5. Reporting HEDIS for Medicaid

Separate Medicaid HEDIS reports must be produced for each state with which an organization has a Medicaid contract.

If an organization contracts with a local entity (i.e., with a county, rather than with a state) and with each locality where it provides service, the state and the organization may consider providing a comprehensive Medicaid HEDIS report that encompasses all geographic areas in the state that are served by the organization.

6. Reporting HEDIS for Medicare

CMS annually issues the reporting requirements for HEDIS, HOS and CAHPS to all MA organizations in a CMS memorandum addressed to all entities that must fulfill these reporting requirements. The following organizations and plan benefit packages listed below are required to report HEDIS measures for their members:

- Medicare Advantage (MA) organizations:
  - Coordinated Care Plan (CCP) (local, regional, employer and religious fraternal benefit) organizations.
  - Special Needs Plan (SNP) benefit packages, including all SNP types (Dual Eligible, Chronic or Disabling Conditions and Institutional).
  - Private Fee-for-Service (PFFS) (including local, employer and religious fraternal benefit) organizations.
  - Medical Savings Account (MSA) contracts.
  - Medicare-Medicaid Plan (MMP) benefit packages.
- Section 1876 cost organizations.

MA organizations that offer SNP and MMP plan benefit packages submit an additional subset of HEDIS measures that are listed in the CMS Memo. The CMS Memo also lists the required HEDIS measures by organizational type because some MA organizations report fewer HEDIS measures. CMS strongly advises that all MA organizations refer to the CMS Memo to determine which HEDIS measures are required for their organization.

Note: *Members in hospice are excluded from HEDIS reporting. Refer to General Guideline 15: Members in Hospice.*

7. Reporting HEDIS for CHIP

CHIP	States may contract with an organization to provide care to CHIP members as part of the organization's Medicaid product line or a CHIP-only product. The state enables the contracting organization to identify CHIP members, when possible.
Reporting guidelines	For NCQA HEDIS/CAHPS reporting purposes, CHIP products must be included in the organization's Medicaid product line using Medicaid measure specifications. This applies for organizations that have other Medicaid products as well as organizations that have only CHIP products. If the state requires contracting organizations to report CHIP products separately, a second, state-specific CHIP-only HEDIS/CAHPS submission is required. Organizations must note this state-specific requirement in the Health Organization Questionnaire (HOQ).

The organization must exclude CHIP products from its commercial product-line reports because including CHIP products in HEDIS reports for commercially enrolled populations may affect organization-to-organization comparison.

Natural Disaster and Cybercrime

This section of the specifications applies only to organizations whose operations were destroyed or severely disrupted due to natural disaster or cybercrime (e.g., impact on access to claims/enrollment files, encrypted or changed data). These organizations must report this to NCQA and should contact their auditors to discuss the current state of their operations and the availability of documentation needed to meet various NCQA program requirements. Specific relief will be determined on a case-by-case basis. Organizations should provide a measure specific assessment of the following areas, as applicable:

- HEDIS and CAHPS reporting.
  - Seek guidance from CMS about CAHPS for Medicare Advantage and Part D organizations.
- Access and availability of practitioners or practitioner records or data systems.

Based on the information provided, NCQA will work with the organization to determine the appropriate level of modification for the HEDIS reporting year.

The NCQA HEDIS Compliance Audit™

The HEDIS Compliance Audit runs concurrent with the data collection process. The audit allows comparability across organizations and ensures validity and integrity of reported HEDIS data.

The audit is required for organizations seeking NCQA Accreditation or for reporting in NCQA public reporting products, including Quality Compass, and by CMS and many states and employer groups.

8. Audit Preparation

Contract with an audit firm

The organization requests an application for a HEDIS Audit from an NCQA Licensed Organization (https://www.ncqa.org/programs/data-and-information-technology/hit-and-data-certification/hedis-compliance-audit-certification/) and is responsible for determining fees and entering into contracts. The first activity in audit preparation is contract execution. An organization contacts NCQA Licensed Organizations (LO) for bids and selects a firm to conduct the HEDIS audit.

The contracting phase includes assessing measures to report, executing the contract with all the necessary ancillary agreements (e.g., confidentiality and conflict of interest) and negotiating a timeline.

All LOs employ or contract with Certified HEDIS Compliance Auditors (CHCA) and select an audit team for the organization.

HEDIS Roadmap

Each organization must complete the HEDIS Record of Administration, Data Management and Processes (Roadmap). The Roadmap contains detailed questions about all audit standards and describes the operational and organizational structure of the organization. Auditors use the HEDIS Roadmap to review information about an organization's systems for collecting and processing data used to produce HEDIS reports and to organize the site visit.

Medical record review validation The medical record review validation (MRRV) process uses like-measure groupings for measure validation; includes hybrid measure exclusions; applies a different statistical test to the process; and defines MRR milestones clearly to ensure consistency across organizations. Refer to *Volume 5, HEDIS Compliance Audit: Standards, Policies and Procedures.*

HEDIS Audit Timeline Organizations must follow the HEDIS Audit Timeline, which will be posted on the NCQA website (www.ncqa.org/hedis/measures) on March 31, 2023, and is published in *Volume 5, HEDIS Compliance Audit: Standards, Policies and Procedures.*

9. Reporting

Audit results HEDIS Compliance Audits result in audited rates or calculations at the measure and indicator level, and indicate if the measures can be publicly reported. All measures selected for public reporting must have a final, audited result. The auditor approves the rate or report status of each measure and survey included in the audit, as shown below.

For Performance Measures

Rate/Result	Comment
R	*Reportable.* A reportable rate was submitted for the measure.
NA	*Small Denominator*. The organization followed the specifications, but the denominator was too small (e.g., less than 30) to report a valid rate. a. For Effectiveness of Care (EOC) and EOC-like measures, when the denominator is less than 30. b. For Utilization measures that count member months, when the denominator is less than 360 member months. c. For all Risk Adjusted Utilization measures, when the denominator is less than150. d. For measures reported using electronic clinical data systems (ECDS), when the denominator is less than 30. *NA (Not Applicable)* is a status, not an audit designation. Measure rates that result in NA are considered Reportable (R), but the denominator is too small to report.
NB	*No Benefit*. The organization did not offer the health benefit required by the measure (e.g., mental health, chemical dependency). Benefits are assessed at the global level, not the service level (refer to *General Guideline 24: Required Benefits*).
NR	*Not Reported*. The organization chose not to report the measure.
NQ	*Not Required.* The organization was not required to report the measure. NQ is not an option for required Medicare, Exchange or Accreditation measures.
BR	*Biased Rate.* The calculated rate was materially biased.
UN	*Unaudited.* The organization chose to report a measure that is not required to be audited. This result only applies when permitted by NCQA.

For Survey Measures

Rate/Result	Comment
SR	*Supports Reporting.* The survey sample frame was reviewed and approved.
NR	*Not Reportable.* Indicates the survey sample frame was incomplete or materially biased.

Material bias	Bias differs by measure and domain and is determined by the degree of data completeness for the data collection method used. Organizations may not report a rate for a measure that the auditor determines is biased. Auditors use a standardized set of bias assessments found in the Bias Determination appendix in *Volume 5, HEDIS Compliance Audit: Standards, Policies and Procedures*.

10. Marketing

Release of HEDIS Audit results must be in accordance with NCQA's *Guidelines for Advertising and Marketing,* posted on the NCQA website at https://my.ncqa.org. Organizations may release the entire Final Audit Report without prior authorization from NCQA, but must obtain written authorization from NCQA before releasing abridged, summarized or quoted information from the Final Audit Report.

Organizations that refer to the HEDIS Audit or any HEDIS data audited by a Certified HEDIS Compliance Auditor must adhere to the Guidelines.

In Which Reports Do HEDIS Members Remain?

11. Eligible Population

The **eligible population** for any measure is all members who satisfy all specified criteria, including age, continuous enrollment, benefit, event and the anchor date enrollment requirement. Organizations must include all members (regardless of benefit type) in the appropriate HEDIS report, except for self-insured members who meet the criteria in *General Guideline 14: Reporting ASO and Self-Insured Members*.

- *For the Administrative Method,* calculate the rate using the eligible population after exclusions are removed.
- *For the Hybrid Method,* calculate the rate using the denominator (the systematic sample drawn from the eligible population) after exclusions are removed.
- *For the ECDS method,* calculate the rate using the denominator (initial population minus exclusions).

Note: *Refer to the measurement specifications for eligible population criteria.*

12. Commercial Members

Include members enrolled through an employer group policy or through an individual or family policy in the commercial HEDIS report. Per *General Guideline 7: Reporting HEDIS for CHIP,* organizations must exclude CHIP members from their commercial product-line reports because including CHIP members in HEDIS reports for commercially enrolled populations may affect organization-to-organization comparison.

13. Members With Dual Enrollment

Dual enrollment is assessed using the continuous enrollment criteria in each measure. To meet criteria for dual enrollment, members must have dual enrollment at the end of the continuous enrollment period. Per *General Guideline 19: Members Who Switch Products/Product Lines*, if a measure allows a gap at the end of the continuous enrollment period, members must have dual enrollment as of the last enrollment segment. For measures without a continuous enrollment requirement, members must have dual enrollment on the date of service or the date of discharge.

Medicare members who have only Part D coverage do not qualify for dual enrollment.

This guideline must be applied to the following enrollment situations; all decisions based on the guideline must be used consistently across all measures.

Dual commercial/ Medicaid enrollment	Members with dual commercial and Medicaid enrollment must be reported in the commercial HEDIS reports. These members may be excluded from the Medicaid HEDIS reports.
Dual Medicaid enrollment	Members dually enrolled in two Medicaid plans must be reported in the primary Medicaid HEDIS report. The secondary Medicaid payer may exclude a member whose primary Medicaid enrollment was offered through a different organization.
Dual commercial enrollment	**Enrollment in different organizations:** Do not account for coordination of benefits with other insurance carriers. If members have commercial enrollment in two organizations, both organizations must include the members in their HEDIS reports, regardless of the primary insurer. For example, dependent children who are enrolled in one organization's commercial product line under the mother's insurance and in another organization's commercial product line under the father's insurance, are included in both HEDIS reports. **Enrollment in the same organization:** Adhere to the following criteria for members with dual enrollment (e.g., children enrolled under each parent): • *If members are enrolled twice in the same product and there is only one HEDIS submission,* include them only once in the submission. • *If products are reported separately,* include members with dual enrollment in both products in both HEDIS submissions. • *If the different product types are reported combined,* include members once in the combined submission.
Dual commercial/ Medicare enrollment	Members with dual commercial and Medicare enrollment must be included only in the product line that provides their primary enrollment (commercial or Medicare).
Dual Medicaid/ Medicare enrollment "dual eligible"	Include these members in *both* the Medicaid and Medicare HEDIS reports *only* if they are enrolled in the organization's Medicare contract required to report HEDIS *and* in the organization's Medicaid managed-care contract. An organization with a Dual-Eligible SNP benefit package must also include these members in its SNP submission. For Plan All-Cause Readmission, remove these members from Medicaid reporting and only include them in Medicare reporting. Members who have Medicare Private Fee-for-Service (PFFS) through another organization or have unknown Medicare coverage as their primary insurer may be excluded from the Medicaid report.
Medicare-Medicaid (MMP)	These members must be included in both the Medicaid and Medicare HEDIS reports. An organization with a MMP benefit package must also include these members in its MMP submission. For Plan All-Cause Readmission, remove these members from Medicaid reporting and only include them in Medicare reporting.

14. Reporting ASO and Self-Insured Members

Administrative services only (ASO) and self-insured members

Include ASO and self-insured members in HEDIS/CAHPS reports. Organizations may use different terms for these members, including, but not limited to "no-touch," "self-funded" or "third party administrator (TPA)." Organizations may exclude these members from HEDIS/CAHPS reports only in either of the following situations and only with **auditor approval:**

- The contract prohibits the organization from contacting members for any reason.
  - The **contractual agreement**, which is a contract or other written agreement between the organization (HMO, PPO, EPO) and the ASO client (e.g., employer), states that the organization may not contact these ASO and self-insured members under any circumstances or include them in HEDIS/CAHPS reporting. ***Note:*** *Exclusion from disease management, case management or quality improvement projects does not meet criteria for exclusion from HEDIS/CAHPS.*
- The agreement to exclude members in the reporting year must be in place (fully executed by both parties, in the case of a contract, or communicated, in the case of a written agreement) by January 1 of the measurement year.
- The organization is not responsible for administering both in-network and out-of-network claims for ASO and self-insured members (employer carve-out for both in-network and out-of-network claims). ***Note:*** *If claims are administered through a third party on behalf of an organization (i.e., a claims delegation arrangement), the organization is considered responsible for administering claims and members may not be excluded.*

An organization *may not* exclude members who cannot be reached (e.g., overseas military or Foreign Service members), unless one of the above situations applies. Non-ASO members may not be excluded under this guideline. Federal government instructions and guidance supersede the requirements in this guideline.

Exclusion of these members must be reviewed with the auditor, and contracts should be provided to the auditor to review and approve the exclusion.

If the organization excludes these ASO and self-insured members, it must also exclude them from HEDIS/CAHPS and from Accreditation.

15. Members in Hospice

Exclude members who use hospice services or elect to use a hospice benefit any time during the measurement year, regardless of when the services began. These members may be identified using various methods, which may include but are not limited to enrollment data, medical record, claims/ encounter data (Hospice Encounter Value Set; Hospice Intervention Value Set) or supplemental data for this required exclusion. If organizations use the Monthly Membership Detail Data File to identify these members, use only the run date of the file to determine if the member elected to use a hospice benefit during the measurement year.

Organizations should attempt to remove these members prior to determining a measure's eligible population and drawing the sample for hybrid measures. If a member is found to be in hospice or using hospice services during medical record review, the member is removed as a valid data error from the sample and replaced by a member from the oversample. Documentation that a member is near the end of life (e.g., comfort care, DNR, DNI) or is in palliative care does not meet criteria for the hospice exclusion.

The exclusion of members in hospice is subject to auditor review.

Note

- *Members in hospice are not excluded from the measures in the Health Plan Descriptive domain.*
- *Supplemental data can be used for the hospice exclusion for all applicable measures, including measures that say "supplemental data may not be used for the measure" (e.g., PCR).*
- *For ECDS reporting, hospice data from Monthly Membership Detail Data Files must be flagged for the administrative Source System of Record.*

16. Deceased Members

Exclude members who die any time during the measurement year. These members may be identified using various methods that may include, but are not limited to, enrollment data, medical record, claims/encounter data or supplemental data for this required exclusion.

Organizations should attempt to remove these members prior to determining a measure's eligible population and drawing the sample for hybrid measures. A deceased member found during medical record review is removed as a valid data error from the sample and replaced by a member from the oversample.

Exclusion of deceased members is subject to auditor review.

Note

- *NCQA does not require organizations to develop databases or other methods to identify deceased members.*
- *Deceased members are not excluded from measures in the Health Plan Descriptive domain or the Utilization domain (except for measures similar to EOC measures: Well-Child Visits in the First 30 Months of Life and Child and Adolescent Well-Care Visits).*
- *Supplemental data can be used for excluding deceased members for all applicable measures, including measures that say "supplemental data may not be used for the measure" (e.g., URI).*
- *This is a member-level exclusion. For episode-based measures, if one event does not meet numerator criteria, remove all member events/episodes from the measure.*

Membership Changes

17. Members Who Switch Organizations

Members who switch to different organizations or to a sister organization may be counted as continuously enrolled if they joined an organization that assumes ownership of or responsibility for members' administrative data and medical records for the entire period of continuous enrollment specified in the measure.

If an organization reports these members as continuously enrolled, it follows the definition of "continuous enrollment" in *General Guideline 20: Continuous Enrollment*, and all other guidelines affecting continuous enrollment (allow switching between products [HMO, POS, PPO, EPO] or product lines [Medicaid, commercial, Medicare]) consistently, across all measures.

18. Members Who Switch Organizations as a Result of a Merger or Acquisition

Measures with a continuous enrollment period	Members who switch organizations because of a merger that occurred during the measurement year may be counted as continuously enrolled.
Measures without a continuous enrollment period	The surviving organization may include members from the non-surviving entity in the eligible population, starting on the official date of the merger or acquisition. For example, if the merger or acquisition occurred on March 1 of the measurement year, the surviving organization excludes members acquired from the non-surviving entity from the eligible population for January and February.

This guideline must be used consistently across all measures.

19. Members Who Switch Products/Product Lines

Measures with a continuous enrollment requirement	Members who enrolled in different **products** or **product lines** in the time specified for continuous enrollment for a measure are continuously enrolled and are included in the product and product-line specific HEDIS report in which they were enrolled as of the end of the continuous enrollment period. For example, a member enrolled in the Medicaid product line who switches to the commercial product line during the continuous enrollment period is reported in the commercial HEDIS report. If a measure allows a gap at the end of the continuous enrollment period, report members in the product and product line-specific HEDIS report in which they were enrolled as of the last enrollment segment. Members who "age in" to a Medicare product line mid-year are considered continuously enrolled if they were members of the organization through another product line (e.g., commercial) during the continuous enrollment period and their enrollment did not exceed allowable gaps. The organization must use claims data from all products/product lines, even when there is a gap in enrollment.
Measures without a continuous enrollment requirement	Members who enrolled in different products or product lines are reported in the product and product line-specific HEDIS report in which they were enrolled on the date of service (outpatient, ED or observation visits) or date of discharge (inpatient stays).

Required Enrollment Periods and Benefits

20. Continuous Enrollment

Continuous enrollment specifies the minimum amount of time that a member must be enrolled in an organization before becoming eligible for a measure. It ensures that the organization has enough time to render services. The continuous enrollment period and allowable gaps are specified in each measure. To be considered continuously enrolled, a member must also be continuously enrolled with the benefit specified for each measure (e.g., pharmacy or mental health), accounting for any allowable gap.

A **gap** is the time when a member is not covered by the organization (i.e., the time between disenrollment and re-enrollment). For example, if a member disenrolls on June 30 and re-enrolls on July 1, there is no gap, because the member is covered by the organization on both June 30 and July 1. If the member disenrolls on June 30 and re-enrolls on July 2, there is a 1-day gap because the member is without coverage on July 1.

An **allowable gap** can occur any time during continuous enrollment. For example, the Child and Adolescent Well-Care Visits measure requires continuous enrollment throughout the measurement year (January 1–December 31) and allows one gap in enrollment of up to 45 days. A member who enrolls for the first time on February 8 of the measurement year is considered continuously enrolled as long as there are no other gaps in enrollment throughout the remainder of the measurement year. The member has one 38-day gap (January 1–February 7).

21. Medicaid Continuous Enrollment

If the organization applies a full-month eligibility criterion to Medicaid beneficiaries and verifies enrollment prospectively in monthly intervals (in 1-month increments), the one gap in enrollment during the continuous enrollment period may not exceed 45 days. For example, a member whose coverage lapses for 2 months (60 days) is not considered continuously enrolled.

If the organization is notified of prospective member enrollment, use the actual date of enrollment to calculate continuous enrollment, not the notification date.

Retroactive eligibility	The elapsed time between the actual date when the organization became financially responsible for the Medicaid member and the date when it received notification of the new member. For measures with a continuous enrollment requirement, members may be excluded if the retroactive eligibility period exceeds the allowable gap requirement. This guideline must be used consistently across all measures.

22. Continuous Enrollment Over Multiple Years

Unless otherwise specified, for measures that span more than 1 year, members are allowed one gap in enrollment of up to 45 days during each year of continuous enrollment. A gap in enrollment that extends over multiple years of a continuous enrollment period may exceed 45 days. For example, in the Colorectal Cancer Screening measure (which requires 2 years of continuous enrollment), a member who disenrolls on November 30 of the year prior to the measurement year and re-enrolls on February 1 of the measurement year is considered continuously enrolled as long as there are no other gaps in enrollment during either year. The member has one gap of 31 days (December 1–31) in the year prior to the measurement year and one gap of 31 days (January 1–31) in the measurement year.

23. Anchor Dates

If a measure requires a member to be enrolled and to have a benefit on a specific date, the allowable gap must not include that date; the member must also have the benefit on that date. For example, a 30-year-old woman who has only one gap in enrollment from November 30 of the measurement year throughout the remainder of the year is not eligible for the Cervical Cancer Screening measure. Although she meets the continuous enrollment criteria, she does not meet the anchor date criteria, which requires her to be enrolled as of December 31 of the measurement year.

24. Required Benefits

HEDIS measures evaluate performance and hold organizations accountable for services provided in their members' benefits package. Measure specifications include benefits (medical, pharmacy, mental health, chemical dependency) required during the continuous enrollment period. HEDIS measures do not define benefits at the service level (e.g., if the organization offers a pharmacy benefit but does not cover a specific medication class, the member has a pharmacy benefit and is included in the applicable measures requiring this benefit, similarly if the member has partial coverage of mental health services (either by service or diagnosis), they are included as having a mental health benefit). Organizations must assess benefits first at the organization level and then at the individual member level using continuous enrollment data.

…at the organization level

Organizations report HEDIS measures requiring a specific benefit provided to members directly or through a contractor.

Organizations are not required to report HEDIS measures specifying a benefit that it does not offer.

Before reporting a measure specifying a benefit, the organization must be able to determine if a member has the required benefit.

If the organization does not offer the benefit, the plan does not report the measure and receives an NB (No Benefit) audit designation. No member assessment is necessary.

…at the member level

Members who do not have a specified benefit are not counted in the measure. For example, exclude members without a pharmacy benefit from the Asthma Medication Ratio measure. When members are dual enrolled, organizations must assess the benefit requirements based on the submission in which the member is included, refer to *General Guideline 13: Members With Dual Enrollment*.

Exhausted benefits *(optional)*

For measures without a continuous enrollment criterion, include only services or procedures that occurred while the member had a benefit. For a member whose benefit is lost or exhausted during the time specified in the measure, include services or procedures that occurred while the member had the benefit.

For measures with a continuous enrollment criterion, the required benefits must be active for the period of continuous enrollment, accounting for any allowable gap. Exclude a member if the period when the benefit is exhausted exceeds any allowable gap or anchor date. For example, the Asthma Medication Ratio measure requires a pharmacy benefit during the measurement year. Exclude a member whose pharmacy benefit is exhausted in September of the measurement year, because this exceeds the 45-day allowable gap period.

Carved-out benefits *(optional)*	Some organizations can obtain the necessary information from a carved-out entity and may include these members in their measures. For example, an employer contracts directly with a pharmacy benefit manager (PBM), which shares pharmacy information with the organization. The employer's members may be included in the measure.

This guideline must be used consistently across all measures.

25. Accessing Medical Records Prior to Enrollment

Data that can be accessed from a medical record are used to calculate a measure. If data from a medical record cannot be accessed because data were updated before the member was enrolled, the organization calculates the measure with the data that are available.

HEDIS Data Submission and Reporting

26. HEDIS Reporting Date

For HEDIS MY 2023, all organizations reporting audited data to NCQA through the IDSS must submit data on or before **June 14, 2024.**

Note: *Organizations must submit and "plan-lock" audited HEDIS data to allow auditors sufficient time to review, approve and audit lock all submissions by the June 14 deadline. For HEDIS MY 2023 reporting, organizations are required to "plan-lock" audited HEDIS data no later than* ***June 1, 2024.***

27. Required Data Elements

Organizations that submit audited HEDIS data to NCQA must report the data elements identified in each measure specification. Data elements are standard for hybrid and administrative measures. Refer to *General Guideline 52: Reporting Tables* and *Appendix 4: Data Element Definitions.*

Data Collection Methods and Data Sources

28. Data Collection Methods

HEDIS measures are specified for one or more data collection methods:

- Administrative Method.
- Hybrid Method.
- Survey Method.
- Electronic Clinical Data Systems (ECDS).

Each measure specifies the data collection methods that must be used. If a measure includes both the Administrative and Hybrid Methods, either method may be used.

Administrative Method	Transaction data or other administrative data are used to identify the eligible population and numerator. The reported rate is based on all members who meet the eligible population criteria and who are found through administrative data to have received the service required for the numerator.

Hybrid Method Organizations look for numerator compliance in both administrative and medical record data. The denominator consists of a systematic sample of members drawn from the measure's eligible population. Organizations review administrative data to determine if members in the systematic sample received the service and review medical record data for members who do not meet the numerator criteria through administrative data. The reported rate is based on members in the sample who received the service required for the numerator.

Survey Method Requires organizations to collect data through a survey. Specifications for survey measures are included in *HEDIS Volume 3: Specifications for Survey Measures* and *HEDIS Volume 6: Specifications for the Medicare Health Outcomes Survey.*

The Medical Assistance With Smoking and Tobacco Use Cessation measure specifications are included in HEDIS Volume 2.

ECDS Method Refer to the *Guidelines for Measures Reported Using Electronic Clinical Data Systems* for additional information for this data collection method.

Note

- *Supplemental data are considered an administrative data source; however, for the EOC measures and Utilization measures similar to EOC (W30 and WCV), numerator events identified using supplemental data are reported separately from numerator events identified by administrative (claims/ encounter) and medical record data, as indicated in the applicable Data Elements for Reporting tables.*
- *Any data found in a supplemental data source are considered a supplemental data hit if the member would not be compliant for the measure/indicator without the data source. If supplemental data are not used, report zero in the "Numerator events by supplemental data" element. For all other measures, numerator events identified using supplemental data are reported in the "Numerator events by administrative data" element. Refer to General Guideline 29: Supplemental Data.*
- *For the Transitions of Care measure, there are two numerators where administrative reporting is not available; only medical record data or supplemental data may be used. Supplemental data must include all elements required by the measure's hybrid specifications for these indicators; once they are validated, the organization must determine how to integrate the results for reporting. Organizations should work with their vendors and the process should be reviewed and approved by the auditor. Supplemental data for Notification of Inpatient Admission and Receipt of Discharge Information indicators must be reported in the "Numerator events by supplemental data" data element in IDSS.*

29. Supplemental Data

Supplemental data uses Organizations may find information about services for their members in administrative data, medical records and other data sources. When evidence to support the measure is found in multiple data sources, a hierarchy is applied. Supplemental data are considered last as long as the specifications are followed as written (e.g., if the organization uses a combination of data sources to identify the HbA1c control indicators in the Hemoglobin A1c Control for Patients With Diabetes measure, the most recent test must be used, regardless of data source).

For administrative-only measures, medical record data are considered supplemental data.

Supplemental data may help determine:

- Numerators that are labeled as *numerators* in the specification.
- Members in hospice and members who have died.
- Eligible-population required exclusions that are labeled as required exclusions in the specification.

Supplemental data may not be used for:

- Denominator events. Organizations *may not* create and use records to identify denominator events, other than for required exclusions.
- Clinical conditions that change. Organizations *may not* create and use records, on an ongoing basis, for exclusions for clinical conditions that change.
- Correcting bills or identifying valid data errors. Organizations *may not* use supplemental data to adjust incorrect billing practices or to identify valid data errors. This practice results in a change in claims data and is not allowed.
- Measures where the specification specifically indicates supplemental data cannot be used, except for applying the hospice exclusion and for excluding deceased members.

Supplemental Data Definitions

The auditor determines the classification of all supplemental data, not the organization.

Standard supplemental data

Electronically generated files that come from service providers (providers who rendered the service). Production of these files follows clear policies and procedures; standard file layouts remain stable from year to year.

Audit requirements. Standard supplemental data are not required to be accompanied by proof-of-service documents and the audit does not require primary source verification, unless requested by the auditor.

Note: *Prior years' validated historic hybrid medical record result files are reviewed as part of the Data Preproduction Processing section of the HEDIS Roadmap. These data are loaded as administrative data.*

Nonstandard supplemental data

Data used to capture missing service data not received through administrative sources (claims or encounters) or in the standard electronically generated files described above, whether collected by a plan, an organization, a provider or a contracted vendor. These types of data might be collected from sources on an irregular basis and could be in files or formats that are not stable over time.

Organizations must have clear policies and procedures that describe how the data are collected and by whom, how they are validated and used for HEDIS reporting.

Organizations *may not* conduct phone calls to members or providers to collect information about services already rendered.

Audit requirements. All nonstandard supplemental data must be substantiated by proof-of-service documentation from the legal health record. Proof-of-service documentation is required for only a sample, selected by the auditor, as part of the audit's annual primary source verification.

Proof-of-service documentation that *is allowed* for primary source verification:

- A copy of the information from the member's chart from the service provider or the PCP.
- A copy of the clinical report or clinical summary from the visit for service, such as lab or radiology reports (i.e., forms from the rendering provider proving the service occurred).
- A screen shot of:
  - Online electronic health record (EHR) records.
  - State- or county-sponsored immunization registry records.

Proof-of-service documentation that is *not allowed* for primary source verification*:*

- Member surveys. Organizations and providers may not use information obtained from surveys or other documents completed by the member, except for data collected for Language Diversity of Membership and Race/Ethnicity Diversity of Membership.
- Phone calls. Recorded phone calls to collect information about services rendered are not proof of service.

CCDs Continuity of Care Documents. CCDs are used for the electronic exchange of clinical data without loss of meaning. The files provide a summary of a patient's care as a snapshot in time, but they are not a replacement for an EHR. These files are typically XML-based and are considered nonstandard supplemental data for at least the first year of use. The organization must demonstrate the accuracy of these (through PSV) to ensure that the data in the file match the EHR. This data source must meet both criteria:

- There is a completed, current year's Roadmap section.
- The Roadmap must include a description of how the CCD is created and by whom (e.g., produced by the provider in the office and sent to the plan or created by a vendor), the validation process and how the data are transmitted.

Audit requirements. The auditor confirms that the data meet all requirements. Primary source verification is required (e.g., go back to each unique EHR) to validate the CCDs' accuracy. This level of validation is required for at least the first year, or the first submission by the EHR, but may continue in subsequent years until the auditor is certain the data are accurate, reliable and unchanged.

NCQA DAV data For data from an NCQA-Validated Data Aggregator Validation (DAV) entity, the auditor must:

- Receive a completed current year's Roadmap Section 5 from the reporting entity using the data. The Roadmap must explain how data from the validated DAV entity is transferred to the reporting entity and what the entity does to the data. This is completed by the health plan; no documentation is required from the DAV entity, which has already been validated.
  - If the reporting entity processes the validated CCD in any way after receipt, the auditor must validate the file back to the original validated CCD to ensure that no data were changed.

- Receive the final validation report that indicates the validated data cases and clusters and the date when they were validated.

If an NCQA-validated DAV entity includes data from an unvalidated data cluster, the auditor must validate the data, following the nonstandard supplemental data guidelines, before the data can be used for HEDIS reporting.

The auditor may not perform PSV on any validated data files.

Required Data Elements

Standard supplemental data

Organizations must have policies and procedures for using data files as standard supplemental data. Data files must have standard file layouts, standard data fields and industry standard codes, and must include all elements required by measure specifications, including payment status when applicable, and evidence that tests or services were performed and not merely ordered.

Nonstandard supplemental data

Nonstandard supplemental data must have all data elements required to meet criteria specified by the measure specifications, including:

- Payment status when applicable.
- Evidence that tests or services were performed, not just ordered.
  - When data are abstracted from medical record sources to be used as supplemental data, codes alone (without additional documentation of the service provided) do not meet criteria for proof of service. If a provider performs a service, it is expected that there is additional documentation in the medical record or in the primary source document. Auditors must validate, through primary source verification, all elements required by the measure specification.
- Evidence of provider accountability from the practitioner or practitioner group (signed contracts with accountability tied to passwords, signatures or TIN/NPI data). For home visits, if clinical services are rendered, there must be evidence of accountability by the practitioner, and at a minimum include the date, name and signature on each in-home form. Documentation of the practitioner's TIN/NPI is not required; however, documentation of TIN/NPI with date, name and signature is preferred.
- More than a simple yes or no attestation on provider forms. Forms must have all necessary data elements and be signed by the rendering practitioner.
- All data elements for a measure must be captured for member-reported services (date and place of service, procedure, prescription, test result or finding, practitioner type). When using supplemental data derived from medical records to meet administrative specifications, documentation must be clinically synonymous with the codes included in the measure's value sets. Refer to *General Guideline 39: Member-Reported Services and Biometric Values*.

All supplemental data

All proof-of-service documents must show that services were rendered by the deadline established for the measure (refer to *General Guideline 34: Date Specificity* for date specificity requirements).

When pharmacy data are classified as supplemental data, all of the following data elements must be present: the generic name (or brand name), strength/

dose, route and date when the medication was dispensed or shipped to the member. For mail order prescriptions "shipped date" meets criteria for dispense date. "Start date" documented in the medical record does not meet criteria. Data elements must map to a medication listed in the Medication List Directory to be eligible for use. Generic documentation in the medical record (e.g., that a patient "was prescribed" or "is taking" a medication) that does not include drug name, strength/dose and dispense date does not meet criteria.

All supplemental data used to show eligibility for exclusions must follow the requirements for exclusions in each measure.

Supplemental Data Timeline

Supplemental data may be collected during the measurement year and into the beginning of the reporting year. Supplemental data collection and use must adhere to all applicable deadlines in the Audit Timeline posted on NCQA's website on March 31, 2023.

Identifying and Validating Supplemental Data

All supplemental data (standard and nonstandard) must be identifiable. Because supplemental data can affect reporting and incentives, plans or vendors that use supplemental data for HEDIS reporting must mark the data files, regardless of the source. Auditors must be able to assess the contribution of each supplemental data source to the applicable components of the measure (numerator events or appropriate exclusions).

Auditors must review all supplemental data annually—there are no exceptions. At a minimum, the annual review includes the following for each supplemental data source:

- A completed current year's *Supplemental Data* section of the HEDIS Roadmap, including all attachments.
- Impact of supplemental data source by measure (e.g., lists of numerator-positive hits from the supplemental data, by measure; year-to-year comparisons of percentage increases associated with supplemental data; proportion of numerator compliance from supplemental data).
- Primary source verification where required or requested by the auditor.

Supplemental data that do not pass all audit validation steps by the deadline may not be used to calculate HEDIS rates. Organizations may wait to load supplemental data until primary source verification is complete and the source is approved.

For additional information about audit requirements for supplemental data, refer to *Volume 5, HEDIS Compliance Audit: Standards, Policies and Procedures.*

30. Obtaining Information for the Systematic Sample

Organizations (and their contractors) that use the Hybrid Method are responsible for determining compliance with HEDIS measurement specifications. Information may be abstracted from the member's legal health record by designated medical record review (MRR) staff. Abstraction of data for members in the systematic sample is performed by entities or vendors who adhere to training, policies and procedures, use of appropriate tools, oversight and all other audit components.

MRR abstractors count a service if the legal health record contains the date of the service and evidence that the service occurred. All services must be rendered and documented in the medical record by the deadline established in the measure (e.g., by the child's second birthday, for the Childhood Immunization Status measure).

Organizations must be able to determine that a test or service was *performed* within the time frame specified, not merely ordered. Only completed events count toward HEDIS compliance. Documentation in a medical record of a diagnosis or procedure code alone does not comply with the numerator criteria.

Processes used to determine the validity and integrity of abstracted data, including interrater reliability, quality control and rater-to-standard results, are reviewed by the certified HEDIS Compliance Auditor.

Data refresh for the systematic sample

Because NCQA requires that the systematic sample be stable and reproducible, organizations may not change the sample after it is created. If an organization refreshes the HEDIS repository after the sample is drawn and chart review is in progress, it should follow the guidelines below to use the newer administrative data for all hybrid measures.

Exclusions found through administrative data in a data refresh must be reported in the "ExclusionValidDataErrors" data element.

Note: *Organizations may elect to refresh data for administrative-only measures but must apply the refresh to all applicable measures.*

Manually updating the sample

Organizations may compare only the numerator-negative members in the sample to screen shots of the refreshed data; they are not required to update every measure manually or to reassess denominator compliance for every member in the sample.

Records used for numerator compliance are subject to medical record review validation.

Automated updates to the sample

Organizations may use an automated process that loads the entire sample for each measure and compares it to the refreshed data. All data must be used consistently in the samples.

- If recent data contradict numerator compliance, those data must be used.
- If recent data exclude a member, those data must be used and the oversample must provide a substitute member.
- If the oversample is exhausted, the organization must use the Sampling Guidelines to ensure meeting the minimum required sample size (MRSS) is possible.
- The auditor must review and approve the timing, processes and results of the refresh, but does not need to include the records used for numerator compliance in the medical record review validation.

31. Race and Ethnicity Stratification

The following measures instruct the organization to categorize Medicare, Medicaid and Commercial members by the race and ethnicity stratification (RES):

- Immunizations for Adolescents (including IMA-E).
- Colorectal Cancer Screening (including COL-E).
- Asthma Medication Ratio.
- Controlling High Blood Pressure.
- Hemoglobin A1c Control for Patients With Diabetes.
- Follow-Up After Emergency Department Visit for Substance Use.
- Pharmacotherapy for Opioid Use Disorder.

- Initiation and Engagement of Substance Use Disorder Treatment.
- Prenatal and Postpartum Care.
- Well-Child Visits in the First 30 Months of Life.
- Child and Adolescent Well-Care Visits.
- Breast Cancer Screening.
- Adult Immunization Status.

Refer to *Appendix 7: Logical Measure Groups* for measures that include RES, by logical measure group.

Reporting categories

NCQA requires reporting race and ethnicity as defined by the Office of Management and Budget (OMB) Standards for Maintaining, Collecting, and Presenting Federal Data on Race and Ethnicity.[1,2,3]

Race and ethnicity values must be rolled up into the OMB categories specified in this guideline. If more detailed race or ethnicity information is collected, these data must be aggregated and reported in the OMB categories provided. For health plans using the CMS classification scheme for race and ethnicity, refer to Table RES-A-1/2/3 for a crosswalk to HEDIS reporting.

Report member race and ethnicity separately. If a combined race/ethnicity category question is used to collect data, data must be disaggregated, and race and ethnicity categories must be reported separately. When using the combined race/ethnicity data format for collection, refer to Table RES-B-1/2/3 for a crosswalk of reporting categories.

Tables RES-C-1/2/3 and RES-D-1/2/3 crosswalk the HEDIS reporting categories to code values specified by the Race and Ethnicity extensions of the HL7 US Core Implementation Guide. Organizations must use or map to the documented Direct reference codes and Value sets described here. Code values originate from two code systems:

- "Race & Ethnicity – CDC" (CDCREC) is used to report distinct OMB race and ethnicity categories.
- "Some Other Race," "Asked but No answer" and "Unknown" use the HL7 version 3 NullFlavor code system.

Determining race reporting category

For each product line, report members in only one of the nine race stratifications listed below and the total.

- *White*: Identification with one or more nationalities or ethnic groups originating in Europe, the Middle East or North Africa. Examples of these groups include, but are not limited to, German, Irish, English, Italian, Lebanese, Egyptian, Polish, French, Iranian, Slavic, Cajun and Chaldean.
- *Black or African American*: Identification with one or more nationalities or ethnic groups originating in any of the Black racial groups of Africa. Examples of these groups include, but are not limited to, African American, Jamaican, Haitian, Nigerian, Ethiopian and Somali. The

[1] Office of Management and Budget Revisions to the Standards for the Classification of Federal Data on Race and Ethnicity. https://www.govinfo.gov/content/pkg/FR-1997-10-30/pdf/97-28653.pdf
[2] 2020 Census Questions: Race. https://2020census.gov/en/about-questions/2020-census-questions-race.html
[3] 2020 Census Questions: Hispanic Origin. https://2020census.gov/en/about-questions/hispanic-origin.html

category also includes groups such as Ghanaian, South African, Barbadian, Kenyan, Liberian and Bahamian.

- *American Indian or Alaska Native*: Identification with any of the original peoples of North and South America (including Central America) and who maintain tribal affiliation or community attachment. It includes people who identify as "American Indian" or "Alaska Native" and includes groups such as Navajo Nation, Blackfeet Tribe, Mayan, Aztec, Native Village of Barrow Inupiat Traditional Government and Nome Eskimo Community.
- *Asian*: Identification with one or more nationalities or ethnic groups originating in the Far East, Southeast Asia or the Indian subcontinent. Examples of these groups include, but are not limited to, Chinese, Filipino, Asian Indian, Vietnamese, Korean and Japanese. The category also includes groups such as Pakistani, Cambodian, Hmong, Thai, Bengali or Mien.
- *Native Hawaiian or Other Pacific Islander*: Identification with one or more nationalities or ethnic groups originating in Hawaii, Guam, Samoa, or other Pacific Islands. Examples of these groups include, but are not limited to, Native Hawaiian, Samoan, Chamorro, Tongan, Fijian and Marshallese. The category also includes groups such as Palauan, Tahitian, Chuukese, Pohnpeian, Saipanese or Yapese.
- *Some Other Race*: People whose race information has been collected but does not fit into any of the other seven race categories. This category includes people who may be Mulatto, Creole and Mestizo or another race not specified in the Census "Race" categories.
- *Two or More Races*: People with any combination of races, including "Some Other Race."
- *Asked but No Answer:* People who the organization asked to identify race but who declined to provide a response.
- *Unknown:* People for whom the organization did not obtain race information and for whom the organization did not receive a declined response (i.e., "Asked but No Answer").
- *Total:* Total of all categories above.

Notes:

- *The "Asked but No Answer" category is only reported using direct data.*
- *The "Unknown" category is only reported using indirect data.*

Determining ethnicity reporting category

For each product line, report members in only one of the four ethnicity stratifications listed below and the total.

- *Hispanic or Latino*: Identification with one or more nationalities or ethnic groups originating in Mexico, Puerto Rico, Cuba, Central and South America and other Spanish cultures. Examples of these groups include, but are not limited to, Mexican or Mexican American, Puerto Rican, Cuban, Salvadoran, Dominican and Colombian. "Hispanic, Latino or Spanish origin" also includes groups such as Guatemalan, Honduran, Spaniard, Ecuadorian, Peruvian or Venezuelan.
- *Not Hispanic or Latino*: People not of Hispanic, Latino or Spanish culture or origin.

- *Asked but No Answer:* People who the organization asked to identify ethnicity but who declined to provide a response.
- *Unknown:* People for whom the organization did not obtain ethnicity information and for whom the organization did not receive a declined response (i.e., "Asked but No Answer").
- *Total:* Total of all categories above.

Notes:

- *The "Asked but No Answer" category is only reported using direct data.*
- *The "Unknown" category is only reported using indirect data.*

Data source

Approved data sources include data collected directly from members or data obtained through indirect methods. NCQA strongly encourages plans to report directly collected data when available and emphasizes the importance of improving completeness of directly collected member race and ethnicity data.

For each measure with the race and ethnicity stratification, plans will report each race and ethnicity value by data source. Plans will report the number of members in the eligible population from direct and indirect data sources, and the number of members in the numerator from direct and indirect data sources. IDSS will calculate the total number of members in the eligible population and numerator (combining both direct and indirect data sources).

Supplemental data may be used as a data source for RES.

Direct data

Data collected directly from members method reflects members' self-identification and is the preferred data source.

Directly collected data includes any source for which the member self-identified race or ethnicity. This includes data collected directly from members by the health plan, as well as third-party data collected directly from a member by another entity (e.g., the state or CMS). Direct sources may include, but are not limited to:

- Surveys.
- Health risk assessments.
- Disease management registries.
- Case management systems.
- EHRs.
- CMS/state databases.
- Enrollment information furnished by enrolling entities (e.g., state Medicaid agencies, employers).
- CCDs.

Indirect data

Plans may choose to report race and ethnicity data supplemented by indirect methods. Indirect assignment of race and ethnicity values include using an alternate data source, such as nationally representative data obtained from databases like the American Community survey, to assign a race or ethnicity value to a member based on their primary location of residence. Some commonly used indirect methods combine geographic data with additional imputation methods such as surname analysis.

NCQA reiterates that directly collected race and ethnicity is considered the gold standard and is highly preferred to indirectly assigned race and ethnicity. For plans choosing to use indirect methods to report the HEDIS race and ethnicity stratification, NCQA emphasizes the following:

- When applying indirect methods that involve assignment of race or ethnicity based on geographic data and member's location of residence, the smallest geographic unit possible is preferred. For example, geographic assignment at the census block level is likely to be more accurate than assignment using census tract or zip code level data.
- Indirect data sources and methods should be evaluated for reliability and validity and selection of a source and method should be prioritized based on demonstrated validity and reliability for the population in which it will be applied (e.g., age group, geography, product line).
- Indirect methods of race and ethnicity assignment are to be used for population-level reporting and analysis but are not appropriate for member-level intervention.

Sampling

For measures collected using the Hybrid Method with the race and ethnicity stratification, follow the guidelines for sampling outlined in *Guidelines for Calculation* and *Sampling Guidelines for the Hybrid Method.* The race and ethnicity stratifications are applied to the eligible population and denominator after hybrid sampling.

Reporting

Reporting of the race and ethnicity stratification follows the parameters for denominator size outlined in *General Guideline 9: Reporting*.

Table RES-A-1/2/3: CMS Categories Crosswalked to HEDIS/OMB Race and Ethnicity

CMS Category	HEDIS/OMB Race	HEDIS/OMB Ethnicity
White	White	Unknown
Black	Black	Unknown
American Indian/Alaska Native	American Indian or Alaska Native	Unknown
Asian/Pacific Islander	Asian	Unknown
Hispanic	Unknown	Hispanic or Latino
Other	Some Other Race	Unknown
Unknown	Unknown	Unknown
(No equivalent category)	Native Hawaiian or Other Pacific Islander	Unknown
(No equivalent category)	Two or more races	Unknown

Table RES-B-1/2/3: Combined Categories Crosswalked to HEDIS/OMB Race and Ethnicity

Race/Ethnicity Combined Category	HEDIS/OMB Race	HEDIS/OMB Ethnicity
White	White	Not Hispanic or Latino
Black	Black	Not Hispanic or Latino
American Indian/Alaska Native	American Indian or Alaska Native	Not Hispanic or Latino
Asian	Asian	Not Hispanic or Latino
Native Hawaiian and Other Pacific Islander	Native Hawaiian or Other Pacific Islander	Not Hispanic or Latino
Hispanic/Latino/White	White	Hispanic or Latino
Hispanic/Latino/Black	Black	Hispanic or Latino
Other	Some Other Race	Unknown
Multiple races marked	Two or more races	Unknown
Unknown	Unknown	Unknown

Table RES-C-1/2/3: HEDIS/OMB Race Crosswalked for Use With HEDIS Reporting Categories

HEDIS/OMB Race	CDCREC OMB Category Direct Reference Code	CDCREC Detailed Category: Value Set
White	2106-3	White Detailed Race Value Set
Black	2054-5	Black or African American Detailed Race Value Set
American Indian or Alaska Native	1002-5	American Indian or Alaska Native Detailed Race Value Set
Asian	2028-9	Asian Detailed Race Value Set
Native Hawaiian or Other Pacific Islander	2076-8	Native Hawaiian or Other Pacific Islander Detailed Race Value Set
Some Other Race	OTH*	NA
Two or more races	NA**	NA
Asked but No Answer	ASKU*	NA
Unknown	UNK*	NA

* HL7 v3 Code System NullFlavor.

**This value is defined by the measure calculation logic as the presence of two or more distinct CDCREC category codes and does not map to a specific direct reference code or value set.

Table RES-D-1/2/3: HEDIS/OMB Ethnicity Crosswalked for Use With HEDIS Reporting Categories

HEDIS/OMB Race	CDCREC OMB Category Direct Reference Code	CDCREC Detailed Category: Value Set
Hispanic or Latino	2135-2	Hispanic or Latino Detailed Ethnicity
Not Hispanic or Latino	2186-5	NA
Asked but No Answer	ASKU*	NA
Unknown	UNK*	NA

*The NullFlavor concepts 'Asked but no answer' and 'Unknown' are not included in the terminology binding for the US Core Ethnicity FHIR extension on which this digital logic is structured. NCQA allows these concepts to express ethnicity data to align with bound values for the US Core Race extension.

Note

- *Race is a social construct, not biological; stratifying HEDIS measures by race and ethnicity is intended to be used to further understanding of racial and ethnic disparities in care and to hold health plans accountable to address such disparities, with the goal of achieving equitable health care and outcomes. Data are not to be used to further bias in health care or suggest that race and ethnicity are biological determinants of health.*
- *When multiple sources of data are used for race and ethnicity, there may be disagreements in the data collected. When this happens, data sources should be prioritized based on evaluation of anticipated accuracy. This includes use of specific categories over nonspecific categories, most frequent or consistently reported category and selection of data with clear provenance (source, method of collection) over data without clear provenance.*
- *Race and ethnicity data may come from different categories of data source (direct, indirect). In such cases, use the data source that applies to the data element (race, ethnicity). If the same data element is received from two different data sources, prioritize data sources based on the note above.*

32. Medicare Socioeconomic Status Stratification

SES stratification

The following measures instruct the organization to categorize Medicare members by socioeconomic status (SES) stratification.

- Breast Cancer Screening.
- Colorectal Cancer Screening.
- Hemoglobin A1c Control for Patients With Diabetes.
- Eye Exam for Patients With Diabetes.

Report Medicare members in only one of the six stratifications listed below. Use the LIS History (LISHIST) file corresponding to the continuous enrollment period to assess if the member meets the definition for the LIS/DE status.

Use the Monthly Membership Detail Data Report (MMDR) file for the Original Reason for Entitlement Code (OREC) Item 48 to determine age and disability status.

Assess continuous enrollment eligibility before assessing SES stratification.

- *Non-LIS/DE, Non-disability:* Member is eligible for Medicare due to age only (does not receive LIS, is not DE for Medicaid, does not have disability status).

- *LIS/DE:* Member is eligible for Medicare due to age and receives LIS (includes members eligible for Medicare due to DE) and does not have disability status.
- *Disability:* Member is eligible for Medicare due to disability status only.
- *LIS/DE and Disability:* Member is eligible for Medicare due to age, receives LIS ***and*** has disability status.
- *Other:* Member has ESRD-only status or is assigned "9—none of the above."
- *Unknown:* Member's SES is unknown. May be >0 only for Puerto Rico plans, or if the auditor approved a small number of unassigned members*.
- *Total Medicare:* Total of all categories above.

***Plans must work with auditors to uncover anomalies early (e.g., during preliminary rate review) and attempt to resolve the cause. Medicare members in Puerto Rico must be counted in the "Unknown" category.*

Except for plans in Puerto Rico, which report all members in "Unknown," it is expected that the member count in this category will be at or below 1%. If more than 1% of eligible members are assigned to "Unknown," the plan must work with the auditor to identify why members are being categorized as "Unknown."

Use the SES Stratification Logic table below to determine the member's stratification at any time during the measure's continuous enrollment period.

Table 1: SES Stratification Logic Using the MMDR (for OREC) and the LISHIST File (for LIS)

Strata	Logic	Rationale
Non-LIS/DE, Non-disability	No LISHIST record and item 48 = 0	No LISHIST record, no disability status, no ESRD.
LIS/DE	LISHIST record and item 48 = 0	LISHIST record; counts only payments due to age.
Disability	No LISHIST record and item 48 = 1 or 3	No LISHIST record; counts only payments with disability status (code 1 or 3).
LIS/DE and Disability	LISHIST record and item 48 = 1 or 3	LISHIST record; counts only payments with disability status (code 1 or 3).
Other	Item 48 = 2 (ESRD) or 9 (None of the above)	Counts ESRD and 9 (None of the above). LISHIST does not get evaluated if item 48 = 2 or 9.
Unknown	Item 48 = blank	Count members with no values assigned in the MMDR. Subject to auditor review and approval.
Total	Sum of the above.	

Determining LIS Status

To determine the LIS status, use the monthly LISHIST file. This file provides a comprehensive list of an organization's current LIS membership, with a record for every enrollee who has a subsidy. The data in the record about each beneficiary spans through the most recent 36 consecutive months of contract enrollment. This report also informs organizations whether a beneficiary is LIS in the next calendar year.

The LISHIST file will contain only beneficiaries (past, present and future) who are or were deemed eligible for a low-income subsidy. The file will not show beneficiaries who do not or have not had any LIS periods.

To use the LISHIST file, select the last monthly file issued during the measure's continuous enrollment period. If the member is in the LISHIST file and the "Low Income Period End Date" is blank or falls within

the continuous enrollment period, count the member as LIS eligible. If the member has no record in the LISHIST file or their end date is before the continuous enrollment period, count the member as not eligible.

Determining OREC

To determine the OREC value, select the files in the MMDR issued during the measure's continuous enrollment period, based on the file's run date. Following the guidance in the Logic Table above, use Item 48 in that file to determine the OREC value and count the member in the appropriate SES strata.

Note: *Although it is rare that a beneficiary's OREC value is blank or changes during the continuous enrollment period, if the last month's value is missing, use the last available value during the continuous enrollment period.*

33. Date of Service for Laboratory Tests

Laboratory tests can have multiple dates of service: an order date (the date when the provider ordered the test), a collection date (the date when the specimen was drawn), a result/reported date (the date when results were calculated and reported), a claim date (the date of service on the claim) and a documented date (the date when the provider documented the result in the medical record).

Order date and documented date are not eligible for use in HEDIS reporting.

For laboratory tests identified using claims data (numerator events by administrative data), use the claim date of service.

When abstracting laboratory tests from the medical record for use in hybrid reporting or for nonstandard supplemental data, the documentation must include the test date and the result (or evidence that the test was performed). The result/reported date may be used as the test date.

Organizations may consider all events with dates no more than 7 days apart to be the same test and may use the collected date for reporting. For example:

- If a child had a lead screening sample collected on December 28 of the measurement year and a lead screening result on January 2 of the year after the measurement year, the dates are within 7 days and can be considered the same test. The result is present and the collection date is eligible for use, making the member numerator compliant for lead screening.
- If a child had a lead screening sample collected on December 28 of the measurement year and a lead screening result on January 15 of the year after the measurement year, the dates are not within 7 days and cannot be considered the same test. There is no result for the lead test collected on December 28 and the member is not numerator compliant for lead screening.
- If a test had a collection date of December 1 and a reported date of December 8, these dates are not more than 7 days apart and can be considered the same test.
- If a test had a collection date of December 1 and a reported date of December 9, these dates are more than 7 days apart and cannot be considered the same test.

34. Date Specificity

HEDIS requires that a date be specific enough to determine that an event occurred during the time frame established in the measure. For example, in the Childhood Immunization Status measure, members must receive three hepatitis B vaccines. For HEDIS MY 2023, assume a member was born on February 5, 2021. Documentation in the medical record that the first hepatitis B vaccine was given "at birth" is specific enough to determine that it was given prior to the deadline for this measure (the child's second birthday), but if the medical record states that the third hepatitis B vaccine was given in February 2023, the organization cannot count the immunization, because the date is not specific enough to

confirm that it occurred prior to the member's second birthday.

There are instances when documentation of the year alone is adequate; for example, measures that look for events in the "measurement year or the year prior to the measurement year." Terms such as "recent," "most recent" or "at a prior visit" are not acceptable.

For documented history of an event (e.g., documented history of a disease), undated documentation may be used if it is specific enough to determine that the event occurred during the time frame specified in the measure. For example, for the Childhood Immunization Status measure, undated documentation on an immunization chart stating "chicken pox at age 1" is specific enough to determine that it occurred prior to the child's second birthday. Similarly, for the Breast Cancer Screening measure, undated documentation on a problem list stating "bilateral mastectomy in 1999" is specific enough to determine that this exclusion occurred on or before December 31 of the measurement year.

35. Collecting Data for Measures With Multiple Numerator Events

The following measures require more than one event to satisfy the numerator:

- Childhood Immunization Status.
- Immunizations for Adolescents.
- Topical Fluoride for Children.
- Well-Child Visits in the First 30 Months of Life.

For only the measures listed above, the organization may use a single data source or a combination of administrative data, which may include audited supplemental data, and medical record data to determine numerator compliance for members in the denominator. To avoid double counting events when only assessing administrative data or when combining administrative and medical record data, all events must be at least 14 days apart.

For example, the organization may count two influenza vaccines identified through administrative data if the dates of service are at least 14 days apart; if the service date for the first vaccine was February 1, then the service date for the second vaccine must be on or after February 15. When combining administrative and medical record data, the dates of service must also be at least 14 days apart in order to count toward numerator compliance.

If the organization has one event from the medical record and one from administrative data and dates are less than 14 days apart (or the organization cannot determine if the dates are at least 14 days apart), it must use only the medical record event. The 14-day threshold does not apply when using only medical record data. For example, the organization may count two influenza vaccines identified through medical record data that are not 14 days apart.

36. Measures That Use Medication Lists

Some measures require the use of clinical pharmacy data or pharmacy claims data to identify dispensed medications. The specifications reference medication lists that must be used for HEDIS reporting for each pharmacy-dependent measure. In the specifications, medication list references are underlined (e.g., ADHD Medications List). Medication lists used for HEDIS reporting are included in the Medication List Directory. A medication list includes the National Drug Codes (NDC) and RxNorm codes that may be used for reporting along with the generic name, the brand name (if applicable), the strength/dose and the route for each code.

If an organization uses both pharmacy data (NDC codes) and clinical data (RxNorm codes) for reporting, to avoid double counting, if there are both NDC codes and RxNorm codes on the same date of service, use only one data source for that date of service (use only NDC codes *or* only RxNorm codes) for reporting.

Note: *RxNorm codes cannot be used to assess the numerator in the Asthma Medication Ratio measure.*

37. Identifying Events/Diagnoses Using Laboratory *or* Pharmacy Data

Many organizations find a high rate of false positives when they use laboratory data to identify members with a disease or condition. Diagnosis codes are frequently reported on laboratory tests in cases where the condition is being ruled out. Use laboratory claims and data only for the Pregnancy Tests Value Set and the Sexual Activity Value Set (which do not contain LOINC codes) and value sets that contain LOINC codes.

Claims data indicating a member had a laboratory test during a visit with a provider are not considered laboratory data. Laboratory data are claims or lab result data for the sole purpose of a laboratory test performed outside of a visit with a provider. Claims with a code from the Independent Laboratory Value Set are considered laboratory claims. Organizations may need to use other methods to differentiate between laboratory claims data and clinical/provider claims that may include a laboratory test.

Diagnosis codes on pharmacy claims may not be used.

38. Member-Collected Samples

Test results from member-collected samples may be used for FOBT, urinalysis testing and blood spots for HbA1c, LDL-C, glucose and total cholesterol. Member-collected samples must be sent to the laboratory or provider's office for analysis.

39. Member-Reported Services and Biometric Values

Member-reported services and biometric values (height, weight, BMI percentile) are acceptable only if the information is collected by a primary care practitioner (refer to Appendix 3 for the definition of "PCP") or specialist, if the specialist is providing a primary care service related to the condition being assessed, while taking a patient's history. The information must be recorded, dated and maintained in the member's legal health record.

Note: *It is a "best practice" to collect data directly from members for the Language Diversity of Membership and Race/Ethnicity Diversity of Membership measures; therefore, a PCP or specialist is not required to collect this information as part of patient's history for these measures only.*

HEDIS Coding Conventions

40. Coding Systems Included in HEDIS Reporting

HEDIS measures include codes from the following coding systems:

- CMS Place of Service (POS).
- Current Procedural Terminology (CPT).
- CVX—Vaccines Administered.
- Healthcare Common Procedure Coding System (HCPCS) Level II.
- International Classification of Diseases, 9th Revision, Clinical Modification (ICD-9-CM).

- International Classification of Diseases, 10th Revision, Clinical Modification (ICD-10-CM).
- International Classification of Diseases, 10th Revision, Procedure Coding System (ICD-10-PCS).
- Logical Observation Identifiers Names and Codes (LOINC).
- Systematized Nomenclature of Medicine—Clinical Terms (SNOMED CT).
- Uniform Bill (UB) Revenue and Type of Bill (TOB).

41. Presentation of Codes in HEDIS

HEDIS measure specifications reference value sets and single codes (referred to as "direct reference" codes) that must be used for HEDIS reporting.

Value sets	A value set contains one or more codes that meet criteria for a service or condition. In the specifications, value set references are capitalized and underlined (e.g., Essential Hypertension Value Set). Organizations refer to the Value Set Directory (VSD) for codes in the value sets.
Direct reference codes	A direct reference code is a single code that meets criteria for a service or condition. Direct reference codes are listed in the measure specification and are also included in the Direct Reference Codes spreadsheet of the VSD (as are direct reference codes used for measures reported using ECDS). ***Note:*** *Value sets that contain only one code will be phased out (and turned into direct reference codes) as measures are digitalized.*

Refer to *ECDS Guideline 7* in the *Guidelines for Measures Reported Using Electronic Clinical Data Systems* for information on how codes are presented for measures reported using ECDS.

42. Telehealth

Synchronous telehealth visits, telephone visits and asynchronous telehealth (e-visits, virtual check-ins) are considered separate modalities for HEDIS reporting.

Synchronous telehealth requires real-time interactive audio and video telecommunications. A measure specification that is silent about telehealth includes synchronous telehealth. This is because telehealth is billed using standard CPT and HCPCS codes for professional services in conjunction with a telehealth modifier and/or a telehealth POS code. Therefore, the CPT or HCPCS code in the value set will meet criteria (regardless of whether a telehealth modifier or POS code is present). A measure specification will indicate when synchronous telehealth is not eligible for use and should be excluded.

A measure will indicate when telephone visits are eligible for use by referencing the Telephone Visits Value Set.

Asynchronous telehealth, sometimes referred to as an e-visit or virtual check-in, is not "real-time" but still requires two-way interaction between the member and provider. For example, asynchronous telehealth can occur using a patient portal, secure text messaging or email. A measure will indicate when asynchronous telehealth visits are eligible for use by referencing the Online Assessments Value Set.

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43. Using Claims to Identify Events in Conjunction With Diagnoses or other Events

Many measures' administrative specifications require that a visit code or procedure code be used in conjunction with a diagnosis code. Some measures (e.g., Osteoporosis Management in Women Who Had a Fracture) require that a visit code be used in conjunction with another procedure code (e.g., fracture fixation).

Except for inpatient stays (as described below) and unless noted otherwise in a measure specification, when a measure requires a code be in conjunction with another code the codes must be from the same visit. The organization develops a method for identifying claims from the same visit (e.g., the same outpatient visit, the same inpatient stay). The method is subject to review by the HEDIS auditor.

Identifying acute or nonacute inpatient stays is a two-step process. The first step uses the Inpatient Stay Value Set to identify all acute and nonacute inpatient stays. The second step uses the Nonacute Inpatient Stay Value Set to identify stays that were nonacute. When identifying nonacute codes in step 2, the nonacute code must be on the same claim that was identified in step 1. In addition, any required diagnosis or procedure must be on the same claim.

44. Visits that Result in an Inpatient Stay

Some measures require exclusion of visits that result in an inpatient stay or observation stay.

A visit results in a stay when the visit date of service occurs on the day prior to the admission date or any time during the admission (admission date through discharge date).

45. Principal vs. Secondary Diagnosis

Principal and secondary diagnoses are mentioned throughout HEDIS. Generally, a **principal diagnosis** (or primary diagnosis) is the diagnosis given at discharge and the one listed first on a claim form. A diagnosis listed on a claim or encounter form that is not classified as the principal diagnosis is a **secondary diagnosis.** A claim form can contain multiple secondary diagnoses. Organizations follow the measure specifications to determine whether a diagnosis must be principal or can be secondary. If the specification does not specify that the principal diagnosis must be used, any applicable diagnosis is used.

Some measures require a specific principal diagnosis for eligibility; other measures allow any diagnosis (principal or secondary). For example, Persistence of Beta-Blocker Treatment After a Heart Attack specifies that any diagnosis of an AMI is eligible. If a member's claim lists the principal diagnosis as "severe head injury trauma," but an AMI is listed as a secondary diagnosis on the same claim form, the member is included in the Persistence of Beta-Blocker Treatment After a Heart Attack measure.

The concept of "principal" and "secondary" diagnoses is unique to claims data. Supplemental data (such as EHR data) may not include this concept. Therefore, when using supplemental data to identify a "principal" diagnosis, use any diagnosis.

46. Code Modifiers

Modifiers are two-digit extensions that, when added to CPT or HCPCS codes, provide additional information about a service or procedure. Exclude any CPT Category II code in conjunction with a 1P, 2P, 3P or 8P modifier code (CPT CAT II Modifier Value Set) from HEDIS reporting. These modifiers indicate the service *did not occur*. In the Value Set Directory, CPT Category II codes are identified in the Code System column as "CPT-CAT-II."

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Unless otherwise specified, if a CPT or HCPCS code specified in HEDIS appears in the organization's database with any modifier other than those specified above, the code may be counted in the HEDIS measure.

47. SNOMED Codes

When using SNOMED codes to identify "history of" procedures, the date of the procedure must be available (do not use *the date when the provider documented the procedure* as the date of the procedure).

48. Uniform Bill Code Specificity

Uniform Bill (UB) codes, primarily type of bill and revenue codes, are used to identify services.

The HEDIS Value Set Directory specifies UB type of bill codes using four digits. The organization may also use the equivalent three-digit version of the code (which consists of the four-digit code without the leading zero); for example, to identify skilled nursing facility (SNF) encounters, use either 21x or 021x.

Note: *The three-digit versions of the codes are not included in the Value Set Directory.*

49. Mapping Proprietary or Other Codes

Organizations may only map the following codes for use in HEDIS reporting:

- *State-specific codes.* The organization must provide the auditor with evidence that the codes are required by the state.
- *NDC codes.* An NDC code that is not in the HEDIS Medication List Directory (MLD) can be mapped if its generic name (or brand name), strength/dose and route match those of a code in the MLD. NDC codes that identify immunizations can be mapped to codes in value sets that identify immunizations.
- *RxNorm codes*. An RxNorm code that is not in the HEDIS MLD can only be mapped if its generic name (or brand name), strength/dose and route match those of a code in the MLD.

For audit purposes, the organization documents the method used to map codes. At a minimum, documentation includes a crosswalk containing the relevant codes, descriptions and clinical information.

The organization documents the process for implementing codes. Auditors may request additional information.

50. Retiring Codes

NCQA annually tracks codes that are designated obsolete. NCQA does not remove codes in the year in which they receive the designation of obsolete because of the look-back period in many HEDIS measures. Obsolete codes are deleted from the HEDIS specifications one year after the look-back period is exhausted.

For example, since the Asthma Medication Ratio measure counts a principal diagnosis of asthma in the measurement year or the year prior to the measurement year, asthma codes, for this measure, have a two-year look-back period. A code that is designated obsolete effective January 1, 2021, is deleted from the specifications in HEDIS MY 2023 after the 2-year look-back period (2022, 2023) plus one additional year (2021) is exhausted.

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HEDIS Specification Tables

51. Table Names

Measure specifications contain two types of tables: one to present medication lists and one used by organizations to submit data. Tables use a standardized naming system.

Medication tables

Medication tables are labeled with the corresponding medication list name found in the Medication List Directory.

Reporting tables

Data element tables begin with the measure's three-character abbreviation. Each product line is assigned a number; for example:

- SPC-1 (Medicaid).
- SPC-2 (commercial).
- SPC-3 (Medicare).

If more than one table will be reported for a product line, the table is assigned an uppercase letter. For example, the tables for the Colorectal Cancer Screening measure are COL-A-1, COL-A-2, COL-A-3, COL-B-1/2/3 and COL-C-1/2/3.

52. Reporting Tables

The reporting tables in the measure specifications outline the data elements required for reporting. Refer to *Appendix 4: Data Element Definitions*.

Format

The reporting tables in the measure specifications follow a standard format corresponding to the structure of the IDSS submission XML file:

- *Metric*: For single-metric measures, the metric describes the subject of the measure. For multi-metric measures, the metrics describe the various concepts evaluated in the measure (e.g., Screening, Follow-up, Influenza, Tdap). For wide tables, the metric column may be shown above the table.
- *Stratification*: Only applies to measures that include one or more stratifications (e.g., age, gender). For measures with multiple stratifications, the reporting instructions apply for all stratification combinations.
- *Data Element*: The data elements required for reporting (depending on data collection method).
- *Reporting Instructions*: These instructions specify how the data elements must be reported (e.g., for each metric, repeat per metric), or the units or formula for IDSS calculated data elements.
- *A*: This column is used in hybrid measures to indicate which data elements are required for reporting for the Administrative Method. For the Hybrid Method, all data elements must be reported, unless otherwise specified in the measure specifications.

  For administrative-only measures, all data elements must be reported.

Example Data Elements for Reporting Table

Metric	Stratification1	Stratification2	Data Element	Reporting Instructions	A
Metric1	Level1	Level1	DataElement1	Instruction1	✓
Metric2	Level2	Level2	DataElement2	Instruction2	✓
	Total	Level3	DataElement3	Instruction3	
		Total	DataElement4	Instruction4	
			DataElement5	Instruction5	✓
			DataElement6	Instruction6	
			Rate	Calculation / (Units)	✓

HEDIS measures consist of one-to-many indicators for reporting. Each indicator corresponds to a unique combination of a metric and any stratifications (if applicable). For example, a measure with two metrics, three age stratifications and a total and two gender stratifications and a total consists of twenty-four indicators.

Example:

# of indicators = # of metrics X (# of stratifications 1 + total) X (# of stratifications 2 + total)

Shading

Cells in the data element tables are shaded according to how data are reported:

- *No shading:* Data are reported by the organization.
- *Light gray shading:* Data are calculated by IDSS.
- *Solid black shading:* Data are not used or reported.

Reported by the organization
Calculated by IDSS
Data not used

Measures Reportable With a Partial Year of Data

If an organization licenses a new product or product line during the measurement period, a number of HEDIS measures may be reported with a partial year of data. In general, a measure without a continuous enrollment requirement is reportable with less than a full measurement period of data. The organization must present a written request for approval to the PCS via My NCQA (https://my.ncqa.org).

Guidelines for Calculations and Sampling

This section contains guidelines for calculating rates based on the Administrative and Hybrid Methods, as well as specifications for sampling when using the Hybrid Method. Organizations that use the Hybrid Method must follow the systematic sampling methodology described in this section or receive written authorization from NCQA for an alternative sort or sampling method; written authorization from NCQA is required annually. Proper use and implementation of these methods is assessed as part of NCQA's HEDIS Compliance Audit.

SUMMARY OF CHANGES TO HEDIS MY 2023

- Updated Table 1: Sample Size Information for Hybrid Measures and guidance for reducing sample sizes for measures with multiple stratifications.
- Updated the HEDIS MY 2023 RAND Table for Measures Using the Hybrid Method.
- Removed references to "optional exclusions," including the three approaches to using optional exclusions in data collection.

How to Use the Administrative Method

Step 1 Identify the eligible population and remove all required exclusions. All required exclusions must be removed from the final eligible population.

Step 2 Search administrative systems to identify numerator events for all members in the eligible population.

Step 3 Calculate the rate.

Guidelines for the Hybrid Method

Measures that can be collected using the Hybrid Method are listed in Table 1. Each hybrid measure can be classified into one of the following categories:

- *Membership-dependent denominator*—Defined by membership data only (e.g., women between 24 and 64 years of age, for Cervical Cancer Screening), ***or***
- *Claims-dependent denominator*—Defined by membership and claims data (e.g., members who were diagnosed with hypertension, for Controlling High Blood Pressure).

Drawing the sample prior to the reporting year

Organizations are strongly encouraged to draw samples no earlier than January 2024 for the 2023 measurement year. This increases the accuracy and completeness of the eligible population from which the sample is drawn.

Organizations must adhere to the following guidelines if samples are drawn prior to these dates.

Membership-dependent denominators

The eligible population for the following measures is determined through membership data. Do not draw the sample prior to December 1 of the measurement year.

- Childhood Immunization Status.
- Immunizations for Adolescents.
- Lead Screening in Children.
- Cervical Cancer Screening.
- Colorectal Cancer Screening.
- Care for Older Adults.

An organization that draws its sample on or between December 1 and December 31 of the measurement year must perform the following tasks.

- Oversample to account for individuals included in the sample who were found to be noncompliant with the denominator criteria, subsequent to December 31 of the measurement year.
- On or after December 31 of the measurement year, verify that members included in the sample remain eligible for the particular measure. Another record must be substituted for a member who does not meet all the denominator criteria.
  - For example, for the Childhood Immunization Status measure, on December 5 of the measurement year, an organization draws a sample of children who turn 2 years of age during the measurement year. On or after December 31 of the measurement year, the organization must ensure that all members included in the sample remain eligible for the measure (i.e., meet the continuous enrollment criteria, are members of the organization as of their second birthday).
  - Any ineligible member (i.e., does not meet one or more denominator criteria) must be excluded and replaced by an eligible member from the oversample group.

Claim-dependent denominators

The eligible population for the following measure is determined through membership data and claims data. Do not draw the sample before the end of the measurement year.

- Transition of Care.

To be drawn from a complete eligible population, the sample must be selected no earlier than January of the reporting year. Allow claims incurred through December 31 to be captured in administrative systems before identifying the eligible population and drawing the sample.

The eligible population for the following measures is determined through membership data and claims data. Organizations may draw the sample for these measures as early as December 1 of the measurement year. If an organization draws the sample on or between December 1 and December 31 of the measurement year, it must perform the tasks included in the “Membership-dependent denominators” section above (i.e., oversample as necessary and verify that members remain eligible on or after December 31 of the measurement year):

- Weight Assessment and Counseling for Nutrition and Physical Activity for Children/Adolescents.
- Controlling High Blood Pressure.
- Hemoglobin A1c Control for Patients With Diabetes.
- Blood Pressure Control for Patients With Diabetes.
- Eye Exam for Patients With Diabetes.
- Prenatal and Postpartum Care.

Determining the required sample size

Using the Hybrid Method to collect and report a measure requires a sample to be drawn from the eligible population. Use Table 1 to determine the appropriate sample size for measures. For hybrid measures reported in the prior year, use the last column of Table 1 to determine whether the prior year's audited result can be used to reduce the current year's sample size.

Use Table 2 if the prior year's rate is used to determine the current year's sample. The organization may also use the product line-specific rate derived from administrative data for the current measurement year and Table 2 to reduce the required sample size. The required sample size decreases as the organization's rate improves; for example, the organization calculates a 77% administrative rate for the commercial product line for a new measure and decides to implement the Hybrid Method.

Instead of using a sample size of 411, the organization reduces the sample size for this measure for its commercial product line by using the 77% administrative rate and Table 2. According to Table 2, the minimum required sample size is 296. The sample size can be reduced even when the original eligible member (EM) population is less than 411.

Organization responsibility for chart review

An organization that uses the Hybrid Method for a measure should attempt to pursue charts for all noncompliant members in the systematic sample, to preserve the integrity of the sample and its representative rate. Chart pursuit is recommended but is determined by the organization.

After the systematic sample is generated and chart pursuit has started, the sample may be reduced on rare occasions, such as after a natural disaster. Removing uninvestigated members from the sample in this situation is an alternative sampling method, and the organization must submit a request for approval to PCS via My NCQA (https://my.ncqa.org) that includes the reason for not completing chart review, and the auditor's approval showing that the members to be removed are distributed systematically across the larger sample and the hybrid results from the reduced sample are reportable.

Statistical assumptions for sample size

Sample size is calculated assuming a two-tailed test of significance between two proportions (α = .05, 80% power, two-tailed test of significance). A normal approximation to the binomial with a continuity correction was employed in the sample size calculation. The worst-case assumption of a 50% expected value was assumed.

The detectable difference for most measures is 10 percentage points. This was chosen because it is a big enough difference to be actionable, it is not a burden for data collection and it is not so small as to be "swamped" by nonsampling error.

Table 1: Sample Size Information for Hybrid Measures

Measure	Medicaid	Commercial	Medicare	Prior Year's Rate May Be Used to Reduce MY 2023 Sample Size[1]
Effectiveness of Care				
Weight Assessment and Counseling for Nutrition and Physical Activity for Children/Adolescents	411	411	NA	Y[2,7]
Childhood Immunization Status	411	411	NA	Y[2,7]
Immunizations for Adolescents	411	411	NA	Y[2,7]
Lead Screening in Children	411	NA	NA	Y[3]
Cervical Cancer Screening	411	411	NA	Y[7]
Colorectal Cancer Screening	NA	411	411	Y
Care for Older Adults	NA	NA	411	Y[2,7]
Controlling High Blood Pressure	411	411	411	Y
Hemoglobin A1c Control for Patients With Diabetes	411	411	411	Y[4,7]
Eye Exam for Patients With Diabetes	411	411	411	Y[4]
Blood Pressure Control for Patients With Diabetes	411	411	411	Y[4]
Transitions of Care	NA	NA	411	Y[5,7]
Access/Availability of Care				
Prenatal and Postpartum Care	411	411	NA	Y[2,7]

[1] Refer to *Table 2: Sample Sizes When Data Are Available on the Product Line Being Measured* in this section to determine the minimum required sample size.

[2] If reducing the sample size based on the current year's administrative rate or the prior year's product line-specific rate for this measure, the lowest rate from all the indicators must be used.

[3] If the same sample from the CIS measure is used for the LSC measure, the organization may reduce the sample size based on the current year's administrative rate or prior year's audited, product line-specific rate for the lowest rate of all CIS antigens, CIS combinations and LSC rate. If a separate sample from the CIS measure is used for the LSC measure, the organization can reduce the sample based on the product-line specific current measurement year's administrative rate or the prior year's reported rate for LSC.

[4] If the same sample is used for the three diabetes measures, the organization must first take the inverse of the HbA1c poor control >9.0% rate (100 minus the HbA1c poor control rate) and then reduce using the lowest rate among all the reported indicators of the three diabetes measures (the HbA1c control [<8.0%], HbA1c poor control [>9.0%] indicators of the HBD measure, the EED measure and the BPD measure). If separate samples are used for these measures, organizations may reduce the sample based on the product line-specific current measurement year's administrative rate or the prior year's audited, product line-specific rate for the measure.

[5] Organizations may reduce the sample size based only on the prior year's audited, product line-specific rate for the lowest rate of all TRC indicators. The current year's administrative rate may not be used for sample size reduction.

[6] Organizations may only use the current year's administrative rate when reducing the sample size.

[7] For measures with stratifications, use the total rate for reducing sample sizes. For measures with multiple indicators and stratifications, use the lowest total rate across indicators when reducing sample sizes.

Organizations may use a rate calculated from the current year's administrative rate or the prior year's reported rate to determine the sample size. Table 1: Sample Size Information for Hybrid Measures must be used first to determine if a prior year's rate can be used to reduce the sample size for a particular measure.

Table 2: Sample Sizes When Data Are Available on the Product Line Being Measured

If the Current Year's Administrative Rate or the Prior Year's Reported Rate Is…	…the Minimum Sample Size Is:	If the Current Year's Administrative Rate or the Prior Year's Reported Rate Is…	…the Minimum Sample Size Is:
≤51%	411	74%	321
52%	410	75%	313
53%	410	76%	305
54%	409	77%	296
55%	407	78%	288
56%	405	79%	279
57%	403	80%	270
58%	401	81%	260
59%	398	82%	250
60%	395	83%	240
61%	392	84%	229
62%	388	85%	219
63%	384	86%	207
64%	380	87%	196
65%	376	88%	184
66%	371	89%	172
67%	366	90%	159
68%	360	91%	147
69%	354	92%	134
70%	348	93%	120
71%	342	94%	106
72%	335	≥95%	100
73%	328		

Note

- *Table 2 reflects the MRSS. When reducing, an organization's sample size may be between the allowed minimum sample size in Table 2 and 411.*
  - *Organizations that report using socioeconomic status (SES) categories must use the total for sample size reduction, not the cohort rates based on SES stratification.*
- *Truncate the decimal portion of the rate to obtain a whole number.*

Systematic Sampling Methodology

NCQA implemented a systematic sampling methodology for the Hybrid Method. Proper use and implementation of this method ensures ongoing integrity of collected data and supports increasing requests for audited data. Complete the following steps for each hybrid measure.

Step 1 Determine the EM population. Develop a list of EMs, including full name (last, first), date of birth and event (if applicable). An organization that reports on combined HMO/POS or HMO/POS/PPO/EPO products must include all EMs from all products.

Step 2 Determine the MRSS from Table 1 or Table 2. This number becomes the denominator for the measure. Use either Table 1 or Table 2, as appropriate, to determine the MRSS. (Refer to *Determining the required sample size* for instructions.) If the EM is ≤MRSS, proceed to step 4.

To use a larger MRSS, an organization must provide written rationale to NCQA through PCS via My NCQA (https://my.ncqa.org).

Step 3 Determine the oversample. The oversample should be an adequate number of additional records to make substitutions. Oversample only enough to guarantee that the MRSS is met; keep substitution criteria in mind.

Written approval from NCQA must be obtained to use an oversampling rate larger than 20%. Refer to *Oversample requests to NCQA* for details.

The oversample records should be used, and reported, only to replace cases taken out of the MRSS because of valid data errors, false positives and so on; otherwise, these records should not be reported on in the final denominator.

Step 4

- If EM ≤MRSS, all eligible members are included in the sample. The MRSS must be reported as the EM.
- If EM >MRSS + all oversample records, go to step 5.
- If MRSS <EM ≤MRSS + all oversample records, proceed to step 8.

Step 5 Sort the list of EMs in alphabetical order (by applicable measurement year) by last name, first name, date of birth and event (if applicable). If the organization reports on combined products, it must alphabetize the combined EM population.

Sort EMs from A to Z in even measurement years and from Z to A in odd measurement years.

For example, for HEDIS MY 2023, sort the list of EMs from Z to A. For HEDIS MY 2024, sort the list from A to Z.

Note: *Sort order applies to all components. For HEDIS MY 2023, sort all fields by descending order (i.e., last name descending, first name descending, date of birth descending, event descending).*

Step 6 Calculate N = EM/(MRSS + all oversample records). Round *down* to a whole number.

Determine N, which is used in the formula to determine which member will start your sample. N is calculated using the equation:

$$N = EM/(MRSS + \text{all oversample records})$$

where EM = the eligible member population (step 1) and MRSS = the minimum required sample size (step 2).

Step 7 Calculate START = (RAND × N). Before choosing members, determine the member to start with (START). It is important that the sample be selected from a single pass through the member list. START can have many values and still allow only one pass.

Use the Random Number (RAND) table for the appropriate measurement year that lists a value between 0 and 1 for each measure where the Hybrid Method is applicable. Refer to this table to determine the RAND to be used when determining START. The RAND for each measure is used to calculate the starting point from which to draw the final sample.

Calculate the number from which to start drawing the final sample as follows:

START = (RAND × N)

(round per the .5 rule to the nearest whole number greater than 0), where RAND = the random number for each respective measure identified in the RAND table.

Step 8 Select the sample, choosing every i^{th} member using the formula:

i^{th} member = START + [(i-1) x EM/(MRSS + all oversample records)]

(rounding [(i-1) x (EM/MRSS + all oversample records)] per the .5 rule to the nearest whole number greater than 0).

For i = 2,3,4, …, MRSS where EM = the eligible member population (step 1). MRSS = the minimum required sample size (step 2).

Starting with the member corresponding to the number START, choose every i^{th} member until the MRSS is met. This becomes the primary list of sampled members.

Continue choosing every i^{th} member until the oversample is met. This set of members becomes the oversample. The oversample records should be used and reported only to replace cases taken out of the MRSS because of valid data errors, false positives and so on; otherwise, these records should not be reported in the final denominator.

Note: *From step 4, if MRSS <EM ≤MRSS + all oversample records, sort the EMs in alphabetical order (by applicable measurement year) by last name, first name, date of birth and event (if applicable). Choose the first MRSS EMs as the primary sample and the remaining EMs as the oversample.*

The oversample list is only used to replace exclusions. All exclusions must be documented because they may be subject to audit.

Oversample requests to NCQA

Any oversampling rate larger than 20% must be approved by NCQA annually. Organizations submit a formal request with the rationale to NCQA through PCS via My NCQA (https://my.ncqa.org).

NCQA provides written notification of approval or disapproval within 7 business days. The organization must maintain the documentation for the HEDIS Compliance Audit.

Oversampling methodology

For hybrid measures, the starting sample size must be higher than the designated sample size because medical records must be substituted if a member is ineligible for the measure; for example, if a member was incorrectly identified as a diabetic through administrative data or meets exclusion criteria for the measure.

To adjust for this, divide the sample size by the percentage of charts expected to be inappropriate for review. Suppose 10% of charts are expected to be inappropriate for the measure.

To determine the oversample, multiply the MRSS by the oversample percentage and round up to the nearest whole number.

411 x 0.10 = 41.1
(rounded up to 42 = oversample).

The recommended methodology for substitution is:

- Replace the member's chart with that of the first member in the oversample list.
- Continue replacing each ineligible member with the next consecutive member of the oversample list.

If the initial oversample was underestimated and all oversample members have been exhausted without satisfying the MRSS, the organization must contact NCQA through PCS via My NCQA (https://my.ncqa.org) to determine next steps.

Organizations must only use the oversample for substitution and must report all measures using their MRSS.

Note: *Many factors must be considered when determining the initial sample size and oversampling percentage—such as previous years' data, frequency of exclusions and claims lag.*

HEDIS MY 2023 RAND Table for Measures Using the Hybrid Method

Measure	RAND
Weight Assessment and Counseling for Nutrition and Physical Activity for Children/Adolescents	.68
Childhood Immunization Status *and* Lead Screening in Children*	.27
Immunizations for Adolescents	.85
Cervical Cancer Screening	.25
Colorectal Cancer Screening	.35
Care for Older Adults	.06
Controlling High Blood Pressure	.39
Hemoglobin A1c Control for Patients With Diabetes, Blood Pressure Control for Patients With Diabetes *and* Eye Exam for Patients With Diabetes	.95**
Transitions of Care	.91
Prenatal and Postpartum Care	.47

* The RANDs for these measures are the same. Organizations may choose to use the same sample for the two measures. If organizations use different samples for these measures, a different MRSS is used in the sampling protocol.

** The RANDs for these measures are the same. Organizations may choose to use the same sample for these three measures. If organizations use different samples for these measures, a different MRSS is used in the sampling protocol.

Example 1

The eligible population for the commercial product line for Immunizations for Adolescents is 9,000. Reduce the minimum required sample size using the commercial rate from the prior year's HEDIS submission, which was 77%. Based on experience, estimate a 5% oversample rate. Follow the systematic sampling methodology.

Step 1 EM = 9,000.

Step 2 From Table 2, the MRSS is 296.

Step 3 Oversample = 296 × .05 = 14.8 (the next whole number *above* is 15, so oversample = 15).

Step 4 Because 9,000 > 296 (MRSS) and 311 (296 + oversample), go to step 5.

Step 5 Sort the list alphabetically and in this order: last name, first name, date of birth.

Step 6 N = 9,000/311 (MRSS + oversample) = 28.

Step 7 For this example, assume that RAND = 0.66, so START = 0.66 x 28 = 18.48.

- Rounding using the .5 rule, START = 18.
- The 18th sorted member is chosen *first.*
- The 2nd member chosen is the 18 + [(2-1) x (9,000/311)] = 18 + 29 = 47th sorted member, after rounding the term [(2-1) x (9,000/311)] to 29, using the .5 rule.
- The 3rd member chosen is the 18 + [(3-1) x (9,000/311)] = 18 + 58 = 76th sorted member.
- The 296th member (the last one in the primary list) is the 18 + [(296-1) x (9,000/311)] = 18 + 8,537 = 8,555th sorted member.
- The last member in the oversample* is the 18 + [(311-1) x (9,000/311)] = 18 + 8,971 = 8,989th sorted member.

**Remember, members in the oversample are used only to replace members excluded from the sample.*

Example 2

The eligible member population for Colorectal Cancer Screening is 389. This measure was not collected last year, nor will the administrative rate from this year be used to reduce the sample size. Follow the systematic sampling methodology.

Step 1 EM = 389.

Step 2 From Table 1, the MRSS is 411. Since 389 <411, skip to step 4.

Step 3 *Skip this step.*

Step 4 Include all 389 members in your primary list.

Example 3

The eligible member population for Childhood Immunization Status is 436. The sample size will not be adjusted using this year's administrative rate. Based on experience with this population, about 10% of the members from the primary sample will have to be excluded. Follow the systematic sampling methodology.

Step 1 EM = 436.

Step 2 From Table 1, the MRSS is 411.

Step 3 Oversample = 411 x .10 = 41.1 (the next whole number *above* is 42, so oversample = 42).

Step 4 Because 411 <436 ≤ (411 + 42), skip to step 8.

Step 5 *Skip this step.*

Step 6 *Skip this step.*

Step 7 *Skip this step.*

Step 8 Sort the list and choose the first 411 as the primary list. The remaining 25 members become the oversample list*.

**Remember, members in the oversample are used only to replace members excluded from the sample.*

Example 4

The eligible member population for the commercial product line for Cervical Cancer Screening is 400. Reduce the minimum required sample size using the commercial rate from the prior year's HEDIS submission, which was 62%. Based on experience, estimate a 5% oversample rate. Follow the systematic sampling methodology.

Step 1 EM = 400.

Step 2 From Table 2, the MRSS is 388.

Step 3 Oversample = 388 x .05 = 19.4 (the next whole number *above* is 20, so oversample = 20).

Step 4 Because 388 < 400 ≤ (388 + 20), skip to step 8.

Step 5 *Skip this step.*

Step 6 *Skip this step.*

Step 7 *Skip this step.*

Step 8 Sort the list and choose the first 388 as the primary list. The remaining 12 members become the oversample list*.

**Remember, members in the oversample are used only to replace members excluded from the sample.*

Complex Probability Sampling

Organization responsibility

Properly applied, other techniques (e.g., stratified sampling, cluster sampling and other complex probability approaches) can improve precision and increase sampling efficiency. To use a probability sampling approach different from the one specified, submit a written rationale and documentation of the approach to NCQA through PCS via My NCQA (https://my.ncqa.org). The organization must demonstrate that the sampling approach is auditable and does not introduce bias against specific members. A committee of statisticians and health policy experts staffed by NCQA reviews the approach. Written notification of NCQA approval or disapproval is provided within 10 business days.

If complex sampling methods are used, report the estimated rate, in addition to any information required to perform a valid test of significance between that rate and another organization's rate.

Report the sample size (if different from the HEDIS recommendation) and document the method used in the calculation (including software used, if applicable). Consult a statistician before implementing a complex sampling methodology.

Substituting Medical Records

Acceptable circumstances for substitution:

Organizations must specify the number of substituted records. Members who are noncompliant because they refused the service or because the organization cannot access their chart may not be substituted. Unless otherwise noted in the specifications for a particular measure, members or events may not be dropped from the sample or substituted, except under the three circumstances described below.

1. Errors in sampling data

Chart review reveals that a member or event does not meet the eligibility criteria for inclusion in the sample. Data errors can be caused by incorrect member or clinical information. Examples of valid data errors:

- A member selected for the Childhood Immunization Status sample is found to be 22 years old.
- A member in the Eye Exam for Patients With Diabetes sample has a diagnosis in the chart showing that a prescription for oral hypoglycemics was not related to diabetes.
- A member in the sample for any measure has a notation entered by the deadline established for the measure, explaining the reason for the erroneous inclusion and stating the member does not have the condition.

The medical record must have evidence that a member does not meet the criteria for the measure. A chart that does not contain a notation that substantiates or refutes the diagnosis is not evidence that the member does not have the condition being measured.

Members may also be identified as valid data errors if administrative data refresh finds they meet exclusion criteria. Report these members as valid data errors.

2. Employee/ dependent was selected for the sample	An employee of the organization or the vendor, or the employee's dependent, was selected for the sample, and the medical record must be reviewed to determine compliance with the measure. The organization or vendor may exclude employees and their dependents in this situation *only*. Employee and employee dependents are not excluded from administrative reporting and should not be removed before the sample is drawn.

References

Deming, W.E. On the interpretation of censuses as samples. 1941. *Journal of the American Statistical Association.* 36: 45–9.

Fleiss, L. *Statistical Methods for Rates and Proportions.* 2nd Ed. (New York: John Wiley & Sons, Inc.): 38–42.

Guidelines for the Rules for Allowable Adjustments of HEDIS

SUMMARY OF CHANGES TO HEDIS MY 2023

- Updated *Guideline 4: Exclusions* to remove optional exclusions.
- Updated *Guideline 7: Measurement Period Adjustments*.
- Revised *Guideline 12: Measures With Special Considerations for Allowable Adjustments* to remove first-year measures from the list of measures that cannot be adjusted.
- Updated *Guideline 13: Allowable Adjustments to Risk Adjusted Utilization Measures* to allow modification of the measurement period for only 1 year.

About the Rules for Allowable Adjustments of HEDIS

The "Rules for Allowable Adjustments of HEDIS" (the "Rules") describe how NCQA's HEDIS measure specifications can be adjusted for other populations, if applicable. The Rules, reviewed and approved by NCQA measure experts, provide for expanded use of HEDIS measures without changing their clinical intent.

Note: *Measures that do not include Rules for Allowable Adjustments may not be adjusted.*

Overview of Rules for Allowable Adjustments

A HEDIS measure specification consists of defined sets and subsets of criteria needed to identify and calculate the concept being measured. At a basic level, a measure includes a description of services counted (the numerator), the definition of an eligible population (the denominator), a list of exclusions, coding or value sets for each factor, reporting instructions and calculation logic. Each measure component is subject to assessment of importance (evidence), reliability, validity, feasibility and usability during development and is monitored over time. Adjustments to any measure component may pose threats to the methodological soundness and evidence alignment of a measure.

HEDIS measures were originally developed, tested and specified at the health plan level. These Rules specify how HEDIS measures may be adapted for use outside the traditional health plan setting for different purposes and levels of analysis (e.g., population health management, quality improvement) and still maintain their clinical intent. The Rules also note where measure specifications may not be adjusted to ensure they maintain alignment with the intended specifications and related clinical guidelines.

Referring to Adjusted HEDIS Measures and Rates

Any measure may be explained in terms of the components needed to produce it, specifically for collecting, calculating and reporting. Only when a measure is used as prescribed by its HEDIS specifications can it be used for auditing and reporting to NCQA and other NCQA-approved programs (e.g., state Medicaid or commercial reports).

Calculated HEDIS measure results ("rates") from *adjusted* HEDIS measures may *not* be used for HEDIS reporting and comparison. Additionally, any HEDIS measure rates, including those from adjusted HEDIS measures, may not be reported or displayed externally without first obtaining an appropriate license with, or permission from, NCQA.

Further, the Rules ensure that adjusted measures and resulting rates can maintain the HEDIS name:

- All HEDIS measures adjusted according to the Rules should be referred to as **"Adjusted HEDIS measures."**
- A calculated measure result rate from a HEDIS measure that has not been certified via NCQA's Measure Certification Program, and is based on *adjusted* HEDIS specifications, may not be called an **"Adjusted HEDIS rate"** until it is audited and designated reportable by an NCQA-Certified HEDIS Compliance Auditor. Until such time, such measure rates shall be designated or referred to as **"Adjusted, Uncertified, Unaudited HEDIS rates"** and may only be used for internal, quality improvement purposes (e.g., trend analysis) and no incentive payments may be made on such rates.
- A calculated measure rate from a HEDIS measure that has been certified via NCQA's Measure Certification Program, and is based on *adjusted* HEDIS specifications, may not be called an **"Adjusted HEDIS rate"** until it is audited and designated reportable by an NCQA-Certified HEDIS Compliance Auditor. Until such time, applicable measure rates shall be designated or referred to as **"Adjusted, Unaudited HEDIS rates."**

Considerations and Guidelines for the Rules for Allowable Adjustments of HEDIS

Organizations must consider all HEDIS General Guidelines and the specific guidelines below when using the Rules. The following are guidelines about measure criteria that may be adjusted; however, each measure that is allowed to be adjusted must be considered individually with respect to national clinical guidelines.

1. Allowable Adjustment Measure Structure

The Rules for Allowable Adjustments of HEDIS tables in applicable measures consist of both clinical and nonclinical components:

1. *Nonclinical components*
   - Nonclinical eligible population criteria:
     - Product lines.
     - Benefits.
     - Continuous enrollment.
     - Anchor dates.
     - Allowed gaps.
2. *Clinical components*
   - Clinical eligible population (event/diagnosis) criteria:
     - Exclusion criteria.
     - Numerator criteria.
     - Value sets.
     - Medication lists.

Each measure in this volume that is allowed to be adjusted has a section titled *Rules for Allowable Adjustments of HEDIS,* which includes a table that identifies components that may be adjusted without changing the measure's intention or underlying clinical logic.

2. Non-Adjustable Measure Components Steps

To protect the integrity of the measure and ensure alignment with clinical guidelines, the following components may not be changed:

- *Event/Diagnosis:* In determining the eligible population, the clinical criteria (value sets, medication lists [if applicable] and logic) may not be changed.
- *Numerators:* The value sets, medication lists (if applicable) and associated logic may not be changed.
- *Hybrid Method:* The Rules do not apply to the hybrid portion of the measure; only the administrative sections may be changed.

Note: *In general, organizations may not adjust any clinical logic to identify the denominator or the numerator; however, narrowing the population to a subset is acceptable. For example, organizations may choose to assess children with only one or two vaccines already administered, for the Childhood Immunization Status measure, or assess children at 1 year of age to determine if lead screening has been performed, for the Lead Screening in Children measure. Using the clinical criteria, but looking at an appropriate subset, is allowed.*

3. Stratifications for Telehealth Services

A measure that allows the use of synchronous telehealth visits, telephone visits and asynchronous telehealth (e-visits, virtual check-ins) may be stratified to identify services performed via telehealth.

Note: *This adjustment is not allowed for event/diagnosis, numerators and exclusions that do not allow the use of telehealth.*

4. Exclusions

Required exclusions: For measures with required exclusions, exclusions must be applied as written as part of identifying the denominator.

Exclusions for hospice, deceased members, palliative care, I-SNP, LTI, advanced illness and/or frailty: The palliative care, advanced illness, frailty and long-term nursing home residence exclusions are specified in HEDIS measures where the services being captured may not be of benefit for this population or may not be in line with patients' goals of care. These exclusions may be removed, but organizations should first consider why they apply to HEDIS measures. If organizations choose to apply these exclusions, they must be applied as written in the specifications.

Note: *For Risk Adjusted Utilization measures, hospice, deceased members, palliative care, I-SNP, LTI, advanced illness and/or frailty exclusions (where applicable) must be applied to the Rules for Allowable Adjustments for Risk-Adjusted Measurement, but may be removed from the Rules for Allowable Adjustments for Observed Measurement for Risk Adjusted Utilization measures.*

5. Age Adjustments

For the majority of HEDIS measures, NCQA specifies age ranges based on current clinical guidelines and recommendations for the specific treatment or condition. These recommendations often align with U.S. Preventive Services Task Force (USPSTF) Preventive Care Recommendations or other guidelines. For more information about the clinical guideline or evidence basis for each measure, refer to *HEDIS Volume 1: Narrative.*

Determining age ranges for any measure is based on the best available clinical evidence and is intended to promote standards of care and accuracy in interpreting evidence. For allowed adjustments, organizations may change determination dates (e.g., age 18 by December 31 of the measurement year); but to maintain alignment with clinical guidelines and the approved measure specifications, adjustments to age ranges outside of those stated in the HEDIS specifications may be made only if specified in a measure's Rules for Allowable Adjustments.

An organization may choose different age groups within a specified age range or remove the age stratification within a measure but should not adjust the actual age range limits unless otherwise specified.

For measures that include medications, the organization should account for FDA labels when considering an age adjustment.

When changing a measurement period, an organization may also affect the age determination date (e.g., in Appropriate Testing for Pharyngitis, the age determination day is July 1). Changing the measurement period or its determination date can affect other date-dependent events. Organizations should account for the impact of any change.

6. Supplemental Data

Supplemental data may be used to identify eligible population, exclusion and numerator events. In general, the data are usable when the codes are in the value sets and the clinical intent of the measure is not changed, but there are restrictions for use of supplemental data:

- For eligible population calculation, the files may be used if they meet all measure requirements.
- For numerator calculation, the files may be used if they meet all measure requirements, even if the organization cannot determine payment status.

7. Measurement Period Adjustments

HEDIS recognizes the need to identify and address care gaps, and allows adjustments to the measurement period; however, changes to this parameter may affect the logic in the event/diagnosis determination and the numerator events. Changes must consider relative timing.

If organizations make changes to the measurement period, results should be used with caution. For Risk Adjusted Utilization measures, organizations may only change the measurement period and associated classification periods by 1 year.

8. Timing Relationships

Measures that use events with relative timing (e.g., events tied to a specific episode of care) include specific guidance for allowed changes. Some measures allow adjustments to measure elements, but the order and relationship of dependent events may not be modified (e.g., the duration of a negative medication history after an episode). Changing the relationship of events would change the intent of the measure and is not allowed.

9. Measures With Multiple Rates

For measures with multiple rates (indicators), organizations may calculate a single rate or a combination of rates. For example, an organization may choose to calculate only the BMI Percentile rate within the Weight Assessment and Counseling for Nutrition and Physical Activity for Children/Adolescents measure.

Note: *Organizations should consider whether the rates in a measure are dependent on each other. For example, for the Statin Therapy for Patients With Cardiovascular Disease measure, the denominator for the Rate 2—Statin Adherence 80% indicator relies on the numerator for Rate 1—Received Statin Therapy indicator. These indicators should not be calculated separately.*

10. Denominator Size

Organizations should assess reliability when the denominator is less than 30 for all measures except Utilization and Risk Adjusted Utilization measures. For utilization measures that count member months, organizations should assess reliability when the denominator is less than 360 member months. For Risk-Adjusted Utilization measures, organizations should assess reliability when the denominator is less than 150.

11. Measure Trending

NCQA encourages consistent adjustments and assessments of measures to trend performance over time. Organizations that use Allowable Adjustments should use caution when trending across multiple years.

12. Measures With Special Considerations for Allowable Adjustments

The Rules for Allowable Adjustments do not apply and were not added to:

- Survey measures.
- Health Plan Descriptive Information measures.

13. Allowable Adjustments for Risk Adjusted Utilization Measures

The measures under the Risk Adjusted Utilization domain allow two types of Rules for Allowable Adjustments sections:

1. **Rules for Allowable Adjustments for Risk-Adjusted Measurement.** This section must be followed for adjustments when calculating the full measure specifications, which includes all risk adjusted rates (Expected Rates, Risk Adjustment, Observed-to-Expected) and calculation components (Count of Index Stays, Observed Events). This includes all risk weighting from the *Guidelines for Risk Adjusted Utilization Measures.*
2. **Rules for Allowable Adjustments for Observed Measurement** (rates without risk adjustment). This section must be followed when adjusting the calculation of Observed Events only. When applying these adjustments, organizations must not include risk adjustment logic.

The intent of including the two different types of Rules for Allowable Adjustments is to allow organizations to adjust measures without compromising the measures' validity. Risk adjustment is based on statistical prediction models that are specifically calibrated for each measure.

- Organizations must follow the guidance of the *Rules for Allowable Adjustments for Risk-Adjusted Measurement* when risk adjusting measures.
- Organizations must follow the *Rules for Allowable Adjustments for Observed Measurement* when adjusting the measures to assess observed events.
  - For example, an organization must use the *Rules for Allowable Adjustments for Observed Measurement* if it is identifying all observed admissions and readmissions for the PCR measure without applying risk adjustment.
- Organizations may only change the measurement period for risk adjustment classification periods by 1 year.

14. Attribution Methods

NCQA does not specify, test or approve any attribution methods at this time. The organization defines how it attributes patient services to clinicians, practices or medical groups, including for PCPs and specialists. Organizations often use different methods for attributing care to clinicians for cost, resource use or utilization analyses and for quality analyses. For quality of care attribution, methods may differ between PCPs and specialists.

Effectiveness of Care

Guidelines for Effectiveness of Care Measures

SUMMARY OF CHANGES TO HEDIS MY 2023

- Removed references to optional exclusions.
- Removed the guideline for SES Stratification; this text has been moved to the *General Guidelines for Data Collection and Reporting* as *General Guideline 32: Medicare Socioeconomic Status Stratification*.

Guidelines

Which services count?

Unless otherwise specified in a measure, report all services for the Effectiveness of Care (EOC) measures, whether or not the organization paid for them. For example, report services paid for by a third party, such as a community center; or services for which payment was denied because they were not properly authorized.

The organization must include all paid, suspended, pending and denied claims, and is ultimately responsible for the quality of care it provides to members.

Organizations may choose whether to include reversed claims when reporting services. If an organization includes reversals, it must include these claims in all measures and avoid double counting services (e.g., if a subsequent claim is filed, use only the corrected or adjudicated claim).

Note:

- *Denied claims are not included when identifying numerator events, but must be used to determine the eligible population (if applicable) for the following measures in the Overuse/Appropriateness subdomain:*
  - *Non-Recommended Cervical Cancer Screening in Adolescent Females.*
  - *Non-Recommended PSA-Based Screening in Older Men.*
  - *Appropriate Treatment for Upper Respiratory Infection.*
  - *Avoidance of Antibiotic Treatment for Acute Bronchitis/Bronchiolitis.*
  - *Use of Imaging Studies for Low Back Pain.*
  - *Potentially Harmful Drug-Disease Interactions in Older Adults.*
  - *Use of High-Risk Medications in Older Adults.*
  - *Deprescribing of Benzodiazepines in Older Adults.*
- *Denied claims are not included when identifying the eligible population (except for required exclusions) or numerator events for the following measures in the Overuse/Appropriateness subdomain:*
  - *Use of Opioids at High Dosage.*
  - *Use of Opioids from Multiple Providers.*
  - *Risk of Continued Opioid Use.*
- *Organizations must include all claims (paid, suspended, pending and denied) for required exclusions in all measures in the Overuse/Appropriateness subdomain.*

Measure format

There are 10 possible sections in each measure specification in this domain:

1. Summary of Changes.
2. Description.
3. Calculation.
4. Definitions.
5. Eligible Population.
6. Administrative Specification.
7. Hybrid Specification.
8. Notes.
9. Data Elements for Reporting.
10. Rules for Allowable Adjustments of HEDIS.

Eligible population criteria

The **eligible population** includes all members who meet the following seven criteria:

1. **Product line** (commercial, Medicaid, Medicare) applicable to the measure.
2. **Age** group and gender requirements.
3. **Continuous enrollment** criteria for the measure.
4. **Allowable gap** in benefits during the continuous enrollment period. There are different allowable gap criteria for the Medicaid product line.
5. **Anchor date** specifies the required enrollment date for the eligible population (e.g., children must be enrolled in the organization on their second birthday for inclusion in the Childhood Immunization Status measure).
6. **Benefit** a member must have during the continuous enrollment period to be included in the eligible population (e.g., members must have both medical and pharmacy benefits for inclusion in the Antidepressant Medication Management measure).
7. **Event/diagnosis** specifies the medical event or diagnosis requirements for the eligible population (e.g., members must have a diagnosis of AMI for inclusion in the Persistence of Beta-Blocker Treatment After a Heart Attack measure).

Administrative Specification

The **Administrative Specification** outlines the collection and calculation of a measure using only administrative data and describes the eligible population and the numerator requirements allowed for the measure.

Hybrid Specification

The **Hybrid Specification** includes sampling requirements for the denominator population and medical record documentation requirements for the numerator allowed for the measure.

Colorectal Cancer Screening (COL)

SUMMARY OF CHANGES TO HEDIS MY 2023

- Revised the optional exclusions for colorectal cancer and total colectomy to be required exclusions.
- Added a required exclusion for members who died during the measurement year.
- Updated the number of occurrences required for the frailty cross-cutting exclusion.
- Added a direct reference code for palliative care.
- Updated the Hybrid Specification to indicate that sample size reduction is allowed.
- Revised the medical record criteria for a completed colonoscopy.
- Revised the "Other" criteria in the Nonclinical Components table under *Rules for Allowable Adjustments of HEDIS.*
- Revised the "Exclusions" criteria in the Clinical Components table under *Rules for Allowable Adjustments of HEDIS*.

Description

The percentage of members 45–75 years of age who had appropriate screening for colorectal cancer.

Note

Only the administrative data collection method may be used when reporting this measure for the Medicaid product line.

Eligible Population

Product lines

Commercial, Medicaid, Medicare (report each product line separately).

Stratifications

For Medicare only, report the following SES stratifications and total:

- Non-LIS/DE, Nondisability.
- LIS/DE.
- Disability.
- LIS/DE and Disability.
- Other.
- Unknown.
- Total Medicare.

Note: *Stratifications are mutually exclusive and the sum of all six stratifications is the total population.*

For each product line, report the following stratifications by race and total, and stratifications by ethnicity and total:

- *Race:*
  - White.
  - Black or African American.
  - American Indian or Alaska Native.

- Asian.
- Native Hawaiian or Other Pacific Islander.
- Some Other Race.
- Two or More Races.
- Asked but No Answer.
- Unknown.
- Total.

- *Ethnicity:*
  - Hispanic or Latino.
  - Not Hispanic or Latino.
  - Asked but No Answer.
  - Unknown.
  - Total.

Note: *Stratifications are mutually exclusive and the sum of all categories in each stratification is the total population.*

Ages

46–75 years as of December 31 of the measurement year. Report two age stratifications and a total rate:

- 46–49 years.
- 50–75 years.
- Total.

The total is the sum of the age stratifications.

Continuous enrollment

The measurement year and the year prior to the measurement year.

Allowable gap

No more than one gap in continuous enrollment of up to 45 days during each year of continuous enrollment.

Anchor date

December 31 of the measurement year.

Benefit

Medical.

Event/diagnosis

None.

Required exclusions

Exclude members who meet any of the following criteria:

- Members who had colorectal cancer (Colorectal Cancer Value Set) or a total colectomy (Total Colectomy Value Set; History of Total Colectomy Value Set) any time during the member's history through December 31 of the measurement year.
- Members in hospice or using hospice services anytime during the measurement year. Refer to *General Guideline 15: Members in Hospice*.
- Members who died any time during the measurement year. Refer to *General Guideline 16: Deceased Members.*
- Members receiving palliative care (Palliative Care Assessment Value Set; Palliative Care Encounter Value Set; Palliative Care Intervention Value Set; ICD-10-CM code Z51.5) any time during the measurement year.

Exclusions

Exclude members who meet any of the following criteria:

Note: *Supplemental and medical record data may not be used for these exclusions.*

- Medicare members 66 years of age and older as of December 31 of the measurement year who meet either of the following:
  - Enrolled in an Institutional SNP (I-SNP) any time during the measurement year.
  - Living long-term in an institution any time during the measurement year as identified by the LTI flag in the Monthly Membership Detail Data File. Use the run date of the file to determine if a member had an LTI flag during the measurement year.
- Members 66 years of age and older as of December 31 of the measurement year (all product lines) with frailty ***and*** advanced illness. Members must meet **BOTH** of the following frailty and advanced illness criteria to be excluded:
  1. At least two indications of frailty (Frailty Device Value Set; Frailty Diagnosis Value Set; Frailty Encounter Value Set; Frailty Symptom Value Set) with different dates of service during the measurement year.
  2. Any of the following during the measurement year or the year prior to the measurement year (count services that occur over both years):
     - At least two outpatient visits (Outpatient Value Set), observation visits (Observation Value Set), ED visits (ED Value Set), telephone visits (Telephone Visits Value Set), e-visits or virtual check-ins (Online Assessments Value Set), nonacute inpatient encounters (Nonacute Inpatient Value Set) or nonacute inpatient discharges (instructions below; the diagnosis must be on the discharge claim) on different dates of service, with an advanced illness diagnosis (Advanced Illness Value Set). Visit type need not be the same for the two visits. To identify a nonacute inpatient discharge:
       1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
       2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set) on the claim.
       3. Identify the discharge date for the stay.
     - At least one acute inpatient encounter (Acute Inpatient Value Set) with an advanced illness diagnosis (Advanced Illness Value Set).
     - At least one acute inpatient discharge with an advanced illness diagnosis (Advanced Illness Value Set) on the discharge claim. To identify an acute inpatient discharge:
       1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
       2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
       3. Identify the discharge date for the stay.
     - A dispensed dementia medication (Dementia Medications List).

Dementia Medications

Description	Prescription
Cholinesterase inhibitors	• Donepezil • Galantamine • Rivastigmine
Miscellaneous central nervous system agents	• Memantine
Dementia combinations	• Donepezil-memantine

Administrative Specification

Denominator The eligible population.

Numerator One or more screenings for colorectal cancer. Any of the following meet criteria:

- Fecal occult blood test (FOBT Lab Test Value Set; FOBT Test Result or Finding Value Set) during the measurement year. For administrative data, assume the required number of samples were returned, regardless of FOBT type.
- Flexible sigmoidoscopy (Flexible Sigmoidoscopy Value Set; History of Flexible Sigmoidoscopy Value Set) during the measurement year or the 4 years prior to the measurement year.
- Colonoscopy (Colonoscopy Value Set; History of Colonoscopy Value Set) during the measurement year or the 9 years prior to the measurement year.
- CT colonography (CT Colonography Value Set) during the measurement year or the 4 years prior to the measurement year.
- Stool DNA (sDNA) with FIT test (sDNA FIT Lab Test Value Set; sDNA FIT Test Result or Finding Value Set) during the measurement year or the 2 years prior to the measurement year.

Hybrid Specification

Denominator A systematic sample drawn from the eligible population for the Medicare and commercial product lines. Organizations may reduce the sample size using the current year's administrative rate or the prior year's audited, product line-specific rate. Refer to the *Guidelines for Calculations and Sampling* for information on reducing the sample size.

For Medicare reporting, the denominator (MRSS) for the Total category is the entire systematic sample. Do not pull samples for each stratification. The individual stratifications for the denominators and all numerators must sum to the totals.

Numerator One or more screenings for colorectal cancer. Appropriate screenings are defined by one of the following:

- FOBT during the measurement year.
- Flexible sigmoidoscopy during the measurement year or the 4 years prior to the measurement year.
- Colonoscopy during the measurement year or the 9 years prior to the measurement year.

- CT colonography during the measurement year or the 4 years prior to the measurement year.
- Stool DNA (sDNA) with FIT test during the measurement year or the 2 years prior to the measurement year.

Administrative Refer to *Administrative Specification* to identify positive numerator hits from the administrative data.

Medical record Documentation in the medical record must include a note indicating the date when the colorectal cancer screening was performed. A result is not required if the documentation is clearly part of the member's "medical history"; if this is not clear, the result or finding must also be present (this ensures that the screening was performed and not merely ordered).

A pathology report that indicates the type of screening (e.g., colonoscopy, flexible sigmoidoscopy) and the date when the screening was performed meets criteria.

For pathology reports that do not indicate the type of screening and for incomplete procedures:

- Evidence that the scope advanced to the cecum meets criteria for a completed colonoscopy.
- Evidence that the scope advanced into the sigmoid colon meets criteria for a completed flexible sigmoidoscopy.

There are two types of FOBT tests: guaiac (gFOBT) and immunochemical (FIT). Depending on the type of FOBT test, a certain number of samples are required for numerator compliance. Follow the instructions below to determine member compliance.

- If the medical record does not indicate the type of test and there is no indication of how many samples were returned, assume the required number was returned. The member meets the screening criteria for inclusion in the numerator.
- If the medical record does not indicate the type of test and the number of returned samples is specified, the member meets the screening criteria only if the number of samples specified is greater than or equal to three samples. If there are fewer than three samples, the member does not meet the screening criteria for inclusion.
- FIT tests may require fewer than three samples. If the medical record indicates that an FIT was done, the member meets the screening criteria, regardless of how many samples were returned.
- If the medical record indicates that a gFOBT was done, follow the scenarios below.
  - *If the medical record does not indicate the number of returned samples,* assume the required number was returned. The member meets the screening criteria for inclusion in the numerator.
  - *If the medical record indicates that three or more samples were returned,* the member meets the screening criteria for inclusion in the numerator.
  - *If the medical record indicates that fewer than three samples were returned,* the member does not meet the screening criteria.

Do not count digital rectal exams (DRE), FOBT tests performed in an office setting or performed on a sample collected via DRE.

Data Elements for Reporting

Organizations that submit HEDIS data to NCQA must provide the following data elements.

Table COL-A-1: Data Elements for Colorectal Cancer Screening

Metric	Age	Data Element	Reporting Instructions
ColorectalCancerScreening	46-49	EligiblePopulation	For each Stratification
	50-75	ExclusionAdminRequired	For each Stratification
	Total	NumeratorByAdmin	For each Stratification
		NumeratorBySupplemental	For each Stratification
		Rate	(Percent)

Table COL-A-2: Data Elements for Colorectal Cancer Screening

Metric	Age	Data Element	Reporting Instructions	A
ColorectalCancerScreening	46-49	CollectionMethod	Repeat per Stratification	✓
	50-75	EligiblePopulation	For each Stratification	✓
	Total	ExclusionAdminRequired	For each Stratification	✓
		NumeratorByAdminElig	For each Stratification	
		CYAR	Only for Total (Percent)	
		MinReqSampleSize	Repeat per Stratification	
		OversampleRate	Repeat per Stratification	
		OversampleRecordsNumber	(Count)	
		ExclusionValidDataErrors	Repeat per Stratification	
		ExclusionEmployeeOrDep	Repeat per Stratification	
		OversampleRecsAdded	Repeat per Stratification	
		Denominator	For each Stratification	
		NumeratorByAdmin	For each Stratification	✓
		NumeratorByMedicalRecords	For each Stratification	
		NumeratorBySupplemental	For each Stratification	✓
		Rate	(Percent)	✓

Table COL-A-3: Data Elements for Colorectal Cancer Screening

Metric	Age	SES Stratification	Data Element	Reporting Instructions	A
ColorectalCancerScreening	46-49	NonLisDeNondisability	CollectionMethod	Repeat per Stratification	✓
	50-75	LisDe	EligiblePopulation	For each Stratification	✓
	Total	Disability	ExclusionAdminRequired	For each Stratification	✓
		LisDeAndDisability	NumeratorByAdminElig	For each Stratification	
		Other	CYAR	Only for Total (Percent)	
		Unknown	MinReqSampleSize	Repeat per Stratification	
		Total	OversampleRate	Repeat per Stratification	
			OversampleRecordsNumber	(Count)	
			ExclusionValidDataErrors	Repeat per Stratification	
			ExclusionEmployeeOrDep	Repeat per Stratification	
			OversampleRecsAdded	Repeat per Stratification	
			Denominator	For each Stratification	
			NumeratorByAdmin	For each Stratification	✓
			NumeratorByMedicalRecords	For each Stratification	
			NumeratorBySupplemental	For each Stratification	✓
			Rate	(Percent)	✓

Table COL-B-1/2/3: Data Elements for Colorectal Cancer Screening: Stratifications by Race

Metric	Race	Source	Data Element	Reporting Instructions	A
ColorectalCancerScreening	White	Direct	CollectionMethod***	Repeat per Stratification	✓
	BlackOrAfricanAmerican	Indirect	EligiblePopulation	For each Stratification	✓
	AmericanIndianOrAlaskaNative	Total	Denominator***	For each Stratification	
	Asian		Numerator	For each Stratification	✓
	NativeHawaiianOrOtherPacificIslander		Rate	(Percent)	✓
	SomeOtherRace				
	TwoOrMoreRaces				
	AskedButNoAnswer*				
	Unknown**				

Table COL-C-1/2/3: Data Elements for Colorectal Cancer Screening: Stratifications by Ethnicity

Metric	Ethnicity	Source	Data Element	Reporting Instructions	A
ColorectalCancerScreening	HispanicOrLatino	Direct	CollectionMethod***	Repeat per Stratification	✓
	NotHispanicOrLatino	Indirect	EligiblePopulation	For each Stratification	✓
	AskedButNoAnswer*	Total	Denominator***	For each Stratification	
	Unknown**		Numerator	For each Stratification	✓
			Rate	(Percent)	✓

*AskedButNoAnswer is only reported for Source='Direct.'
**Unknown is only reported for Source='Indirect.'
***The CollectionMethod and Denominator data elements are not available for Medicaid reporting.

Rules for Allowable Adjustments of HEDIS

The "Rules for Allowable Adjustments of HEDIS" (the "Rules") describe how NCQA's HEDIS measure specifications can be adjusted for other populations, if applicable. The Rules, reviewed and approved by NCQA measure experts, provide for expanded use of HEDIS measures without changing their clinical intent.

Adjusted HEDIS measures *may not* be used for HEDIS health plan reporting.

Rules for Allowable Adjustments of Colorectal Cancer Screening

NONCLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Product lines	Yes	Organizations are not required to use product line criteria; product lines may be combined and all (or no) product line criteria may be used.
Ages	Yes, with limits	The age determination dates may be changed (e.g., select, "age as of June 30"). The denominator age may not be expanded.
Continuous enrollment, allowable gap, anchor date	Yes	Organizations are not required to use enrollment criteria; adjustments are allowed.
Benefit	Yes	Organizations are not required to use a benefit; adjustments are allowed.
Other	Yes	Organizations may use additional eligible population criteria to focus on an area of interest defined by gender, race, ethnicity, socioeconomic or sociodemographic characteristics, geographic region or another characteristic.
CLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Event/diagnosis	NA	There is no event/diagnosis for this measure.
Denominator Exclusions	**Adjustments Allowed (Yes/No)**	**Notes**
Required exclusions	Yes, with limits	Apply required exclusions according to specified value sets. The hospice, deceased member and palliative care exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments*
Exclusions: I-SNP, LTI, frailty or advanced illness	Yes	These exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments*.
Numerator Criteria	**Adjustments Allowed (Yes/No)**	**Notes**
Colorectal Cancer Screening	No	The value sets and the logic may not be changed.

Care for Older Adults (COA)

SUMMARY OF CHANGES TO HEDIS MY 2023

- Added a required exclusion for members who died during the measurement year.
- Revised the "Other" criteria in the Nonclinical Components table under *Rules for Allowable Adjustments of HEDIS.*
- Revised the "Required exclusions" criteria in the Clinical Components table under *Rules for Allowable Adjustments of HEDIS*.

Description

The percentage of adults 66 years of age and older who had each of the following during the measurement year:

- Medication Review.
- Functional Status Assessment.
- Pain Assessment.

Definitions

Medication list	A list of the member's medications in the medical record. The medication list may include medication names only or may include medication names, dosages and frequency, over-the-counter (OTC) medications and herbal or supplemental therapies.
Medication review	A review of all a member's medications, including prescription medications, OTC medications and herbal or supplemental therapies.
Standardized tool	A set of structured questions that elicit member information. May include person-reported outcome measures, screening or assessment tools or standardized questionnaires developed by the health plan to assess risks and needs.

Eligible Population

Product line	Medicare (only SNP and MMP benefit packages).
Ages	66 years and older as of December 31 of the measurement year.
Continuous enrollment	The measurement year.
Allowable gap	No more than one gap in continuous enrollment of up to 45 days during the measurement year.
Anchor date	December 31 of the measurement year.
Benefit	Medical.
Event/diagnosis	None.

Required exclusions

Exclude members who meet either of the following criteria:

- Members in hospice or using hospice services any time during the measurement year. Refer to *General Guideline 15: Members in Hospice*.
- Members who died any time during the measurement year. Refer to *General Guideline 16: Deceased Members*.

Administrative Specification

Denominator

The eligible population.

Numerators

Medication Review

Either of the following meets criteria.

- Both of the following during the same visit during the measurement year where the provider type is a prescribing practitioner or clinical pharmacist:
  - At least one medication review (Medication Review Value Set).
  - The presence of a medication list in the medical record (Medication List Value Set).
- Transitional care management services (Transitional Care Management Services Value Set) during the measurement year.

Exclude services provided in an acute inpatient setting (Acute Inpatient Value Set; Acute Inpatient POS Value Set).

Functional Status Assessment

At least one functional status assessment (Functional Status Assessment Value Set) during the measurement year. Exclude services provided in an acute inpatient setting (Acute Inpatient Value Set; Acute Inpatient POS Value Set).

Pain Assessment

At least one pain assessment (Pain Assessment Value Set) during the measurement year. Exclude services provided in an acute inpatient setting (Acute Inpatient Value Set; Acute Inpatient POS Value Set).

Hybrid Specification

Denominator

A systematic sample drawn from the eligible population. Organizations may reduce the sample size using the current year's administrative rate or the prior year's audited, product line-specific rate. Refer to the *Guidelines for Calculations and Sampling* for information on reducing the sample size.

Numerators

Medication Review

At least one medication review conducted by a prescribing practitioner or clinical pharmacist during the measurement year ***and*** the presence of a medication list in the medical record, as documented through either administrative data or medical record review.

A medication list, signed and dated during the measurement year by the appropriate practitioner type (prescribing practitioner or clinical pharmacist), meets criteria (the practitioner's signature is considered evidence that the medications were reviewed).

Administrative Refer to *Administrative Specification* to identify positive numerator hits from administrative data.

Medical record Documentation must come from the same medical record and must include one of the following:

- A medication list in the medical record ***and*** evidence of a medication review by a prescribing practitioner or clinical pharmacist and the date when it was performed.
- Notation that the member is not taking any medication and the date when it was noted.

A review of side effects for a single medication at the time of prescription alone is not sufficient. An outpatient visit is not required to meet criteria. Do not include medication lists or medication reviews performed in an acute inpatient setting.

Functional Status Assessment At least one functional status assessment during the measurement year, as documented through either administrative data or medical record review.

Administrative Refer to *Administrative Specification* to identify positive numerator hits from administrative data.

Medical record Documentation in the medical record must include evidence of a complete functional status assessment and the date when it was performed.

Notations for a complete functional status assessment must include one of the following:

- Notation that Activities of Daily Living (ADL) were assessed or that at least five of the following were assessed: bathing, dressing, eating, transferring [e.g., getting in and out of chairs], using toilet, walking.
- Notation that Instrumental Activities of Daily Living (IADL) were assessed or at least four of the following were assessed: shopping for groceries, driving or using public transportation, using the telephone, cooking or meal preparation, housework, home repair, laundry, taking medications, handling finances.
- Result of assessment using a standardized functional status assessment tool, not limited to:
  - SF-36®.
  - Assessment of Living Skills and Resources (ALSAR).
  - Barthel ADL Index Physical Self-Maintenance (ADLS) Scale.
  - Bayer ADL (B-ADL) Scale.
  - Barthel Index.
  - Edmonton Frail Scale.
  - Extended ADL (EADL) Scale.
  - Groningen Frailty Index.
  - Independent Living Scale (ILS).
  - Katz Index of Independence in ADL.
  - Kenny Self-Care Evaluation.
  - Klein-Bell ADL Scale.

- Kohlman Evaluation of Living Skills (KELS).
- Lawton & Brody's IADL scales.
- Patient Reported Outcome Measurement Information System (PROMIS) Global or Physical Function Scales.

A functional status assessment limited to an acute or single condition, event or body system (e.g., lower back, leg) does not meet criteria for a comprehensive functional status assessment. The components of the functional status assessment numerator may take place during separate visits within the measurement year. Do not include comprehensive functional status assessments performed in an acute inpatient setting.

Pain Assessment At least one pain assessment during the measurement year, as documented through either administrative data or medical record review.

Administrative Refer to *Administrative Specification* to identify positive numerator hits from administrative data.

Medical record Documentation in the medical record must include evidence of a pain assessment and the date when it was performed.

Notations for a pain assessment must include one of the following:

- Documentation that the patient was assessed for pain (which may include positive or negative findings for pain).
- Result of assessment using a standardized pain assessment tool, not limited to:
  - Numeric rating scales (verbal or written).
  - Face, Legs, Activity, Cry Consolability (FLACC) scale.
  - Verbal descriptor scales (5–7 Word Scales, Present Pain Inventory).
  - Pain Thermometer.
  - Pictorial Pain Scales (Faces Pain Scale, Wong-Baker Pain Scale).
  - Visual analogue scale.
  - Brief Pain Inventory.
  - Chronic Pain Grade.
  - PROMIS Pain Intensity Scale.
  - Pain Assessment in Advanced Dementia (PAINAD) Scale.

Do not include pain assessments performed in an acute inpatient setting.

Note

- *Notation alone of a pain management plan does not meet criteria.*
- *Notation alone of a pain treatment plan does not meet criteria.*
- *Notation alone of screening for chest pain or documentation alone of chest pain does not meet criteria.*
- *Refer to Appendix 3 for the definition of* clinical pharmacist *and* prescribing practitioner*.*
- *A medication review performed without the member present meets criteria.*
- *The Functional Status Assessment and Pain Assessment indicators do not require a specific setting; therefore, services rendered during a telephone visit, e-visit or virtual check-in meet criteria.*

Data Elements for Reporting

Organizations that submit HEDIS data to NCQA must provide the following data elements.

Table COA-3: Data Elements for Care for Older Adults

Metric	Data Element	Reporting Instructions	A
MedicationReview	CollectionMethod	For each Metric	✓
FunctionalStatusAssessment	EligiblePopulation*	For each Metric	✓
PainAssessment	ExclusionAdminRequired*	For each Metric	✓
	NumeratorByAdminElig	For each Metric	
	CYAR	(Percent)	
	MinReqSampleSize	Repeat per Metric	
	OversampleRate	Repeat per Metric	
	OversampleRecordsNumber	(Count)	
	ExclusionValidDataErrors	Repeat per Metric	
	ExclusionEmployeeOrDep	Repeat per Metric	
	OversampleRecsAdded	Repeat per Metric	
	Denominator	Repeat per Metric	
	NumeratorByAdmin	For each Metric	✓
	NumeratorByMedicalRecords	For each Metric	
	NumeratorBySupplemental	For each Metric	✓
	Rate	(Percent)	✓

*Repeat the EligiblePopulation and ExclusionAdminRequired values for metrics using the Administrative Method.

Rules for Allowable Adjustments of HEDIS

The "Rules for Allowable Adjustments of HEDIS" (the "Rules") describe how NCQA's HEDIS measure specifications can be adjusted for other populations, if applicable. The Rules, reviewed and approved by NCQA measure experts, provide for expanded use of HEDIS measures without changing their clinical intent.

Adjusted HEDIS measures *may not* be used for HEDIS health plan reporting.

Rules for Allowable Adjustments of Care for Older Adults

NONCLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Product lines	Yes	Organizations are not required to use product line criteria; product lines may be combined and all (or no) product line criteria may be used.
Ages	Yes, with limits	Age determination dates may be changed (e.g., select, "age as of June 30"). The denominator age for all 3 indicators may be changed within the specified age range (66 years and older). The denominator age for the Medication Review, Functional Status Assessment, and Pain Assessment indicators may be expanded to 18 years of age and older only for dual-eligible members and Medicaid LTSS members.
Continuous enrollment, allowable gap, anchor date	Yes	Organizations are not required to use enrollment criteria; adjustments are allowed.
Benefit	Yes	Organizations are not required to use a benefit; adjustments are allowed.
Other	Yes	Organizations may use additional eligible population criteria to focus on an area of interest defined by gender, race, ethnicity, socioeconomic or sociodemographic characteristics, geographic region or another characteristic.
CLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Event/diagnosis	NA	There is no event/diagnosis for this measure.
Denominator Exclusions	**Adjustments Allowed (Yes/No)**	**Notes**
Required exclusions	Yes	The hospice and deceased member exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments*.
Numerator Criteria	**Adjustments Allowed (Yes/No)**	**Notes**
• Medication Review • Functional Status Assessment • Pain Assessment	No	Value sets and logic may not be changed.

Cardiovascular Conditions

Controlling High Blood Pressure (CBP)

SUMMARY OF CHANGES TO HEDIS MY 2023

1. Added a required exclusion for members who died during the measurement year.
2. Replaced the reference of “female members” to “members” in the required exclusions.
3. Added a direct reference code for palliative care.
4. Revised the optional exclusions to be required exclusions.
5. Updated the number of occurrences required for the frailty cross-cutting exclusion.
6. Revised the “Other” criteria in the Nonclinical Components table under *Rules for Allowable Adjustments of HEDIS.*
7. Revised the “Exclusions” criteria in the Clinical Components table under *Rules for Allowable Adjustments of HEDIS*.

Description

The percentage of members 18–85 years of age who had a diagnosis of hypertension (HTN) and whose blood pressure (BP) was adequately controlled (<140/90 mm Hg) during the measurement year.

Definitions

Adequate control Both a representative systolic BP <140 mm Hg and a representative diastolic BP of <90 mm Hg.

Representative BP The most recent BP reading during the measurement year on or after the second diagnosis of hypertension. If multiple BP measurements occur on the same date, or are noted in the chart on the same date, use the lowest systolic and lowest diastolic BP reading. If no BP is recorded during the measurement year, assume that the member is “not controlled.”

Eligible Population

Product lines Commercial, Medicaid, Medicare (report each product line separately).

Stratifications For each product line, report the following stratifications by race and total, and stratifications by ethnicity and total:

1. *Race:*
   - White.
   - Black or African American.
   - American Indian or Alaska Native.
   - Asian.
   - Native Hawaiian or Other Pacific Islander.
   - Some Other Race.
   - Two or More Races.
   - Asked but No Answer.
   - Unknown.

- Total.

2. *Ethnicity:*
   - Hispanic or Latino.
   - Not Hispanic or Latino.
   - Asked but No Answer.
   - Unknown.
   - Total.

Note: *Stratifications are mutually exclusive and the sum of all categories in each stratification is the total population.*

Ages 18–85 years as of December 31 of the measurement year.

Continuous enrollment The measurement year.

Allowable gap No more than one gap in continuous enrollment of up to 45 days during the measurement year. To determine continuous enrollment for a Medicaid beneficiary for whom enrollment is verified monthly, the member may not have more than a 1-month gap in coverage (e.g., a member whose coverage lapses for 2 months [60 days] is not considered continuously enrolled).

Anchor date December 31 of the measurement year.

Benefit Medical.

Event/diagnosis Follow the steps below to identify the eligible population.

Step 1 Identify members who had at least two visits on different dates of service with a diagnosis of hypertension on or between January 1 of the year prior to the measurement year and June 30 of the measurement year. Visit type need not be the same for the two visits. Any of the following code combinations meet criteria:

- Outpatient visit (Outpatient Without UBREV Value Set) with any diagnosis of hypertension (Essential Hypertension Value Set).
- A telephone visit (Telephone Visits Value Set) with any diagnosis of hypertension (Essential Hypertension Value Set).
- An e-visit or virtual check-in (Online Assessments Value Set) with any diagnosis of hypertension (Essential Hypertension Value Set).

Step 2 Remove members who had a nonacute inpatient admission during the measurement year. To identify nonacute inpatient admissions:

1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set) on the claim.
3. Identify the admission date for the stay.

Required exclusions Exclude members who meet any of the following criteria:

3. Members in hospice or using hospice services any time during the measurement year. Refer to *General Guideline 15: Members in Hospice.*
4. Members who died any time during the measurement year. Refer to *General Guideline 16: Deceased Members.*

5. Members receiving palliative care (Palliative Care Assessment Value Set; Palliative Care Encounter Value Set; Palliative Care Intervention Value Set; ICD-10-CM code Z51.5) any time during the measurement year.
6. Members with evidence of end-stage renal disease (ESRD) (ESRD Diagnosis Value Set), dialysis (Dialysis Procedure Value Set), nephrectomy (Total Nephrectomy Value Set; Partial Nephrectomy Value Set) or kidney transplant (Kidney Transplant Value Set; History of Kidney Transplant Value Set) any time during the member's history on or prior to December 31 of the measurement year.
7. Members with a diagnosis of pregnancy (Pregnancy Value Set) any time during the measurement year.

Exclusions

Exclude members who meet any of the following criteria:

Note: *Supplemental and medical record data may not be used for these exclusions.*

8. Medicare members 66 years of age and older as of December 31 of the measurement year who meet either of the following:
   - Enrolled in an Institutional SNP (I-SNP) any time during the measurement year.
   - Living long-term in an institution any time during the measurement year as identified by the LTI flag in the Monthly Membership Detail Data File. Use the run date of the file to determine if a member had an LTI flag during the measurement year.
9. Members 66–80 years of age as of December 31 of the measurement year (all product lines) with frailty ***and*** advanced illness. Members must meet *both* of the following frailty and advanced illness criteria to be excluded:
   1. At least two indications of frailty (Frailty Device Value Set; Frailty Diagnosis Value Set; Frailty Encounter Value Set; Frailty Symptom Value Set) with different dates of service during the measurement year.
   2. Any of the following during the measurement year or the year prior to the measurement year (count services that occur over both years):
      - At least two outpatient visits (Outpatient Value Set), observation visits (Observation Value Set), ED visits (ED Value Set), telephone visits (Telephone Visits Value Set), e-visits or virtual check-ins (Online Assessments Value Set), nonacute inpatient encounters (Nonacute Inpatient Value Set) or nonacute inpatient discharges (instructions below; the diagnosis must be on the discharge claim) on different dates of service, with an advanced illness diagnosis (Advanced Illness Value Set). Visit type need not be the same for the two visits. To identify a nonacute inpatient discharge:
        1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
        2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set) on the claim.
        3. Identify the discharge date for the stay.
      - At least one acute inpatient encounter (Acute Inpatient Value Set) with an advanced illness diagnosis (Advanced Illness Value Set).
      - At least one acute inpatient discharge with an advanced illness diagnosis (Advanced Illness Value Set) on the discharge claim. To identify an acute inpatient discharge:

1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
3. Identify the discharge date for the stay.

- A dispensed dementia medication (Dementia Medications List).

10. Members 81 years of age and older as of December 31 of the measurement year (all product lines) with at least two indications of frailty (Frailty Device Value Set; Frailty Diagnosis Value Set; Frailty Encounter Value Set; Frailty Symptom Value Set) with different dates of service during the measurement year.

Dementia Medications

Description	Prescription
Cholinesterase inhibitors	1. Donepezil 2. Galantamine 3. Rivastigmine
Miscellaneous central nervous system agents	4. Memantine
Dementia combinations	5. Donepezil-memantine

Administrative Specification

Denominator The eligible population.

Numerator Identify the most recent BP reading (Systolic Blood Pressure Value Set; Diastolic Blood Pressure Value Set) taken during the measurement year. Exclude BPs taken in an acute inpatient setting (Acute Inpatient Value Set; Acute Inpatient POS Value Set) or during an ED visit (ED Value Set; ED POS Value Set).

The BP reading must occur *on or after* the date of the second diagnosis of hypertension (identified using the event/diagnosis criteria).

The member is numerator compliant if the BP is <140/90 mm Hg. The member is not compliant if the BP is ≥140/90 mm Hg, if there is no BP reading during the measurement year or if the reading is incomplete (e.g., the systolic or diastolic level is missing). If there are multiple BPs on the same date of service, use the lowest systolic and lowest diastolic BP on that date as the representative BP.

Organizations that use CPT Category II codes to identify numerator compliance for this indicator must search for all codes in the following value sets and use the most recent codes during the measurement year to determine numerator compliance for both systolic and diastolic levels.

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Value Set	Numerator Compliance
Systolic Less Than 140 Value Set	Systolic compliant
Systolic Greater Than or Equal To 140 Value Set	Systolic not compliant
Diastolic Less Than 80 Value Set	Diastolic compliant
Diastolic 80–89 Value Set	Diastolic compliant
Diastolic Greater Than or Equal To 90 Value Set	Diastolic not compliant

Hybrid Specification

Denominator

A systematic sample drawn from the eligible population.

The organization may reduce the sample size using the current year's administrative rate or the prior year's audited, product line specific rate. Refer to the *Guidelines for Calculations and Sampling* for information on reducing the sample size.

Identifying the medical record

All eligible BP measurements recorded in the record must be considered. If an organization cannot find the medical record, the member remains in the measure denominator and is considered noncompliant for the numerator.

Use the following guidance to find the appropriate medical record to review.

11. Identify the member's PCP.
12. If the member had more than one PCP for the time period, identify the PCP who most recently provided care to the member.
13. If the member did not visit a PCP for the time period or does not have a PCP, identify the practitioner who most recently provided care to the member.
14. If a practitioner other than the member's PCP manages the hypertension, the organization may use the medical record of that practitioner.

Numerator

The number of members in the denominator whose most recent BP (both systolic and diastolic) is adequately controlled during the measurement year. For a member's BP to be controlled, the systolic and diastolic BP must be <140/90 mm Hg (adequate control). To determine if a member's BP is adequately controlled, the representative BP must be identified.

Administrative

Refer to *Administrative Specification* to identify positive numerator hits from administrative data.

Medical record

Identify the most recent BP reading noted during the measurement year.

The BP reading must occur on or after the date when the second diagnosis of hypertension (identified using the event/diagnosis criteria) occurred.

Do not include BP readings:

15. Taken during an acute inpatient stay or an ED visit.
16. Taken on the same day as a diagnostic test or diagnostic or therapeutic procedure that requires a change in diet or change in medication on or one day before the day of the test or procedure, with the exception of fasting blood tests.

17. Taken by the member using a non-digital device such as with a manual blood pressure cuff and a stethoscope.

Identify the lowest systolic and lowest diastolic BP reading from the most recent BP notation in the medical record. If multiple readings were recorded for a single date, use the lowest systolic and lowest diastolic BP on that date as the representative BP. The systolic and diastolic results do not need to be from the same reading.

BP readings taken by the member and documented in the member's medical record are eligible for use in reporting (provided the BP does not meet any exclusion criteria). There is no requirement that there be evidence the BP was collected by a PCP or specialist.

The member is not compliant if the BP reading is ≥140/90 mm Hg or is missing, or if there is no BP reading during the measurement year or if the reading is incomplete (e.g., the systolic or diastolic level is missing).

Ranges and thresholds do not meet criteria for this measure. A distinct numeric result for both the systolic and diastolic BP reading is required for numerator compliance. A BP documented as an "average BP" (e.g., "average BP: 139/70") is eligible for use.

Note

8. *When identifying the most recent BP reading, all eligible BP readings in the appropriate medical record should be considered, regardless of practitioner type and setting (excluding acute inpatient and ED visit settings).*
9. *An EMR can be used to identify the most recent BP reading if it meets the criteria for appropriate medical record.*
10. *When excluding BP readings from the numerator, the intent is to identify diagnostic or therapeutic procedures that require a medication regimen, a change in diet or a change in medication. For example (this list is for reference only and is not exhaustive):*
    - *A colonoscopy requires a change in diet (NPO on the day of the procedure) and a medication change (a medication is taken to prep the colon).*
    - *Dialysis, infusions and chemotherapy (including oral chemotherapy) are all therapeutic procedures that require a medication regimen.*
    - *A nebulizer treatment with albuterol is considered a therapeutic procedure that requires a medication regimen (the albuterol).*
    - *A patient forgetting to take regular medications on the day of the procedure is not considered a required change in medication and therefore the BP reading is eligible.*
11. *BP readings taken on the same day that the member receives a common low-intensity or preventive procedure are eligible for use. For example, the following procedures are considered common low-intensity or preventive (this list is for reference only and is not exhaustive):*
    - *Vaccinations.*
    - *Injections (e.g., allergy, vitamin B-12, insulin, steroid, toradol, Depo-Provera, testosterone, lidocaine).*
    - *TB test.*
    - *IUD insertion.*
    - *Eye exam with dilating agents.*
    - *Wart or mole removal.*

Data Elements for Reporting

Organizations that submit HEDIS data to NCQA must provide the following data elements.

Table CBP-A-1/2/3: Data Elements for Controlling High Blood Pressure

Metric	Data Element	Reporting Instructions	A
ControlHighBP	CollectionMethod	Report once	✓
	EligiblePopulation	Report once	✓
	ExclusionAdminRequired	Report once	✓
	NumeratorByAdminElig	Report once	
	CYAR	(Percent)	
	MinReqSampleSize	Report once	
	OversampleRate	Report once	
	OversampleRecordsNumber	(Count)	
	ExclusionValidDataErrors	Report once	
	ExclusionEmployeeOrDep	Report once	
	OversampleRecsAdded	Report once	
	Denominator	Report once	
	NumeratorByAdmin	Report once	✓
	NumeratorByMedicalRecords	Report once	
	NumeratorBySupplemental	Report once	✓
	Rate	(Percent)	✓

Table CBP-B-1/2/3: Data Elements for Controlling High Blood Pressure: Stratifications by Race

Metric	Race	Source	Data Element	Reporting Instructions	A
ControlHighBP	White	Direct	CollectionMethod	Repeat per Stratification	✓
	BlackOrAfricanAmerican	Indirect	EligiblePopulation	For each Stratification	✓
	AmericanIndianOrAlaskaNative	Total	Denominator	For each Stratification	
	Asian		Numerator	For each Stratification	✓
	NativeHawaiianOrOtherPacificIslander		Rate	(Percent)	✓
	SomeOtherRace				
	TwoOrMoreRaces				
	AskedButNoAnswer*				
	Unknown**				

Table CBP-C-1/2/3: Data Elements for Controlling High Blood Pressure: Stratifications by Ethnicity

Metric	Ethnicity	Source	Data Element	Reporting Instructions	A
ControlHighBP	HispanicOrLatino	Direct	CollectionMethod	Repeat per Stratification	✓
	NotHispanicOrLatino	Indirect	EligiblePopulation	For each Stratification	✓
	AskedButNoAnswer*	Total	Denominator	For each Stratification	
	Unknown**		Numerator	For each Stratification	✓
			Rate	(Percent)	✓

*AskedButNoAnswer is only reported for Source='Direct.'
**Unknown is only reported for Source='Indirect.'

Rules for Allowable Adjustments of HEDIS

The "Rules for Allowable Adjustments of HEDIS" (the "Rules") describe how NCQA's HEDIS measure specifications can be adjusted for other populations, if applicable. The Rules, reviewed and approved by NCQA measure experts, provide for expanded use of HEDIS measures without changing their clinical intent.

Adjusted HEDIS measures *may not* be used for HEDIS health plan reporting.

Rules for Allowable Adjustments of Controlling High Blood Pressure

NONCLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Product lines	Yes	Using product line criteria is not required. Including any product line, combining product lines or not including product line criteria is allowed.
Ages	Yes, with limits	Age determination dates may be changed (e.g., select, "age as of June 30"). The denominator age may be changed if the range is within the specified age range (ages 18–85 years). The denominator age may not be expanded.
Continuous enrollment, allowable gap, anchor date	Yes	Organizations are not required to use enrollment criteria; adjustments are allowed.
Benefit	Yes	Organizations are not required to use enrollment criteria; adjustments are allowed.
Other	Yes	Organizations may use additional eligible population criteria to focus on an area of interest defined by gender, race, ethnicity, socioeconomic or sociodemographic characteristics, geographic region or another characteristic.
CLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Event/diagnosis	No	Only events that contain (or map to) codes in the value sets may be used to identify visits. Value sets and logic may not be changed.
Denominator Exclusions	**Adjustments Allowed (Yes/No)**	**Notes**
Required exclusions	Yes, with limits	Apply required exclusions according to specified value sets. The hospice, deceased member and palliative care exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments.*
Exclusions: I-SNP, LTI, frailty or advanced illness	Yes	These exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments*.
Numerator Criteria	**Adjustments Allowed (Yes/No)**	**Notes**
Adequate Control of Blood Pressure	No	Value sets and logic may not be changed.

Statin Therapy for Patients With Cardiovascular Disease (SPC)

SUMMARY OF CHANGES TO HEDIS MY 2023

12. Clarified in the "Event/diagnosis" criteria that required exclusions are not a step.
13. Replaced the reference to "female members" with "members" in the pregnancy required exclusion.
14. Added a required exclusion for members who died during the measurement year.
15. Added a direct reference code for palliative care.
16. Updated the number of occurrences required for the frailty cross-cutting exclusion.
17. Revised the "Other" criteria in the Nonclinical Components table under *Rules for Allowable Adjustments of HEDIS.*
18. Revised the "Required exclusions" criteria in the Clinical Components table under *Rules for Allowable Adjustments of HEDIS*.

Description

The percentage of males 21–75 years of age and females 40–75 years of age during the measurement year who were identified as having clinical atherosclerotic cardiovascular disease (ASCVD) and met the following criteria. The following rates are reported:

19. *Received Statin Therapy.* Members who were dispensed at least one high-intensity or moderate-intensity statin medication during the measurement year.
20. *Statin Adherence 80%.* Members who remained on a high-intensity or moderate-intensity statin medication for at least 80% of the treatment period.

Definitions

IPSD	Index prescription start date. The earliest prescription dispensing date for any statin medication of at least moderate intensity during the measurement year.
Treatment period	The period of time beginning on the IPSD through the last day of the measurement year.
PDC	Proportion of days covered. The number of days the member is covered by at least one statin medication prescription of appropriate intensity, divided by the number of days in the treatment period.
Calculating number of days covered for multiple prescriptions	If multiple prescriptions for different medications are dispensed on the same day, calculate the number of days covered by a statin medication (for the numerator) using the prescriptions with the longest days supply. For multiple different prescriptions dispensed on different days with overlapping days supply, count each day in the treatment period only once toward the numerator. If multiple prescriptions for the same medication are dispensed on the same day or on different days, sum the days supply and use the total to calculate the number of days covered by a statin medication (for the numerator). For example, three prescriptions for the same medication are dispensed on the same day, each with a 30-days supply. Sum the days supply for a total of 90

days covered by a statin. Subtract any days supply that extends beyond December 31 of the measurement year.

Use the medication lists to determine if drugs are the same or different. Drugs in different medication lists are considered different drugs. For example, a dispensing event from the Amlodipine Atorvastatin High Intensity Medications List and a dispensing event from the Amlodipine Atorvastatin Moderate Intensity Medications List are dispensing events for different medications.

Eligible Population: *Rate 1*—Received Statin Therapy

Product line

Commercial, Medicaid, Medicare (report each product line separately).

Age

Report two age/gender stratifications and a total rate:

18. Males 21–75 years as of December 31 of the measurement year.
19. Females 40–75 years as of December 31 of the measurement year.
20. Total.

Continuous enrollment

The measurement year and the year prior to the measurement year.

Allowable gap

No more than one gap in enrollment of up to 45 days during each year of continuous enrollment. To determine continuous enrollment for a Medicaid beneficiary for whom enrollment is verified monthly, the member may not have more than a 1-month gap in coverage (i.e., a member whose coverage lapses for 2 months [60 days] is not considered continuously enrolled).

Anchor date

December 31 of the measurement year.

Benefit

Medical. Pharmacy during the measurement year.

Event/diagnosis

Members are identified for the eligible population in two ways: by event or by diagnosis. The organization must use *both* methods to identify the eligible population, but a member only needs to be identified by one method to be included in the measure.

Event. Any of the following during the year prior to the measurement year meet criteria:

21. *MI.* Discharged from an inpatient setting with an MI (MI Value Set; Old Myocardial Infarction Value Set) on the discharge claim. To identify discharges:
    1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
    2. Identify the discharge date for the stay.
22. *CABG.* Members who had CABG (CABG Value Set) in any setting.
23. *PCI.* Members who had PCI (PCI Value Set) in any setting.
24. *Other revascularization*. Members who had any other revascularization procedures (Other Revascularization Value Set) in any setting.

25. *Diagnosis.* Identify members as having ischemic vascular disease (IVD) who met at least one of the following criteria during both the measurement year and the year prior to the measurement year. Criteria need not be the same across both years.
26. At least one outpatient visit (Outpatient Value Set) with an IVD diagnosis (IVD Value Set).
27. A telephone visit (Telephone Visits Value Set) with an IVD diagnosis (IVD Value Set).
28. An e-visit or virtual check-in (Online Assessments Value Set) with an IVD diagnosis (IVD Value Set).
29. At least one acute inpatient encounter (Acute Inpatient Value Set) with an IVD diagnosis (IVD Value Set) ***without*** telehealth (Telehealth Modifier Value Set; Telehealth POS Value Set).
30. At least one acute inpatient discharge with an IVD diagnosis (IVD Value Set) on the discharge claim. To identify an acute inpatient discharge:
    Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
    Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
    Identify the discharge date for the stay.

Required exclusions

Exclude members who meet any of the following criteria:

31. Members with a diagnosis of pregnancy (Pregnancy Value Set) during the measurement year or the year prior to the measurement year.
32. In vitro fertilization (IVF Value Set) in the measurement year or the year prior to the measurement year.
33. Dispensed at least one prescription for clomiphene (Estrogen Agonists Medications List) during the measurement year or the year prior to the measurement year.
34. ESRD (ESRD Diagnosis Value Set) or dialysis (Dialysis Procedure Value Set) during the measurement year or the year prior to the measurement year.
35. Cirrhosis (Cirrhosis Value Set) during the measurement year or the year prior to the measurement year.
36. Myalgia, myositis, myopathy or rhabdomyolysis (Muscular Pain and Disease Value Set) during the measurement year.
37. Members in hospice or using hospice services any time during the measurement year. Refer to *General Guideline 15: Members in Hospice*.
38. Members who died any time during the measurement year. Refer to *General Guideline 16: Deceased Members.*
39. Members receiving palliative care (Palliative Care Assessment Value Set; Palliative Care Encounter Value Set; Palliative Care Intervention Value Set; ICD-10-CM code Z51.5) any time during the measurement year.

Estrogen Agonists Medications

Description	Prescription
Estrogen agonists	Clomiphene

Exclusions Exclude members who meet any of the following criteria:

Note: *Supplemental and medical record data may not be used for these exclusions.*

40. Medicare members 66 years of age and older as of December 31 of the measurement year who meet either of the following:
    - Enrolled in an Institutional SNP (I-SNP) any time during the measurement year.
    - Living long-term in an institution any time during the measurement year as identified by the LTI flag in the Monthly Membership Detail Data File. Use the run date of the file to determine if a member had an LTI flag during the measurement year.
41. Members 66 years of age and older as of December 31 of the measurement year (all product lines) with frailty ***and*** advanced illness. Members must meet *both* of the following frailty and advanced illness criteria to be excluded:
    1. At least two indications of frailty (Frailty Device Value Set; Frailty Diagnosis Value Set; Frailty Encounter Value Set; Frailty Symptom Value Set) with different dates of service during the measurement year.
    2. Any of the following during the measurement year or the year prior to the measurement year (count services that occur over both years):
        - At least two outpatient visits (Outpatient Value Set), observation visits (Observation Value Set), ED visits (ED Value Set), telephone visits (Telephone Visits Value Set), e-visits or virtual check-ins (Online Assessments Value Set), nonacute inpatient encounters (Nonacute Inpatient Value Set) or nonacute inpatient discharges (instructions below; the diagnosis must be on the discharge claim) on different dates of service, with an advanced illness diagnosis (Advanced Illness Value Set). Visit type need not be the same for the two visits. To identify a nonacute inpatient discharge:
            1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
            2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set) on the claim.
            3. Identify the discharge date for the stay.
        - At least one acute inpatient encounter (Acute Inpatient Value Set) with an advanced illness diagnosis (Advanced Illness Value Set).
        - At least one acute inpatient discharge with an advanced illness diagnosis (Advanced Illness Value Set) on the discharge claim. To identify an acute inpatient discharge:
            1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
            2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
            3. Identify the discharge date for the stay.
        - A dispensed dementia medication (Dementia Medications List).

Dementia Medications

Description	Prescription
Cholinesterase inhibitors	• Donepezil • Galantamine • Rivastigmine
Miscellaneous central nervous system agents	• Memantine
Dementia combinations	• Donepezil-memantine

Administrative Specification: Rate 1—Received Statin Therapy

Denominator The Rate 1 eligible population.

Numerator The number of members who had at least one dispensing event for a high-intensity or moderate-intensity statin medication during the measurement year. Use all the medication lists below to identify statin medication dispensing events.

High- and Moderate-Intensity Statin Medications

Description	Prescription	Medication Lists
High-intensity statin therapy	Atorvastatin 40-80 mg	Atorvastatin High Intensity Medications List
High-intensity statin therapy	Amlodipine-atorvastatin 40-80 mg	Amlodipine Atorvastatin High Intensity Medications List
High-intensity statin therapy	Rosuvastatin 20-40 mg	Rosuvastatin High Intensity Medications List
High-intensity statin therapy	Simvastatin 80 mg	Simvastatin High Intensity Medications List
High-intensity statin therapy	Ezetimibe-simvastatin 80 mg	Ezetimibe Simvastatin High Intensity Medications List
Moderate-intensity statin therapy	Atorvastatin 10-20 mg	Atorvastatin Moderate Intensity Medications List
Moderate-intensity statin therapy	Amlodipine-atorvastatin 10-20 mg	Amlodipine Atorvastatin Moderate Intensity Medications List
Moderate-intensity statin therapy	Rosuvastatin 5-10 mg	Rosuvastatin Moderate Intensity Medications List
Moderate-intensity statin therapy	Simvastatin 20-40 mg	Simvastatin Moderate Intensity Medications List
Moderate-intensity statin therapy	Ezetimibe-simvastatin 20-40 mg	Ezetimibe Simvastatin Moderate Intensity Medications List
Moderate-intensity statin therapy	Pravastatin 40-80 mg	Pravastatin Moderate Intensity Medications List
Moderate-intensity statin therapy	Lovastatin 40 mg	Lovastatin Moderate Intensity Medications List
Moderate-intensity statin therapy	Fluvastatin 40-80 mg	Fluvastatin Moderate Intensity Medications List
Moderate-intensity statin therapy	Pitavastatin 1-4 mg	Pitavastatin Moderate Intensity Medications List

Eligible Population: *Rate 2*—Statin Adherence 80%

Product line	Commercial, Medicaid, Medicare (report each product line separately).
Age	Report two age/gender stratifications and a total rate: 42. Males 21–75 years as of December 31 of the measurement year. 43. Females 40–75 years as of December 31 of the measurement year. 44. Total.
Continuous enrollment	The measurement year and the year prior to the measurement year.
Allowable gap	No more than one gap in enrollment of up to 45 days during each year of continuous enrollment. To determine continuous enrollment for a Medicaid beneficiary for whom enrollment is verified monthly, the member may not have more than a 1-month gap in coverage (e.g., a member whose coverage lapses for 2 months [60 days] is not considered continuously enrolled).
Anchor date	December 31 of the measurement year.
Benefit	Medical. Pharmacy during the measurement year.
Event/diagnosis	All members who meet the numerator criteria for Rate 1.

Administrative Specification: *Rate 2*—Statin Adherence 80%

Denominator The Rate 2 eligible population.

Numerator The number of members who achieved a PDC of at least 80% during the treatment period.

Follow the steps below to identify numerator compliance.

Step 1 Identify the IPSD. The IPSD is the earliest dispensing event for any high-intensity or moderate-intensity statin medication during the measurement year. Use all the medications lists above to identify statin medication dispensing events.

Step 2 To determine the treatment period, calculate the number of days beginning on the IPSD through the end of the measurement year.

Step 3 Count the days covered by at least one prescription for any high-intensity or moderate-intensity statin medication during the treatment period. To ensure that days supply that extends beyond the measurement year is not counted, subtract any days supply that extends beyond December 31 of the measurement year.

Step 4 Calculate the member's PDC using the following equation. Multiply the equation by 100 and round (using the .5 rule) to the nearest whole number. For example, if a member has 291 total days covered by a medication during a 365-day treatment period, this calculates to 0.7972. Multiply this number by 100, convert it to 79.72% and round it to 80%, the nearest whole number.

$$\frac{\text{Total Days Covered by a Statin Medication in the Treatment Period (step 3)}}{\text{Total Days in Treatment Period (step 2)}}$$

Step 5 Sum the number of members whose PDC is ≥80% for the treatment period.

Note

21. All members who are numerator compliant for Rate 1 must be used as the eligible population for Rate 2 (regardless of the data source used to capture the Rate 1 numerator). For example, if supplemental data were used to identify compliance for the Rate 1 numerator, then supplemental data will be included in identifying the Rate 2 eligible population.

Data Elements for Reporting

Organizations that submit HEDIS data to NCQA must provide the following data elements.

Table SPC-1/2/3: Data Elements for Statin Therapy for Patients With Cardiovascular Disease

Metric	Gender	Data Element	Reporting Instructions
ReceivedTherapy	F	Benefit	Metadata
Adherence	M	EligiblePopulation	For each Metric and Stratification
	Total	ExclusionAdminRequired	Only for ReceivedTherapy Metric
		NumeratorByAdmin	For each Metric and Stratification
		NumeratorBySupplemental	For each Metric and Stratification
		Rate	(Percent)

Rules for Allowable Adjustments of HEDIS

The "Rules for Allowable Adjustments of HEDIS" (the "Rules") describe how NCQA's HEDIS measure specifications can be adjusted for other populations, if applicable. The Rules, reviewed and approved by NCQA measure experts, provide for expanded use of HEDIS measures without changing their clinical intent. **Adjusted HEDIS measures *may not* be used for HEDIS health plan reporting.**

Rules for Allowable Adjustments of Statin Therapy for Patients With Cardiovascular Disease

NONCLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Product lines	Yes	Using product line criteria is not required. Including any product line, combining product lines, or not including product line criteria is allowed.
Ages	Yes, with limits	Age determination dates may be changed (e.g., select, "age as of June 30"). The denominator age may be changed if the range is within the specified age range (ages 21–75 or 40–75 years). The denominator age may not be expanded.
Continuous enrollment, allowable gap, anchor date	Yes	Organizations are not required to use enrollment criteria; adjustments are allowed.
Benefits	Yes	Organizations are not required to use enrollment criteria; adjustments are allowed.
Other	Yes	Organizations may use additional eligible population criteria to focus on an area of interest defined by gender, race, ethnicity, socioeconomic or sociodemographic characteristics, geographic region or another characteristic.
CLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Event/diagnosis	No	Only events that contain (or map to) codes in the value sets may be used to identify discharges. Value sets and logic may not be changed.
Denominator Exclusions	**Adjustments Allowed (Yes/No)**	**Notes**
Required exclusions	Yes, with limits	Apply required exclusions according to specified value sets and medication lists. The hospice, deceased member and palliative care exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments.*
Exclusions: I-SNP, LTI, frailty or advanced illness	Yes	These exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments*.
Numerator Criteria	**Adjustments Allowed (Yes/No)**	**Notes**
Rate 1: Received Statin Therapy Rate 2: Statin Adherence 80%	No	Medication lists, value sets and logic may not be changed.

Diabetes

Hemoglobin A1c Control for Patients With Diabetes (HBD)

SUMMARY OF CHANGES TO HEDIS MY 2023

- Added a required exclusion for members who died during the measurement year.
- Added a direct reference code for palliative care.
- Updated the number of occurrences required for the frailty cross-cutting exclusion.
- Revised the "Other" criteria in the Nonclinical Components table under *Rules for Allowable Adjustments of HEDIS.*
- Revised the "Required exclusions" criteria in the Clinical Components table under *Rules for Allowable Adjustments of HEDIS*.

Description

The percentage of members 18–75 years of age with diabetes (types 1 and 2) whose hemoglobin A1c (HbA1c) was at the following levels during the measurement year:

- HbA1c Control (<8.0%).
- HbA1c Poor Control (>9.0%).

Note: *Organizations must use the same data collection method (Administrative or Hybrid) to report these indicators.*

Eligible Population

Product lines Commercial, Medicaid, Medicare (report each product line separately).

Stratification For each product line, report the following stratifications by race and total, and stratifications by ethnicity and total:

- *Race:*
  - White.
  - Black or African American.
  - American Indian or Alaska Native.
  - Asian.
  - Native Hawaiian or Other Pacific Islander.
  - Some Other Race.
  - Two or More Races.
  - Asked but No Answer.
  - Unknown.
  - Total.
- *Ethnicity:*
  - Hispanic or Latino.
  - Not Hispanic or Latino.
  - Asked but No Answer.
  - Unknown.
  - Total.

Note: *Stratifications are mutually exclusive and the sum of all categories in each stratification is the total population.*

Ages 18–75 years as of December 31 of the measurement year.

Continuous enrollment The measurement year.

Allowable gap No more than one gap in enrollment of up to 45 days during the measurement year. To determine continuous enrollment for a Medicaid beneficiary for whom enrollment is verified monthly, the member may not have more than a 1-month gap in coverage (e.g., a member whose coverage lapses for 2 months [60 days] is not considered continuously enrolled).

Anchor date December 31 of the measurement year.

Benefit Medical.

Event/diagnosis There are two ways to identify members with diabetes: by claim/encounter data and by pharmacy data. The organization must use both methods to identify the eligible population, but a member only needs to be identified by one method to be included in the measure. Members may be identified as having diabetes during the measurement year or the year prior to the measurement year.

Claim/encounter data. Members who met any of the following criteria during the measurement year or the year prior to the measurement year (count services that occur over both years):

- At least one acute inpatient encounter (Acute Inpatient Value Set) with a diagnosis of diabetes (Diabetes Value Set) ***without*** telehealth (Telehealth Modifier Value Set; Telehealth POS Value Set).
- At least one acute inpatient discharge with a diagnosis of diabetes (Diabetes Value Set) on the discharge claim. To identify an acute inpatient discharge:
  1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
  2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
  3. Identify the discharge date for the stay.
- At least two outpatient visits (Outpatient Value Set), observation visits (Observation Value Set), telephone visits (Telephone Visits Value Set), e-visits or virtual check-ins (Online Assessments Value Set), ED visits (ED Value Set), nonacute inpatient encounters (Nonacute Inpatient Value Set) or nonacute inpatient discharges (instructions below; the diagnosis must be on the discharge claim), on different dates of service, with a diagnosis of diabetes (Diabetes Value Set). Visit type need not be the same for the two encounters. To identify a nonacute inpatient discharge:
  1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
  2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set) on the claim.

3. Identify the discharge date for the stay.

Only include nonacute inpatient encounters (Nonacute Inpatient Value Set) ***without*** telehealth (Telehealth Modifier Value Set; Telehealth POS Value Set).

Pharmacy data. Members who were dispensed insulin or hypoglycemics/ antihyperglycemics during the measurement year or the year prior to the measurement year (Diabetes Medications List).

Diabetes Medications

Description	Prescription		
Alpha-glucosidase inhibitors	• Acarbose	• Miglitol	
Amylin analogs	• Pramlintide		
Antidiabetic combinations	• Alogliptin-metformin • Alogliptin-pioglitazone • Canagliflozin-metformin • Dapagliflozin-metformin • Dapagliflozin-saxagliptin • Empagliflozin-linagliptin • Empagliflozin-linagliptin-metformin	• Empagliflozin-metformin • Ertugliflozin-metformin • Ertugliflozin-sitagliptin • Glimepiride-pioglitazone • Glipizide-metformin • Glyburide-metformin	• Linagliptin-metformin • Metformin-pioglitazone • Metformin-repaglinide • Metformin-rosiglitazone • Metformin-saxagliptin • Metformin-sitagliptin
Insulin	• Insulin aspart • Insulin aspart-insulin aspart protamine • Insulin degludec • Insulin degludec-liraglutide • Insulin detemir • Insulin glargine • Insulin glargine-lixisenatide	• Insulin glulisine • Insulin isophane human • Insulin isophane-insulin regular • Insulin lispro • Insulin lispro-insulin lispro protamine • Insulin regular human • Insulin human inhaled	
Meglitinides	• Nateglinide	• Repaglinide	
Glucagon-like peptide-1 (GLP1) agonists	• Albiglutide • Dulaglutide • Exenatide	• Liraglutide (excluding Saxenda®) • Lixisenatide • Semaglutide	
Sodium glucose cotransporter 2 (SGLT2) inhibitor	• Canagliflozin • Dapagliflozin (excluding Farxiga®)	• Ertugliflozin • Empagliflozin	
Sulfonylureas	• Chlorpropamide • Glimepiride	• Glipizide • Glyburide	• Tolazamide • Tolbutamide
Thiazolidinediones	• Pioglitazone	• Rosiglitazone	
Dipeptidyl peptidase-4 (DDP-4) inhibitors	• Alogliptin • Linagliptin	• Saxagliptin • Sitagliptin	

Note: *Glucophage/metformin as a solo agent is not included because it is used to treat conditions other than diabetes; members with diabetes on these medications are identified through diagnosis codes only.*

Required exclusions

Exclude members who meet any of the following criteria:

- Members who did not have a diagnosis of diabetes (Diabetes Value Set), in any setting, during the measurement year or the year prior to the measurement year ***and*** who had a diagnosis of polycystic ovarian syndrome, gestational diabetes or steroid-induced diabetes (Diabetes Exclusions Value Set), in any setting, during the measurement year or the year prior to the measurement year.
- Members in hospice or using hospice services any time during the measurement year. Refer to *General Guideline 15: Members in Hospice.*
- Members who died any time during the measurement year. Refer to *General Guideline 16: Deceased Members.*
- Members receiving palliative care (Palliative Care Assessment Value Set; Palliative Care Encounter Value Set; Palliative Care Intervention Value Set; ICD-10-CM code Z51.5) any time during the measurement year.

Exclusions

Exclude members who meet any of the following criteria:

Note: *Supplemental and medical record data may not be used for these exclusions.*

- Medicare members 66 years of age and older as of December 31 of the measurement year who meet either of the following:
  - Enrolled in an Institutional SNP (I-SNP) any time during the measurement year.
  - Living long-term in an institution any time during the measurement year as identified by the LTI flag in the Monthly Membership Detail Data File. Use the run date of the file to determine if a member had an LTI flag during the measurement year.
- Members 66 years of age and older as of December 31 of the measurement year (all product lines) with frailty ***and*** advanced illness. Members must meet **BOTH** of the following frailty and advanced illness criteria to be excluded:
  1. At least two indications of frailty (Frailty Device Value Set; Frailty Diagnosis Value Set; Frailty Encounter Value Set; Frailty Symptom Value Set) with different dates of service during the measurement year.
  2. Any of the following during the measurement year or the year prior to the measurement year (count services that occur over both years):
     - At least two outpatient visits (Outpatient Value Set), observation visits (Observation Value Set), ED visits (ED Value Set), telephone visits (Telephone Visits Value Set), e-visits or virtual check-ins (Online Assessments Value Set), nonacute inpatient encounters (Nonacute Inpatient Value Set) or nonacute inpatient discharges (instructions below; the diagnosis must be on the discharge claim) on different dates of service, with an advanced illness diagnosis (Advanced Illness Value Set). Visit type need not be the same for the two visits. To identify a nonacute inpatient discharge:
       1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
       2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set) on the claim.

- Identify the discharge date for the stay.
- At least one acute inpatient encounter (Acute Inpatient Value Set) with an advanced illness diagnosis (Advanced Illness Value Set).
- At least one acute inpatient discharge with an advanced illness diagnosis (Advanced Illness Value Set) on the discharge claim. To identify an acute inpatient discharge:
  1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
  2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
  3. Identify the discharge date for the stay.
- A dispensed dementia medication (Dementia Medications List).

Dementia Medications

Description	Prescription
Cholinesterase inhibitors	• Donepezil • Galantamine • Rivastigmine
Miscellaneous central nervous system agents	• Memantine
Dementia combinations	• Donepezil-memantine

Administrative Specification

Denominator The eligible population.

Numerators

HbA1c Control <8% Use codes (HbA1c Lab Test Value Set; HbA1c Test Result or Finding Value Set) to identify the most recent HbA1c test during the measurement year. The member is numerator compliant if the most recent HbA1c level is <8.0%. The member is not numerator compliant if the result for the most recent HbA1c test is ≥8.0% or is missing a result, or if an HbA1c test was not done during the measurement year.

Organizations that use CPT Category II codes to identify numerator compliance for this indicator must search for all codes in the following value sets and use the most recent code during the measurement year to evaluate whether the member is numerator compliant.

Value Set	Numerator Compliance
HbA1c Level Less Than 7.0 Value Set	Compliant
HbA1c Level Greater Than or Equal To 7.0 and Less Than 8.0 Value Set	Compliant
HbA1c Level Greater Than or Equal To 8.0 and Less Than or Equal To 9.0 Value Set	Not compliant
HbA1c Level Greater Than 9.0 Value Set	Not compliant

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HbA1c Poor Control >9%

Use codes (HbA1c Lab Test Value Set; HbA1c Test Result or Finding Value Set) to identify the *most recent* HbA1c test during the measurement year. The member is numerator compliant if the most recent HbA1c level is >9.0% or is missing a result, or if an HbA1c test was not done during the measurement year. The member is not numerator compliant if the result for the most recent HbA1c test during the measurement year is ≤9.0%.

Organizations that use CPT Category II codes to identify numerator compliance for this indicator must search for all codes in the following value sets and use the most recent code during the measurement year to evaluate whether the member is numerator compliant.

Value Set	Numerator Compliance
HbA1c Level Less Than 7.0 Value Set	Not compliant
HbA1c Level Greater Than or Equal To 7.0 and Less Than 8.0 Value Set	Not compliant
HbA1c Level Greater Than or Equal To 8.0 and Less Than or Equal To 9.0 Value Set	Not compliant
HbA1c Level Greater Than 9.0 Value Set	Compliant

Note: *A lower rate indicates better performance for this indicator (i.e., low rates of poor control indicate better care).*

Hybrid Specification

Denominator

A systematic sample drawn from the eligible population.

Organizations that use the Hybrid Method to report the Hemoglobin A1c Control for Patients With Diabetes (HBD), Eye Exam for Patients With Diabetes (EED) and Blood Pressure Control for Patients With Diabetes (BPD) measures may use the same sample for all three measures. If the same sample is used for the three diabetes measures, the organization must first take the inverse of the HbA1c poor control >9.0% rate (100 minus the HbA1c poor control rate) before reducing the sample.

Organizations may reduce the sample size based on the current year's administrative rate or the prior year's audited, product line-specific rate for the lowest rate of all HBD indicators and EED and BPD measures.

If separate samples are used for the HBD, EED and BPD measures, organizations may reduce the sample based on the product line-specific current measurement year's administrative rate or the prior year's audited, product line-specific rate for the measure.

Refer to the *Guidelines for Calculations and Sampling* for information on reducing sample size.

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Numerators

HbA1c Control <8%

The *most recent* HbA1c level (performed during the measurement year) is <8.0% as identified by laboratory data or medical record review.

Administrative

Refer to *Administrative Specification* to identify positive numerator hits from administrative data.

Medical record

At a minimum, documentation in the medical record must include a note indicating the date when the HbA1c test was performed and the result. The member is numerator compliant if the most recent HbA1c level during the measurement year is <8.0%. The member is not numerator compliant if the result for the most recent HbA1c level during the measurement year is ≥8.0% or is missing, or if an HbA1c test was not performed during the measurement year.

Ranges and thresholds do not meet criteria for this indicator. A distinct numeric result is required for numerator compliance.

HbA1c Poor Control >9%

The *most recent* HbA1c level (performed during the measurement year) is >9.0% or is missing, or was not done during the measurement year, as documented through laboratory data or medical record review.

Note: *A lower rate indicates better performance for this indicator (i.e., low rates of poor control indicate better care).*

Administrative

Refer to *Administrative Specification* to identify positive numerator hits from administrative data.

Medical record

At a minimum, documentation in the medical record must include a note indicating the date when the HbA1c test was performed and the result. The member is numerator compliant if the result for the most recent HbA1c level during the measurement year is >9.0% or is missing, or if an HbA1c test was not done during the measurement year. The member is not numerator compliant if the most recent HbA1c level during the measurement year is ≤9.0%.

Ranges and thresholds do not meet criteria for this indicator. A distinct numeric result is required for numerator compliance.

Note

- *If a combination of administrative, supplemental or hybrid data are used, the most recent HbA1c result must be used, regardless of data source.*

Data Elements for Reporting

Organizations that submit HEDIS data to NCQA must provide the following data elements.

Table HBD-A-1/2/3: Data Elements for Hemoglobin A1c Control for Patients With Diabetes

Metric	Data Element	Reporting Instructions	A
AdequateHbA1cControl	CollectionMethod	Repeat per Metric	✓
PoorHbA1cControl	EligiblePopulation*	For each Metric	✓
	ExclusionAdminRequired*	For each Metric	✓
	NumeratorByAdminElig	For each Metric	
	CYAR	(Percent)	
	MinReqSampleSize	Repeat per Metric	
	OversampleRate	Repeat per Metric	
	OversampleRecordsNumber	(Count)	
	ExclusionValidDataErrors	Repeat per Metric	
	ExclusionEmployeeOrDep	Repeat per Metric	
	OversampleRecsAdded	Repeat per Metric	
	Denominator	Repeat per Metric	
	NumeratorByAdmin	For each Metric	✓
	NumeratorByMedicalRecords	For each Metric	
	NumeratorBySupplemental	For each Metric	✓
	Rate	(Percent)	✓

Table HBD-B-1/2/3: Data Elements for Hemoglobin A1c Control for Patients With Diabetes: Stratifications by Race

Metric
AdequateHbA1cControl
PoorHbA1cControl

Race	Source	Data Element	Reporting Instructions	A
White	Direct	CollectionMethod	Repeat per Metric and Stratification	✓
BlackOrAfricanAmerican	Indirect	EligiblePopulation*	For each Metric and Stratification	✓
AmericanIndianOrAlaskaNative	Total	Denominator	For each Stratification, repeat per Metric	
Asian		Numerator	For each Metric and Stratification	✓
NativeHawaiianOrOtherPacificIslander		Rate	(Percent)	✓
SomeOtherRace				
TwoOrMoreRaces				
AskedButNoAnswer**				
Unknown***				

Table HBD-C-1/2/3: Data Elements for Hemoglobin A1c Control for Patients With Diabetes: Stratifications by Ethnicity

Metric
AdequateHbA1cControl
PoorHbA1cControl

Ethnicity	Source	Data Element	Reporting Instructions	A
HispanicOrLatino	Direct	CollectionMethod	Repeat per Metric and Stratification	✓
NotHispanicOrLatino	Indirect	EligiblePopulation*	For each Metric and Stratification	✓
AskedButNoAnswer**	Total	Denominator	For each Stratification, repeat per Metric	
Unknown***		Numerator	For each Metric and Stratification	✓
		Rate	(Percent)	✓

*Repeat the EligiblePopulation and ExclusionAdminRequired values for metrics using the Administrative Method.
**AskedButNoAnswer is only reported for Source='Direct.'
***Unknown is only reported for Source='Indirect.'

Rules for Allowable Adjustments of HEDIS

The "Rules for Allowable Adjustments of HEDIS" (the "Rules") describe how NCQA's HEDIS measure specifications can be adjusted for other populations, if applicable. The Rules, reviewed and approved by NCQA measure experts, provide for expanded use of HEDIS measures without changing their clinical intent.

Adjusted HEDIS measures *may not* be used for HEDIS health plan reporting.

Rules for Allowable Adjustments of Hemoglobin A1c Control for Patients With Diabetes

NONCLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Product lines	Yes	Organizations are not required to use product line criteria; product lines may be combined and all (or no) product line criteria may be used.
Ages	Yes, with limits	Age determination dates may be changed (e.g., select, "age as of June 30"). Changing denominator age range is allowed within a specified age range (ages 18–75 years). The denominator age may not be expanded.
Continuous enrollment, allowable gap, anchor date	Yes	Organizations are not required to use enrollment criteria; adjustments are allowed.
Benefits	Yes	Organizations are not required to use a benefit; adjustments are allowed.
Other	Yes	Organizations may use additional eligible population criteria to focus on an area of interest defined by gender, race, ethnicity, socioeconomic or sociodemographic characteristics, geographic region or another characteristic.
CLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Event/diagnosis	No	Only events or diagnoses that contain (or map to) codes in the medication lists and value sets may be used to identify visits. Medication lists, value sets and logic may not be changed.
Denominator Exclusions	**Adjustments Allowed (Yes/No)**	**Notes**
Required exclusions	Yes, with limits	Apply required exclusions according to specified value sets. The hospice, deceased member and palliative care exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments*.
Exclusions: I-SNP, LTI, frailty or advanced illness	Yes	These exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments*.
Numerator Criteria	**Adjustments Allowed (Yes/No)**	**Notes**
• HbA1c Control (<8.0%) • HbA1c Poor Control (>9.0%)	No	Value sets and logic may not be changed.

Blood Pressure Control for Patients With Diabetes (BPD)

SUMMARY OF CHANGES TO HEDIS MY 2023

- Added a required exclusion for members who died during the measurement year.
- Added a direct reference code for palliative care.
- Updated the number of occurrences required for the frailty cross-cutting exclusion.
- Revised the "Other" criteria in the Nonclinical Components table under *Rules for Allowable Adjustments of HEDIS.*
- Revised the "Required exclusions" criteria in the Clinical Components table under *Rules for Allowable Adjustments of HEDIS*.

Description

The percentage of members 18–75 years of age with diabetes (types 1 and 2) whose blood pressure (BP) was adequately controlled (<140/90 mm Hg) during the measurement year.

Eligible Population

Product lines	Commercial, Medicaid, Medicare (report each product line separately).
Ages	18–75 years as of December 31 of the measurement year.
Continuous enrollment	The measurement year.
Allowable gap	No more than one gap in enrollment of up to 45 days during the measurement year. To determine continuous enrollment for a Medicaid beneficiary for whom enrollment is verified monthly, the member may not have more than a 1-month gap in coverage (e.g., a member whose coverage lapses for 2 months [60 days] is not considered continuously enrolled).
Anchor date	December 31 of the measurement year.
Benefit	Medical.
Event/diagnosis	There are two ways to identify members with diabetes: by claim/encounter data and by pharmacy data. The organization must use both methods to identify the eligible population, but a member only needs to be identified by one method to be included in the measure. Members may be identified as having diabetes during the measurement year or the year prior to the measurement year. *Claim/encounter data*. Members who met any of the following criteria during the measurement year or the year prior to the measurement year (count services that occur over both years): • At least one acute inpatient encounter (Acute Inpatient Value Set) with a diagnosis of diabetes (Diabetes Value Set) ***without*** telehealth (Telehealth Modifier Value Set; Telehealth POS Value Set). • At least one acute inpatient discharge with a diagnosis of diabetes (Diabetes Value Set) on the discharge claim. To identify an acute inpatient discharge:

1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
3. Identify the discharge date for the stay.

- At least two outpatient visits (Outpatient Value Set), observation visits (Observation Value Set), telephone visits (Telephone Visits Value Set), e-visits or virtual check-ins (Online Assessments Value Set), ED visits (ED Value Set), nonacute inpatient encounters (Nonacute Inpatient Value Set) or nonacute inpatient discharges (instructions below; the diagnosis must be on the discharge claim), on different dates of service, with a diagnosis of diabetes (Diabetes Value Set). Visit type need not be the same for the two encounters. To identify a nonacute inpatient discharge:
  1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
  2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set) on the claim.
  3. Identify the discharge date for the stay.

  Only include nonacute inpatient encounters (Nonacute Inpatient Value Set) ***without*** telehealth (Telehealth Modifier Value Set; Telehealth POS Value Set).

Pharmacy data. Members who were dispensed insulin or hypoglycemics/antihyperglycemics during the measurement year or the year prior to the measurement year (Diabetes Medications List).

Diabetes Medications

Description	Prescription		
Alpha-glucosidase inhibitors	• Acarbose	• Miglitol	
Amylin analogs	• Pramlintide		
Antidiabetic combinations	• Alogliptin-metformin • Alogliptin-pioglitazone • Canagliflozin-metformin • Dapagliflozin-metformin • Dapagliflozin-saxagliptin • Empagliflozin-linagliptin • Empagliflozin-linagliptin-metformin	• Empagliflozin-metformin • Ertugliflozin-metformin • Ertugliflozin-sitagliptin • Glimepiride-pioglitazone • Glipizide-metformin • Glyburide-metformin • Linagliptin-metformin	• Metformin-pioglitazone • Metformin-repaglinide • Metformin-rosiglitazone • Metformin-saxagliptin • Metformin-sitagliptin
Insulin	• Insulin aspart • Insulin aspart-insulin aspart protamine • Insulin degludec • Insulin degludec-liraglutide • Insulin detemir • Insulin glargine • Insulin glargine-lixisenatide	• Insulin glulisine • Insulin isophane human • Insulin isophane-insulin regular • Insulin lispro • Insulin lispro-insulin lispro protamine • Insulin regular human • Insulin human inhaled	

Description	Prescription		
Meglitinides	• Nateglinide	• Repaglinide	
Glucagon-like peptide-1 (GLP1) agonists	• Albiglutide • Dulaglutide • Exenatide	• Liraglutide (excluding Saxenda®) • Lixisenatide • Semaglutide	
Sodium glucose cotransporter 2 (SGLT2) inhibitor	• Canagliflozin • Dapagliflozin (excluding Farxiga®)	• Empagliflozin • Ertugliflozin	
Sulfonylureas	• Chlorpropamide • Glimepiride	• Glipizide • Glyburide	• Tolazamide • Tolbutamide
Thiazolidinediones	• Pioglitazone	• Rosiglitazone	
Dipeptidyl peptidase-4 (DDP-4) inhibitors	• Alogliptin • Linagliptin	• Saxagliptin • Sitagliptin	

Note: *Glucophage/metformin as a solo agent is not included because it is used to treat conditions other than diabetes; members with diabetes on these medications are identified through diagnosis codes only.*

Required exclusions

Exclude members who meet any of the following criteria:

- Members who did not have a diagnosis of diabetes (Diabetes Value Set), in any setting, during the measurement year or the year prior to the measurement year ***and*** who had a diagnosis of polycystic ovarian syndrome, gestational diabetes or steroid-induced diabetes (Diabetes Exclusions Value Set), in any setting, during the measurement year or the year prior to the measurement year.
- Members in hospice or using hospice services any time during the measurement year. Refer to *General Guideline 15: Members in Hospice.*
- Members who died any time during the measurement year. Refer to *General Guideline 16: Deceased Members.*
- Members receiving palliative care (Palliative Care Assessment Value Set; Palliative Care Encounter Value Set; Palliative Care Intervention Value Set; ICD-10-CM code Z51.5) any time during the measurement year.

Exclusions

Exclude members who meet any of the following criteria:

Note: *Supplemental and medical record data may not be used for these exclusions.*

- Medicare members 66 years of age and older as of December 31 of the measurement year who meet either of the following:
  - Enrolled in an Institutional SNP (I-SNP) any time during the measurement year.
  - Living long-term in an institution any time during the measurement year as identified by the LTI flag in the Monthly Membership Detail Data File. Use the run date of the file to determine if a member had an LTI flag during the measurement year.
- Members 66 years of age and older as of December 31 of the measurement year (all product lines) with frailty ***and*** advanced illness. Members must meet **BOTH** of the following frailty and advanced illness criteria to be excluded:

1. At least two indications of frailty (Frailty Device Value Set; Frailty Diagnosis Value Set; Frailty Encounter Value Set; Frailty Symptom Value Set) with different dates of service during the measurement year.
2. Any of the following during the measurement year or the year prior to the measurement year (count services that occur over both years):
   - At least two outpatient visits (Outpatient Value Set), observation visits (Observation Value Set), ED visits (ED Value Set), telephone visits (Telephone Visits Value Set), e-visits or virtual check-ins (Online Assessments Value Set), nonacute inpatient encounters (Nonacute Inpatient Value Set) or nonacute inpatient discharges (instructions below; the diagnosis must be on the discharge claim) on different dates of service, with an advanced illness diagnosis (Advanced Illness Value Set). Visit type need not be the same for the two visits. To identify a nonacute inpatient discharge:
     1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
     2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set) on the claim.
     3. Identify the discharge date for the stay.
   - At least one acute inpatient encounter (Acute Inpatient Value Set) with an advanced illness diagnosis (Advanced Illness Value Set).
   - At least one acute inpatient discharge with an advanced illness diagnosis (Advanced Illness Value Set) on the discharge claim. To identify an acute inpatient discharge:
     1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
     2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
     3. Identify the discharge date for the stay.
   - A dispensed dementia medication (Dementia Medications List).

Dementia Medications

Description	Prescription
Cholinesterase inhibitors	• Donepezil • Galantamine • Rivastigmine
Miscellaneous central nervous system agents	• Memantine
Dementia combinations	• Donepezil-memantine

Administrative Specification

Denominator The eligible population.

Numerator Identify the most recent BP reading (Systolic Blood Pressure Value Set; Diastolic Blood Pressure Value Set) taken during the measurement year. Exclude BPs taken in an acute inpatient setting (Acute Inpatient Value Set; Acute Inpatient POS Value Set) or during an ED visit (ED Value Set; ED POS Value Set).

The member is numerator compliant if the BP is <140/90 mm Hg. The member is not compliant if the BP is ≥140/90 mm Hg, if there is no BP reading during the measurement year or if the reading is incomplete (e.g., the systolic or diastolic level is missing). If there are multiple BPs on the same date of service, use the lowest systolic and lowest diastolic BP on that date as the representative BP. Organizations that use CPT Category II codes to identify numerator compliance must search for all codes in the following value sets and use the most recent codes during the measurement year to determine numerator compliance for both systolic and diastolic levels.

Value Set	Numerator Compliance
Systolic Less Than 140 Value Set	Systolic compliant
Systolic Greater Than or Equal To 140 Value Set	Systolic not compliant
Diastolic Less Than 80 Value Set	Diastolic compliant
Diastolic 80–89 Value Set	Diastolic compliant
Diastolic Greater Than or Equal To 90 Value Set	Diastolic not compliant

Hybrid Specification

Denominator A systematic sample drawn from the eligible population.

Organizations that use the Hybrid Method to report the Hemoglobin A1c Control for Patients With Diabetes (HBD), Eye Exam for Patients With Diabetes (EED) and Blood Pressure Control for Patients With Diabetes (BPD) measures may use the same sample for all three measures. If the same sample is used for the three diabetes measures, the organization must first take the inverse of the HbA1c poor control >9.0% rate (100 minus the HbA1c poor control rate) before reducing the sample.

Organizations may reduce the sample size based on the current year's administrative rate or the prior year's audited, product line-specific rate for the lowest rate of all HBD indicators and EED and BPD measures.

If separate samples are used for the HBD, EED and BPD measures, organizations may reduce the sample based on the product line-specific current measurement year's administrative rate or the prior year's audited, product line-specific rate for the measure.

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Refer to the *Guidelines for Calculations and Sampling* for information on reducing sample size.

Numerator

The *most recent* BP level (taken during the measurement year) is <140/90 mm Hg, as documented through administrative data or medical record review.

Administrative

Refer to *Administrative Specification* to identify positive numerator hits from administrative data.

Medical record

Organizations that use the same sample for the HBD, EED and BPD measures may use the medical record from which it abstracts data for the HBD and EED measures. If the organization uses separate samples for the HBD, EED and BPD measures, it should use the medical record of the provider that manages the member's diabetes. If that medical record does not contain a BP, the organization may use the medical record of another PCP or specialist from whom the member receives care.

Identify the most recent BP reading noted during the measurement year.

Do not include BP readings:

- Taken during an acute inpatient stay or an ED visit.
- Taken on the same day as a diagnostic test or diagnostic or therapeutic procedure that requires a change in diet or change in medication on or one day before the day of the test or procedure, with the exception of fasting blood tests.
- Taken by the member using a non-digital device such as with a manual blood pressure cuff and a stethoscope.

Identify the lowest systolic and lowest diastolic BP reading from the most recent BP notation in the medical record. If multiple readings were recorded for a single date, use the lowest systolic and lowest diastolic BP on that date as the representative BP. The systolic and diastolic results do not need to be from the same reading.

BP readings taken by the member and documented in the member's medical record are eligible for use in reporting (provided the BP does not meet any exclusion criteria). There is no requirement that there be evidence the BP was collected by a PCP or specialist.

The member is not compliant if the BP reading is ≥140/90 mm Hg or is missing, if there is no BP reading during the measurement year or if the reading is incomplete (i.e., the systolic or diastolic level is missing).

Ranges and thresholds do not meet criteria for this measure. A distinct numeric result for both the systolic and diastolic BP reading is required for numerator compliance. A BP documented as an "average BP" (e.g., "average BP: 139/70") is eligible for use.

Note

- *If a combination of administrative, supplemental or hybrid data are used, the most recent BP result must be used, regardless of data source.*
- *When excluding BP readings from the numerator, the intent is to identify diagnostic or therapeutic procedures that require a medication regimen, a change in diet or a change in medication. For example (this list is for reference only and is not exhaustive):*
  - *A colonoscopy requires a change in diet (NPO on the day of procedure) and a medication change (a medication is taken to prep the colon).*
  - *Dialysis, infusions and chemotherapy (including oral chemotherapy) are all therapeutic procedures that require a medication regimen.*
  - *A nebulizer treatment with albuterol is considered a therapeutic procedure that requires a medication regimen (the albuterol).*
  - *A patient forgetting to take regular medications on the day of the procedure is not considered a required change in medication, and therefore the BP reading is eligible.*
- *BP readings taken on the same day that the patient receives a common low-intensity or preventive procedure are eligible for use. For example, the following procedures are considered common low-intensity or preventive procedures (this list is for reference only and is not exhaustive):*
  - *Vaccinations.*
  - *Injections (e.g., allergy, vitamin B-12, insulin, steroid, toradol, Depo-Provera, testosterone, lidocaine).*
  - *TB test.*
  - *IUD insertion.*
  - *Eye exam with dilating agents.*
  - *Wart or mole removal.*

Data Elements for Reporting

Organizations that submit HEDIS data to NCQA must provide the following data elements.

Table BPD-1/2/3: Data Elements for Blood Pressure Control for Patients With Diabetes

Metric	Data Element	Reporting Instructions	A
BPUnder140Over90	CollectionMethod	Report once	✓
	EligiblePopulation	Report once	✓
	ExclusionAdminRequired	Report once	✓
	NumeratorByAdminElig	Report once	
	CYAR	(Percent)	
	MinReqSampleSize	Report once	
	OversampleRate	Report once	
	OversampleRecordsNumber	(Count)	
	ExclusionValidDataErrors	Report once	
	ExclusionEmployeeOrDep	Report once	
	OversampleRecsAdded	Report once	
	Denominator	Report once	
	NumeratorByAdmin	Report once	✓
	NumeratorByMedicalRecords	Report once	
	NumeratorBySupplemental	Report once	✓
	Rate	(Percent)	✓

Rules for Allowable Adjustments of HEDIS

The "Rules for Allowable Adjustments of HEDIS" (the "Rules") describe how NCQA's HEDIS measure specifications can be adjusted for other populations, if applicable. The Rules, reviewed and approved by NCQA measure experts, provide for expanded use of HEDIS measures without changing their clinical intent.

Adjusted HEDIS measures *may not* be used for HEDIS health plan reporting.

Rules for Allowable Adjustments of Blood Pressure Control for Patients With Diabetes

NONCLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Product lines	Yes	Organizations are not required to use product line criteria; product lines may be combined and all (or no) product line criteria may be used.
Ages	Yes, with limits	Age determination dates may be changed (e.g., select, "age as of June 30"). Changing denominator age range is allowed within a specified age range (ages 18–75 years). The denominator age may not be expanded.
Continuous enrollment, allowable gap, anchor date	Yes	Organizations are not required to use enrollment criteria; adjustments are allowed.
Benefits	Yes	Organizations are not required to use a benefit; adjustments are allowed.
Other	Yes	Organizations may use additional eligible population criteria to focus on an area of interest defined by gender, race, ethnicity, socioeconomic or sociodemographic characteristics, geographic region or another characteristic.
CLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Event/diagnosis	No	Only events or diagnoses that contain (or map to) codes in the medication lists and value sets may be used to identify visits. Medication lists, value sets and logic may not be changed.
Denominator Exclusions	**Adjustments Allowed (Yes/No)**	**Notes**
Required exclusions	Yes, with limits	Apply required exclusions according to specified value sets. The hospice, deceased member and palliative care exclusion are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments*.
Exclusions: I-SNP, LTI, frailty or advanced illness	Yes	These exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments*.
Numerator Criteria	**Adjustments Allowed (Yes/No)**	**Notes**
Blood Pressure Control	No	Value sets and logic may not be changed.

Eye Exam for Patients With Diabetes (EED)

SUMMARY OF CHANGES TO HEDIS MY 2023

- Added a required exclusion for members who died during the measurement year.
- Added a direct reference code for palliative care.
- Updated the number of occurrences required for the frailty cross-cutting exclusion.
- Added a *Note* to clarify that an eye exam result documented as "unknown" does not meet criteria.
- Revised the "Other" criteria in the Nonclinical Components table under *Rules for Allowable Adjustments of HEDIS.*
- Revised the "Required exclusions" criteria in the Clinical Components table under *Rules for Allowable Adjustments of HEDIS*.

Description

The percentage of members 18–75 years of age with diabetes (types 1 and 2) who had a retinal eye exam.

Eligible Population

Product lines Commercial, Medicaid, Medicare (report each product line separately).

Stratification For Medicare only, report the following SES stratifications and total:

- Non-LIS/DE, Nondisability.
- LIS/DE.
- Disability.
- LIS/DE and Disability.
- Other.
- Unknown.
- Total Medicare.

Note: *The stratifications are mutually exclusive and the sum of all six stratifications is the total population.*

Ages 18–75 years as of December 31 of the measurement year.

Continuous enrollment The measurement year.

Allowable gap No more than one gap in enrollment of up to 45 days during the measurement year. To determine continuous enrollment for a Medicaid beneficiary for whom enrollment is verified monthly, the member may not have more than a 1-month gap in coverage (e.g., a member whose coverage lapses for 2 months [60 days] is not considered continuously enrolled).

Anchor date December 31 of the measurement year.

Benefit Medical.

Event/diagnosis There are two ways to identify members with diabetes: by claim/encounter data and by pharmacy data. The organization must use both methods to identify the eligible population, but a member only needs to be identified by one method to be

included in the measure. Members may be identified as having diabetes during the measurement year or the year prior to the measurement year.

Claim/encounter data. Members who met any of the following criteria during the measurement year or the year prior to the measurement year (count services that occur over both years):

- At least one acute inpatient encounter (Acute Inpatient Value Set) with a diagnosis of diabetes (Diabetes Value Set) ***without*** telehealth (Telehealth Modifier Value Set; Telehealth POS Value Set).
- At least one acute inpatient discharge with a diagnosis of diabetes (Diabetes Value Set) on the discharge claim. To identify an acute inpatient discharge:
  1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
  2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
  3. Identify the discharge date for the stay.
- At least two outpatient visits (Outpatient Value Set), observation visits (Observation Value Set), telephone visits (Telephone Visits Value Set), e-visits or virtual check-ins (Online Assessments Value Set), ED visits (ED Value Set), nonacute inpatient encounters (Nonacute Inpatient Value Set) or nonacute inpatient discharges (instructions below; the diagnosis must be on the discharge claim), on different dates of service, with a diagnosis of diabetes (Diabetes Value Set). Visit type need not be the same for the two encounters. To identify a nonacute inpatient discharge:
  1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
  2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set) on the claim.
  3. Identify the discharge date for the stay.

  Only include nonacute inpatient encounters (Nonacute Inpatient Value Set) ***without*** telehealth (Telehealth Modifier Value Set; Telehealth POS Value Set).
- *Pharmacy data*. Members who were dispensed insulin or hypoglycemics/antihyperglycemics during the measurement year or the year prior to the measurement year (Diabetes Medications List).

Diabetes Medications

Description	Prescription		
Alpha-glucosidase inhibitors	• Acarbose	• Miglitol	
Amylin analogs	• Pramlintide		
Antidiabetic combinations	• Alogliptin-metformin • Alogliptin-pioglitazone • Canagliflozin-metformin • Dapagliflozin-metformin • Dapagliflozin-saxagliptin • Empagliflozin-linagliptin • Empagliflozin-linagliptin-metformin	• Empagliflozin-metformin • Ertugliflozin-metformin • Ertugliflozin-sitagliptin • Glimepiride-pioglitazone • Glipizide-metformin • Glyburide-metformin	• Linagliptin-metformin • Metformin-pioglitazone • Metformin-repaglinide • Metformin-rosiglitazone • Metformin-saxagliptin • Metformin-sitagliptin

Description	Prescription		
Insulin	• Insulin aspart • Insulin aspart-insulin aspart protamine • Insulin degludec • Insulin degludec-liraglutide • Insulin detemir • Insulin glargine • Insulin glargine-lixisenatide	• Insulin glulisine • Insulin isophane human • Insulin isophane-insulin regular • Insulin lispro • Insulin lispro-insulin lispro protamine • Insulin regular human • Insulin human inhaled	
Meglitinides	• Nateglinide	• Repaglinide	
Glucagon-like peptide-1 (GLP1) agonists	• Albiglutide • Dulaglutide • Exenatide	• Liraglutide (excluding Saxenda®) • Lixisenatide • Semaglutide	
Sodium glucose cotransporter 2 (SGLT2) inhibitor	• Canagliflozin	• Dapagliflozin (excluding Farxiga®)	• Empagliflozin • Ertugliflozin
Sulfonylureas	• Chlorpropamide • Glimepiride	• Glipizide • Glyburide	• Tolazamide • Tolbutamide
Thiazolidinediones	• Pioglitazone	• Rosiglitazone	
Dipeptidyl peptidase-4 (DDP-4) inhibitors	• Alogliptin • Linagliptin	• Saxagliptin • Sitagliptin	

Note: *Glucophage/metformin as a solo agent is not included because it is used to treat conditions other than diabetes; members with diabetes on these medications are identified through diagnosis codes only.*

Required exclusions

Exclude members who meet any of the following criteria:

- Members who did not have a diagnosis of diabetes (Diabetes Value Set), in any setting, during the measurement year or the year prior to the measurement year ***and*** who had a diagnosis of polycystic ovarian syndrome, gestational diabetes or steroid-induced diabetes (Diabetes Exclusions Value Set), in any setting, during the measurement year or the year prior to the measurement year.
- Members in hospice or using hospice services any time during the measurement year. Refer to *General Guideline 15: Members in Hospice.*
- Members who died any time during the measurement year. Refer to *General Guideline 16: Deceased Members.*
- Members receiving palliative care (Palliative Care Assessment Value Set; Palliative Care Encounter Value Set; Palliative Care Intervention Value Set; ICD-10-CM code Z51.5) any time during the measurement year.

Exclusions

Exclude members who meet any of the following criteria:

Note: *Supplemental and medical record data may not be used for these exclusions.*

- Medicare members 66 years of age and older as of December 31 of the measurement year who meet either of the following:
  - Enrolled in an Institutional SNP (I-SNP) any time during the measurement year.
  - Living long-term in an institution any time during the measurement year as identified by the LTI flag in the Monthly Membership Detail Data File. Use the run date of the file to determine if a member had an LTI flag during the measurement year.
- Members 66 years of age and older as of December 31 of the measurement year (all product lines) with frailty ***and*** advanced illness. Members must meet **BOTH** of the following frailty and advanced illness criteria to be excluded:
  1. At least two indications of frailty (Frailty Device Value Set; Frailty Diagnosis Value Set; Frailty Encounter Value Set; Frailty Symptom Value Set) with different dates of service during the measurement year.
  2. Any of the following during the measurement year or the year prior to the measurement year (count services that occur over both years):
     - At least two outpatient visits (Outpatient Value Set), observation visits (Observation Value Set), ED visits (ED Value Set), telephone visits (Telephone Visits Value Set), e-visits or virtual check-ins (Online Assessments Value Set), nonacute inpatient encounters (Nonacute Inpatient Value Set) or nonacute inpatient discharges (instructions below; the diagnosis must be on the discharge claim) on different dates of service, with an advanced illness diagnosis (Advanced Illness Value Set). Visit type need not be the same for the two visits. To identify a nonacute inpatient discharge:
       1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
       2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set) on the claim.
       3. Identify the discharge date for the stay.
     - At least one acute inpatient encounter (Acute Inpatient Value Set) with an advanced illness diagnosis (Advanced Illness Value Set).
     - At least one acute inpatient discharge with an advanced illness diagnosis (Advanced Illness Value Set) on the discharge claim. To identify an acute inpatient discharge:
       1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
       2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
       3. Identify the discharge date for the stay.
     - A dispensed dementia medication (Dementia Medications List).

Dementia Medications

Description	Prescription
Cholinesterase inhibitors	• Donepezil • Galantamine • Rivastigmine
Miscellaneous central nervous system agents	• Memantine
Dementia combinations	• Donepezil-memantine

Administrative Specification

Denominator The eligible population.

Numerator Screening or monitoring for diabetic retinal disease as identified by administrative data. This includes diabetics who had one of the following:

- A retinal or dilated eye exam by an eye care professional (optometrist or ophthalmologist) in the measurement year.
- A *negative* retinal or dilated eye exam (negative for retinopathy) by an eye care professional in the year prior to the measurement year.
- Bilateral eye enucleation any time during the member's history through December 31 of the measurement year.

Any of the following meet criteria:

- Any code in the Diabetic Retinal Screening Value Set billed by an eye care professional (optometrist or ophthalmologist) during the measurement year.
- Any code in the Diabetic Retinal Screening Value Set billed by an eye care professional (optometrist or ophthalmologist) during the year prior to the measurement year, with a diagnosis of diabetes without complications (Diabetes Mellitus Without Complications Value Set).
- Any code in the Eye Exam With Evidence of Retinopathy Value Set, Eye Exam Without Evidence of Retinopathy Value Set or Automated Eye Exam Value Set billed by any provider type during the measurement year.
- Any code in the Eye Exam Without Evidence of Retinopathy Value Set billed by any provider type during the year prior to the measurement year.
- Any code in the Diabetic Retinal Screening Negative In Prior Year Value Set billed by any provider type during the measurement year.
- Unilateral eye enucleation (Unilateral Eye Enucleation Value Set) ***with*** a bilateral modifier (Bilateral Modifier Value Set).
- Two unilateral eye enucleations (Unilateral Eye Enucleation Value Set) with service dates 14 days or more apart. For example, if the service date for the first unilateral eye enucleation was February 1 of the measurement year, the service date for the second unilateral eye enucleation must be on or after February 15.
- Left unilateral eye enucleation (Unilateral Eye Enucleation Left Value Set) ***and*** right unilateral eye enucleation (Unilateral Eye Enucleation Right Value Set) on the same or different dates of service.

- A unilateral eye enucleation (Unilateral Eye Enucleation Value Set) and a left unilateral eye enucleation (Unilateral Eye Enucleation Left Value Set) with service dates 14 days or more apart.
- A unilateral eye enucleation (Unilateral Eye Enucleation Value Set) and a right unilateral eye enucleation (Unilateral Eye Enucleation Right Value Set) with service dates 14 days or more apart.

Hybrid Specification

Denominator

A systematic sample drawn from the eligible population.

For Medicare reporting, the denominator for the Total Medicare SES stratification is the entire systematic sample. Do not pull samples for each stratification. The individual stratifications for the denominators and all numerators must sum to the total.

Organizations that use the Hybrid Method to report the Hemoglobin A1c Control for Patients With Diabetes (HBD), Eye Exam for Patients With Diabetes (EED) and Blood Pressure Control for Patients With Diabetes (BPD) measures may use the same sample for all three measures. If the same sample is used for the three diabetes measures, the organization must first take the inverse of the HbA1c poor control >9.0% rate (100 minus the HbA1c poor control rate) before reducing the sample.

Organizations may reduce the sample size based on the current year's administrative rate or the prior year's audited, product line-specific rate for the lowest rate of all HBD indicators and EED and BPD measures.

If separate samples are used for the HBD, EED and BPD measures, organizations may reduce the sample based on the product line-specific current measurement year's administrative rate or the prior year's audited, product line-specific rate for the measure.

Refer to the *Guidelines for Calculations and Sampling* for information on reducing sample size.

Numerator

Screening or monitoring for diabetic retinal disease as identified by administrative data or medical record review. This includes diabetics who had one of the following:

- A retinal or dilated eye exam by an eye care professional (optometrist or ophthalmologist) in the measurement year.
- A negative retinal or dilated exam (negative for retinopathy) by an eye care professional (optometrist or ophthalmologist) in the year prior to the measurement year.

Bilateral eye enucleation any time during the member's history through December 31 of the measurement year.

Administrative

Refer to *Administrative Specification* to identify positive numerator hits from administrative data.

Medical record At a minimum, documentation in the medical record must include one of the following:

- A note or letter prepared by an ophthalmologist, optometrist, PCP or other health care professional indicating that an ophthalmoscopic exam was completed by an eye care professional (optometrist or ophthalmologist), the date when the procedure was performed and the results.
- A chart or photograph indicating the date when the fundus photography was performed and one of the following:
  - Evidence that an eye care professional (optometrist or ophthalmologist) reviewed the results.
  - Evidence results were read by a qualified reading center that operates under the direction of a medical director who is a retinal specialist.

Evidence results were read by a system that provides an artificial intelligence (AI) interpretation.

- Evidence that the member had bilateral eye enucleation or acquired absence of both eyes. Look as far back as possible in the member's history through December 31 of the measurement year.
- Documentation of a negative retinal or dilated exam by an eye care professional (optometrist or ophthalmologist) in the year prior to the measurement year, where results indicate retinopathy was not present (e.g., documentation of normal findings).
  - Documentation does not have to state specifically "no diabetic retinopathy" to be considered negative for retinopathy; however, it must be clear that the patient had a dilated or retinal eye exam by an eye care professional (optometrist or ophthalmologist) and that retinopathy was not present. Notation limited to a statement that indicates "diabetes without complications" does not meet criteria.

Note

- *Blindness is not an exclusion for a diabetic eye exam because it is difficult to distinguish between individuals who are legally blind but require a retinal exam and those who are completely blind and therefore do not require an exam.*
- *Hypertensive retinopathy is not handled differently from diabetic retinopathy when reporting this measure; for example, an eye exam documented as positive for hypertensive retinopathy is counted as positive for diabetic retinopathy and an eye exam documented as negative for hypertensive retinopathy is counted as negative for diabetic retinopathy. The intent of this measure is to ensure that members with evidence of any type of retinopathy have an eye exam annually, while members who remain free of retinopathy (i.e., the retinal exam was negative for retinopathy) are screened every other year.*
- *An eye exam result documented as "unknown" does not meet criteria.*

Data Elements for Reporting

Organizations that submit HEDIS data to NCQA must provide the following data elements.

Table EED-1/2: Data Elements for Eye Exam for Patients With Diabetes

Metric	Data Element	Reporting Instructions	A
EyeExams	CollectionMethod	Report once	✓
	EligiblePopulation	Report once	✓
	ExclusionAdminRequired	Report once	✓
	NumeratorByAdminElig	Report once	
	CYAR	(Percent)	
	MinReqSampleSize	Report once	
	OversampleRate	Report once	
	OversampleRecordsNumber	(Count)	
	ExclusionValidDataErrors	Report once	
	ExclusionEmployeeOrDep	Report once	
	OversampleRecsAdded	Report once	
	Denominator	Report once	
	NumeratorByAdmin	Report once	✓
	NumeratorByMedicalRecords	Report once	
	NumeratorBySupplemental	Report once	✓
	Rate	(Percent)	✓

Table EED-3: Data Elements for Eye Exam for Patients With Diabetes

Metric	SES Stratification	Data Element	Reporting Instructions	A
EyeExams	NonLisDeNondisability	CollectionMethod	Repeat per Stratification	✓
	LisDe	EligiblePopulation	For each Stratification	✓
	Disability	ExclusionAdminRequired	For each Stratification	✓
	LisDeAndDisability	NumeratorByAdminElig	For each Stratification	
	Other	CYAR	Only for Total (Percent)	
	Unknown	MinReqSampleSize	Repeat per Stratification	
	Total	OversampleRate	Repeat per Stratification	
		OversampleRecordsNumber	(Count)	
		ExclusionValidDataErrors	Repeat per Stratification	
		ExclusionEmployeeOrDep	Repeat per Stratification	
		OversampleRecsAdded	Repeat per Stratification	
		Denominator	For each Stratification	
		NumeratorByAdmin	For each Stratification	✓
		NumeratorByMedicalRecords	For each Stratification	
		NumeratorBySupplemental	For each Stratification	✓
		Rate	(Percent)	✓

Rules for Allowable Adjustments of HEDIS

The "Rules for Allowable Adjustments of HEDIS" (the "Rules") describe how NCQA's HEDIS measure specifications can be adjusted for other populations, if applicable. The Rules, reviewed and approved by NCQA measure experts, provide for expanded use of HEDIS measures without changing their clinical intent.

Adjusted HEDIS measures may not be used for HEDIS health plan reporting.

Rules for Allowable Adjustments of Eye Exam for Patients With Diabetes

NONCLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Product lines	Yes	Organizations are not required to use product line criteria; product lines may be combined and all (or no) product line criteria may be used.
Ages	Yes, with limits	Age determination dates may be changed (e.g., select, "age as of June 30"). Changing denominator age range is allowed within a specified age range (ages 18–75 years). The denominator age may not be expanded.
Continuous enrollment, allowable gap, anchor date	Yes	Organizations are not required to use enrollment criteria; adjustments are allowed.
Benefits	Yes	Organizations are not required to use a benefit; adjustments are allowed.
Other	Yes	Organizations may use additional eligible population criteria to focus on an area of interest defined by gender, race, ethnicity, socioeconomic or sociodemographic characteristics, geographic region or another characteristic.
CLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Event/diagnosis	No	Only events or diagnoses that contain (or map to) codes in the medication lists and value sets may be used to identify visits. Medication lists, value sets and logic may not be changed.
Denominator Exclusions	**Adjustments Allowed (Yes/No)**	**Notes**
Required exclusions	Yes, with limits	Apply required exclusions according to specified value sets. The hospice, deceased member and palliative care exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments*.
Exclusions: I-SNP, LTI, frailty or advanced illness	Yes	These exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments*.
Numerator Criteria	**Adjustments Allowed (Yes/No)**	**Notes**
Eye Exam for Patients With Diabetes	No	Value sets and logic may not be changed.

*Kidney Health Evaluation for Patients With Diabetes (KED)**

***This measure was developed by NCQA with input from the National Kidney Foundation.**

SUMMARY OF CHANGES TO HEDIS MY 2023

- Revised the optional exclusions for polycystic ovarian syndrome, gestational diabetes or steroid-induced diabetes to be required exclusions.
- Added a required exclusion for members who died during the measurement year.
- Added a direct reference code for palliative care.
- Updated the number of occurrences required for the frailty cross-cutting exclusion.
- Revised the "Other" criteria in the Nonclinical Components table under *Rules for Allowable Adjustments of HEDIS.*
- Revised the "Required exclusions" criteria in the Clinical Components table under *Rules for Allowable Adjustments of HEDIS*.

Description

The percentage of members 18–85 years of age with diabetes (type 1 and type 2) who received a kidney health evaluation, defined by an estimated glomerular filtration rate (eGFR) ***and*** a urine albumin-creatinine ratio (uACR), during the measurement year.

Eligible Population

Product lines	Commercial, Medicaid, Medicare (report each product line separately).
Ages	18–85 years as of December 31 of the measurement year. Report three age stratifications and a total rate: • 18–64. • 75–85. • 65–74. • Total. The total is the sum of the age stratifications.
Continuous enrollment	The measurement year.
Allowable gap	No more than one gap in enrollment of up to 45 days during the measurement year. To determine continuous enrollment for a Medicaid beneficiary for whom enrollment is verified monthly, the member may not have more than a 1-month gap in coverage (e.g., a member whose coverage lapses for 2 months [60 days] is not considered continuously enrolled).
Anchor date	December 31 of the measurement year.
Benefit	Medical.
Event/diagnosis	There are two ways to identify members with diabetes: by claim/encounter data and by pharmacy data. The organization must use both methods to identify the eligible population, but a member only needs to be identified by one method to be included in the measure. Members may be identified as having diabetes during the measurement year or the year prior to the measurement year.

Claim/encounter data. Members who met any of the following criteria during the measurement year or the year prior to the measurement year (count services that occur over both years):

- At least one acute inpatient encounter (Acute Inpatient Value Set) with a diagnosis of diabetes (Diabetes Value Set) ***without*** telehealth (Telehealth Modifier Value Set; Telehealth POS Value Set).
- At least one acute inpatient discharge with a diagnosis of diabetes (Diabetes Value Set) on the discharge claim. To identify an acute inpatient discharge:
  1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
  2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
  3. Identify the discharge date for the stay.
- At least two outpatient visits (Outpatient Value Set), observation visits (Observation Value Set), telephone visits (Telephone Visits Value Set), e-visits or virtual check-ins (Online Assessments Value Set), ED visits (ED Value Set), nonacute inpatient encounters (Nonacute Inpatient Value Set) or nonacute inpatient discharges (instructions below; the diagnosis must be on the discharge claim), on different dates of service, with a diagnosis of diabetes (Diabetes Value Set). Visit type need not be the same for the two encounters. To identify a nonacute inpatient discharge:
  1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
  2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set) on the claim.
  3. Identify the discharge date for the stay.

  Only include nonacute inpatient encounters (Nonacute Inpatient Value Set) ***without*** telehealth (Telehealth Modifier Value Set; Telehealth POS Value Set).

Pharmacy data. Members who were dispensed insulin or hypoglycemics/antihyperglycemics during the measurement year or the year prior to the measurement year (Diabetes Medications List).

Diabetes Medications

Description	Prescription		
Alpha-glucosidase inhibitors	• Acarbose	• Miglitol	
Amylin analogs	• Pramlintide		
Antidiabetic combinations	• Alogliptin-metformin • Alogliptin-pioglitazone • Canagliflozin-metformin • Dapagliflozin-metformin • Dapagliflozin-saxagliptin • Empagliflozin-linagliptin • Empagliflozin-linagliptin-metformin	• Empagliflozin-metformin • Ertugliflozin-metformin • Ertugliflozin-sitagliptin • Glimepiride-pioglitazone • Glipizide-metformin • Glyburide-metformin • Linagliptin-metformin	• Metformin-pioglitazone • Metformin-repaglinide • Metformin-rosiglitazone • Metformin-saxagliptin • Metformin-sitagliptin

Description	Prescription		
Insulin	• Insulin aspart • Insulin aspart-insulin aspart protamine • Insulin degludec • Insulin degludec-liraglutide • Insulin detemir • Insulin glargine • Insulin glargine-lixisenatide	• Insulin glulisine • Insulin isophane human • Insulin isophane-insulin regular • Insulin lispro • Insulin lispro-insulin lispro protamine • Insulin regular human • Insulin human inhaled	
Meglitinides	• Nateglinide	• Repaglinide	
Glucagon-like peptide-1 (GLP1) agonists	• Albiglutide • Dulaglutide • Exenatide	• Liraglutide (excluding Saxenda®) • Lixisenatide • Semaglutide	
Sodium glucose cotransporter 2 (SGLT2) inhibitor	• Canagliflozin • Dapagliflozin (excluding Farxiga®)	• Ertugliflozin • Empagliflozin	
Sulfonylureas	• Chlorpropamide • Glimepiride	• Glipizide • Glyburide	• Tolazamide • Tolbutamide
Thiazolidinediones	• Pioglitazone	• Rosiglitazone	
Dipeptidyl peptidase-4 (DDP-4) inhibitors	• Alogliptin • Linagliptin	• Saxagliptin • Sitagliptin	

Note: *Glucophage/metformin as a solo agent is not included because it is used to treat conditions other than diabetes; members with diabetes on these medications are identified through diagnosis codes only.*

Required exclusions

Exclude members who meet any of the following criteria:

- Members who did not have a diagnosis of diabetes (Diabetes Value Set), in any setting, during the measurement year or the year prior to the measurement year ***and*** who had a diagnosis of polycystic ovarian syndrome, gestational diabetes or steroid-induced diabetes (Diabetes Exclusions Value Set), in any setting, during the measurement year or the year prior to the measurement year.
- Members with evidence of ESRD (ESRD Diagnosis Value Set) or dialysis (Dialysis Procedure Value Set) any time during the member's history on or prior to December 31 of the measurement year.
- Members in hospice or using hospice services any time during the measurement year. Refer to *General Guideline 15: Members in Hospice.*
- Members who died any time during the measurement year. Refer to *General Guideline 16: Deceased Members.*
- Members receiving palliative care (Palliative Care Assessment Value Set; Palliative Care Encounter Value Set; Palliative Care Intervention Value Set; ICD-10-CM code Z51.5) any time during the measurement year.

Exclusions

Exclude members who meet any of the following criteria:

Note: *Supplemental and medical record data may not be used for these exclusions.*

- Medicare members 66 years of age and older as of December 31 of the measurement year who meet either of the following:
  Enrolled in an Institutional SNP (I-SNP) any time during the measurement year.
  Living long-term in an institution any time during the measurement year as identified by the LTI flag in the Monthly Membership Detail Data File. Use the run date of the file to determine if a member had an LTI flag during the measurement year.
- Members 66-80 years of age as of December 31 of the measurement year (all product lines) with frailty ***and*** advanced illness. Members must meet **BOTH** of the following frailty and advanced illness criteria to be excluded:
  1. At least two indications of frailty (Frailty Device Value Set; Frailty Diagnosis Value Set; Frailty Encounter Value Set; Frailty Symptom Value Set) with different dates of service during the measurement year.
  2. Any of the following during the measurement year or the year prior to the measurement year (count services that occur over both years):
     At least two outpatient visits (Outpatient Value Set), observation visits (Observation Value Set), ED visits (ED Value Set), telephone visits (Telephone Visits Value Set), e-visits or virtual check-ins (Online Assessments Value Set), nonacute inpatient encounters (Nonacute Inpatient Value Set) or nonacute inpatient discharges (instructions below; the diagnosis must be on the discharge claim) on different dates of service, with an advanced illness diagnosis (Advanced Illness Value Set). Visit type need not be the same for the two visits. To identify a nonacute inpatient discharge:
     1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
     2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set) on the claim.
     3. Identify the discharge date for the stay.

     At least one acute inpatient encounter (Acute Inpatient Value Set) with an advanced illness diagnosis (Advanced Illness Value Set).
     At least one acute inpatient discharge with an advanced illness diagnosis (Advanced Illness Value Set) on the discharge claim. To identify an acute inpatient discharge:
     1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
     2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
     3. Identify the discharge date for the stay.

     A dispensed dementia medication (Dementia Medications List).
- Members 81 years of age and older as of December 31 of the measurement year (all product lines) with at least two indications of frailty (Frailty Device Value Set; Frailty Diagnosis Value Set; Frailty Encounter Value Set; Frailty Symptom Value Set) with different dates of service during the measurement year.

Dementia Medications

Description	Prescription
Cholinesterase inhibitors	• Donepezil • Galantamine • Rivastigmine
Miscellaneous central nervous system agents	• Memantine
Dementia combinations	• Donepezil-memantine

Administrative Specification

Denominator The eligible population.

Numerator

Kidney Health Evaluation Members who received ***both*** an eGFR and a uACR during the measurement year on the same or different dates of service:

- At least one eGFR (Estimated Glomerular Filtration Rate Lab Test Value Set).
- At least one uACR identified by either of the following:
  - ***Both*** a quantitative urine albumin test (Quantitative Urine Albumin Lab Test Value Set) ***and*** a urine creatinine test (Urine Creatinine Lab Test Value Set) ***with*** service dates four days or less apart. For example, if the service date for the quantitative urine albumin test was December 1 of the measurement year, then the urine creatinine test must have a service date on or between November 27 and December 5 of the measurement year.
  - A uACR (Urine Albumin Creatinine Ratio Lab Test Value Set).

Data Elements for Reporting

Organizations that submit HEDIS data to NCQA must provide the following data elements.

Table KED-1/2/3: Data Elements for Kidney Health Evaluation for Patients With Diabetes

Metric	Age	Data Element	Reporting Instructions
KidneyHealthEvaluation	18-64	EligiblePopulation	For each Stratification
	65-74	ExclusionAdminRequired	For each Stratification
	75-85	NumeratorByAdmin	For each Stratification
	Total	NumeratorBySupplemental	For each Stratification
		Rate	(Percent)

Rules for Allowable Adjustments of HEDIS

The "Rules for Allowable Adjustments of HEDIS" (the "Rules") describe how NCQA's HEDIS measure specifications can be adjusted for other populations, if applicable. The Rules, reviewed and approved by NCQA measure experts, provide for expanded use of HEDIS measures without changing their clinical intent.

Adjusted HEDIS measures *may not* be used for HEDIS health plan reporting.

Rules for Allowable Adjustments of Kidney Health Evaluation for Patients With Diabetes

NONCLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Product lines	Yes	Organizations are not required to use product line criteria; product lines may be combined and all (or no) product line criteria may be used.
Ages	Yes, with limits	Age determination dates may be changed (e.g., select, "age as of June 30"). The denominator age may be changed if the range is within the specified age range (18–85 years).
Continuous enrollment, allowable gap, anchor date	Yes	Organizations are not required to use enrollment criteria; adjustments are allowed.
Benefit	Yes	Organizations are not required to use a benefit; adjustments are allowed.
Other	Yes	Organizations may use additional eligible population criteria to focus on an area of interest defined by gender, race, ethnicity, socioeconomic or sociodemographic characteristics, geographic region or another characteristic.
CLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Event/diagnosis	No	Only events or diagnoses that contain (or map to) codes in the medication lists and value sets may be used to identify visits. Medication lists, value sets and logic may not be changed.
Denominator Exclusions	**Adjustments Allowed (Yes/No)**	**Notes**
Required exclusions	Yes, with limits	Apply required exclusions according to specified value sets. The hospice, deceased member and palliative care exclusions are not required. Refer to Exclusions in the Guidelines for the Rules for Allowable Adjustments.
Exclusions: I-SNP, LTI, frailty or advanced illness	Yes	These exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments*.
Numerator Criteria	**Adjustments Allowed (Yes/No)**	**Notes**
Kidney Health Evaluation	No	Value sets and logic may not be changed.

Statin Therapy for Patients With Diabetes (SPD)

SUMMARY OF CHANGES TO HEDIS MY 2023

- Revised the optional exclusions for polycystic ovarian syndrome, gestational diabetes or steroid-induced diabetes to be required exclusions.
- Replaced the reference to “female members” with “members” in the required exclusions.
- Added a required exclusion for members who died during the measurement year.
- Simplified the required exclusion language for cardiovascular events and diagnoses.
- Added a direct reference code for palliative care.
- Updated the number of occurrences required for the frailty cross-cutting exclusion.
- Revised the “Other” criteria in the Nonclinical Components table under *Rules for Allowable Adjustments of HEDIS.*
- Revised the “Exclusions” criteria in the Clinical Components table under *Rules for Allowable Adjustments of HEDIS*.
- Removed the *Note* from the “Event/diagnosis” criteria in the Clinical Components table under *Rules for Allowable Adjustments of HEDIS*.

Description

The percentage of members 40–75 years of age during the measurement year with diabetes who do not have clinical atherosclerotic cardiovascular disease (ASCVD) who met the following criteria. Two rates are reported:

22. *Received Statin Therapy.* Members who were dispensed at least one statin medication of any intensity during the measurement year.
23. *Statin Adherence 80%.* Members who remained on a statin medication of any intensity for at least 80% of the treatment period.

Definitions

IPSD	Index prescription start date. The earliest prescription dispensing date for any statin medication of any intensity during the measurement year.
Treatment period	The period of time beginning on the IPSD through the last day of the measurement year.
PDC	Proportion of days covered. The number of days the member is covered by at least one statin medication prescription of appropriate intensity, divided by the number of days in the treatment period.
Calculating number of days covered for multiple prescriptions	If multiple prescriptions for different medications are dispensed on the same day, calculate number of days covered by a statin medication (for the numerator) using the prescriptions with the longest days supply. For multiple different prescriptions dispensed on different days with overlapping days supply, count each day within the treatment period only once toward the numerator.

If multiple prescriptions for the same medication are dispensed on the same or different day, sum the days supply and use the total to calculate the number of days covered by a statin medication (for the numerator). For example, three prescriptions for the same medication are dispensed on the same day, each with a 30-days supply, sum the days supply for a total of 90 days covered by a statin. Subtract any days supply that extends beyond December 31 of the measurement year.

Use the medication lists to determine if drugs are the same or different. Drugs in different lists are considered different drugs. For example, a dispensing event from the Amlodipine Atorvastatin High Intensity Medications List and a dispensing event from the Amlodipine Atorvastatin Moderate Intensity Medications List are dispensing events for different medications.

Eligible Population: *Rate 1*—Received Statin Therapy

Product lines	Commercial, Medicaid, Medicare (report each product line separately).
Ages	40–75 years as of December 31 of the measurement year.
Continuous enrollment	The measurement year and the year prior to the measurement year.
Allowable gap	No more than one gap in enrollment of up to 45 days during each year of continuous enrollment. To determine continuous enrollment for a Medicaid beneficiary for whom enrollment is verified monthly, the member may not have more than a 1-month gap in coverage (e.g., a member whose coverage lapses for 2 months [60 days] is not considered continuously enrolled).
Anchor date	December 31 of the measurement year.
Benefit	Medical. Pharmacy during the measurement year.
Event/diagnosis	There are two ways to identify members with diabetes: by claim/encounter data and by pharmacy data. The organization must use both methods to identify the eligible population, but a member only needs to be identified by one method to be included in the measure. Members may be identified as having diabetes during the measurement year or the year prior to the measurement year.

Claim/encounter data. Members who met any of the following criteria during the measurement year or the year prior to the measurement year (count services that occur over both years):

- At least one acute inpatient encounter (Acute Inpatient Value Set) with a diagnosis of diabetes (Diabetes Value Set) ***without*** telehealth (Telehealth Modifier Value Set; Telehealth POS Value Set).
- At least one acute inpatient discharge with a diagnosis of diabetes (Diabetes Value Set) on the discharge claim. To identify an acute inpatient discharge:
  1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
  2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
  3. Identify the discharge date for the stay.

- At least two outpatient visits (Outpatient Value Set), observation visits (Observation Value Set), telephone visits (Telephone Visits Value Set), e-visits or virtual check-ins (Online Assessments Value Set), ED visits (ED Value Set), nonacute inpatient encounters (Nonacute Inpatient Value Set) or nonacute inpatient discharges (instructions below; the diagnosis must be on the discharge claim), on different dates of service, with a diagnosis of diabetes (Diabetes Value Set). Visit type need not be the same for the two encounters. To identify a nonacute inpatient discharge:
  1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
  2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set) on the claim.
  3. Identify the discharge date for the stay.

  Only include nonacute inpatient encounters (Nonacute Inpatient Value Set) ***without*** telehealth (Telehealth Modifier Value Set; Telehealth POS Value Set).

Pharmacy data. Members who were dispensed insulin or hypoglycemics/antihyperglycemics during the measurement year or the year prior to the measurement year (Diabetes Medications List).

Diabetes Medications

Description	Prescription		
Alpha-glucosidase inhibitors	• Acarbose	• Miglitol	
Amylin analogs	• Pramlintide		
Antidiabetic combinations	• Alogliptin-metformin • Alogliptin-pioglitazone • Canagliflozin-metformin • Dapagliflozin-metformin • Dapagliflozin-saxagliptin • Empagliflozin-linagliptin • Empagliflozin-linagliptin-metformin	• Empagliflozin-metformin • Ertugliflozin-metformin • Ertugliflozin-sitagliptin • Glimepiride-pioglitazone • Glipizide-metformin • Glyburide-metformin • Linagliptin-metformin	• Metformin-pioglitazone • Metformin-repaglinide • Metformin-rosiglitazone • Metformin-saxagliptin • Metformin-sitagliptin
Insulin	• Insulin aspart • Insulin aspart-insulin aspart protamine • Insulin degludec • Insulin degludec-liraglutide • Insulin detemir • Insulin glargine • Insulin glargine-lixisenatide	• Insulin glulisine • Insulin isophane human • Insulin isophane-insulin regular • Insulin lispro • Insulin lispro-insulin lispro protamine • Insulin regular human • Insulin human inhaled	
Meglitinides	• Nateglinide	• Repaglinide	
Glucagon-like peptide-1 (GLP1) agonists	• Albiglutide • Dulaglutide • Exenatide	• Lixisenatide • Liraglutide (excluding Saxenda®) • Semaglutide	

Description	Prescription		
Sodium glucose cotransporter 2 (SGLT2) inhibitor	• Canagliflozin • Dapagliflozin (excluding Farxiga®)	• Empagliflozin • Ertugliflozin	
Sulfonylureas	• Chlorpropamide • Glimepiride	• Glipizide • Glyburide	• Tolazamide • Tolbutamide
Thiazolidinediones	• Pioglitazone	• Rosiglitazone	
Dipeptidyl peptidase-4 (DDP-4) inhibitors	• Alogliptin • Linagliptin	• Saxagliptin • Sitaglipin	

Note: *Glucophage/metformin as a solo agent is not included because it is used to treat conditions other than diabetes; members with diabetes on these medications are identified through diagnosis codes only.*

Required exclusions

Exclude members who meet any of the following criteria:

- Members who did not have a diagnosis of diabetes (Diabetes Value Set), in any setting, during the measurement year or the year prior to the measurement year ***and*** who had a diagnosis of polycystic ovarian syndrome, gestational diabetes or steroid-induced diabetes (Diabetes Exclusions Value Set), in any setting, during the measurement year or the year prior to the measurement year.
- Members with at least one of the following during the year prior to the measurement year:
  - *MI.* Discharged from an inpatient setting with an MI (MI Value Set; Old Myocardial Infarction Value Set) on the discharge claim. To identify discharges:
    1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
    2. Identify the discharge date for the stay.
  - *CABG.* Members who had CABG (CABG Value Set) in any setting.
  - *PCI.* Members who had PCI (PCI Value Set) in any setting.
  - *Other revascularization.* Members who had any other revascularization procedure (Other Revascularization Value Set) in any setting.
- Members who met at least one of the following criteria during both the measurement year and the year prior to the measurement year. Criteria need not be the same across both years.
  - At least one outpatient visit (Outpatient Value Set) with an IVD diagnosis (IVD Value Set).
  - A telephone visit (Telephone Visits Value Set) with an IVD diagnosis (IVD Value Set).
  - An e-visit or virtual check-in (Online Assessments Value Set) with an IVD diagnosis (IVD Value Set).
  - At least one acute inpatient encounter (Acute Inpatient Value Set) with an IVD diagnosis (IVD Value Set) ***without*** telehealth (Telehealth Modifier Value Set; Telehealth POS Value Set).
  - At least one acute inpatient discharge with an IVD diagnosis (IVD Value Set) on the discharge claim. To identify an acute inpatient discharge:

1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
3. Identify the discharge date for the stay.

- Members with a diagnosis of pregnancy (Pregnancy Value Set) during the measurement year or year prior to the measurement year.
- In vitro fertilization (IVF Value Set) in the measurement year or year prior to the measurement year.
- Dispensed at least one prescription for clomiphene (Estrogen Agonists Medications List) during the measurement year or the year prior to the measurement year.
- ESRD (ESRD Diagnosis Value Set) or dialysis (Dialysis Procedure Value Set) during the measurement year or the year prior to the measurement year.
- Cirrhosis (Cirrhosis Value Set) during the measurement year or the year prior to the measurement year.
- Myalgia, myositis, myopathy or rhabdomyolysis (Muscular Pain and Disease Value Set) during the measurement year.
- Members in hospice or using hospice services any time during the measurement year. Refer to *General Guideline 15: Members in Hospice*.
- Members who died any time during the measurement year. Refer to *General Guideline 16: Deceased Members.*
- Members receiving palliative care (Palliative Care Assessment Value Set; Palliative Care Encounter Value Set; Palliative Care Intervention Value Set; ICD-10-CM code Z51.5) any time during the measurement year.

Estrogen Agonists Medications

Description	Prescription
Estrogen agonists	• Clomiphene

Exclusions

Exclude members who meet any of the following criteria:

Note: *Supplemental and medical record data may not be used for these exclusions.*

- Medicare members 66 years of age and older as of December 31 of the measurement year who meet either of the following:
  - Enrolled in an Institutional SNP (I-SNP) any time during the measurement year.
  - Living long-term in an institution any time during the measurement year as identified by the LTI flag in the Monthly Membership Detail Data File. Use the run date of the file to determine if a member had an LTI flag during the measurement year.
- Members 66 years of age and older as of December 31 of the measurement year (all product lines) with frailty ***and*** advanced illness. Members must meet ***both*** of the following frailty and advanced illness criteria to be excluded:

1. At least two indications of frailty (Frailty Device Value Set; Frailty Diagnosis Value Set; Frailty Encounter Value Set; Frailty Symptom Value Set) with different dates of service during the measurement year.
2. Any of the following during the measurement year or the year prior to the measurement year (count services that occur over both years):
   - At least two outpatient visits (Outpatient Value Set), observation visits (Observation Value Set), ED visits (ED Value Set), telephone visits (Telephone Visits Value Set), e-visits or virtual check-ins (Online Assessments Value Set), nonacute inpatient encounters (Nonacute Inpatient Value Set) or nonacute inpatient discharges (instructions below; the diagnosis must be on the discharge claim) on different dates of service, with an advanced illness diagnosis (Advanced Illness Value Set). Visit type need not be the same for the two visits. To identify a nonacute inpatient discharge:
     1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
     2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set) on the claim.
     3. Identify the discharge date for the stay.
   - At least one acute inpatient encounter (Acute Inpatient Value Set) with an advanced illness diagnosis (Advanced Illness Value Set).
   - At least one acute inpatient discharge with an advanced illness diagnosis (Advanced Illness Value Set) on the discharge claim. To identify an acute inpatient discharge:
     1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
     2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
     3. Identify the discharge date for the stay.
   - A dispensed dementia medication (Dementia Medications List).

Dementia Medications

Description	Prescription
Cholinesterase inhibitors	• Donepezil • Galantamine • Rivastigmine
Miscellaneous central nervous system agents	• Memantine
Dementia combinations	• Donepezil-memantine

Administrative Specification: *Rate 1*—Received Statin Therapy

Denominator The Rate 1 eligible population.

Numerator The number of members who had at least one dispensing event for a high-intensity, moderate intensity, or low-intensity statin medication during the measurement year. Use all the medication lists below to identify statin medication dispensing events.

High, Moderate and Low-Intensity Statin Medications

Description	Prescription	Medication Lists
High-intensity statin therapy	Atorvastatin 40-80 mg	Atorvastatin High Intensity Medications List
High-intensity statin therapy	Amlodipine-atorvastatin 40-80 mg	Amlodipine Atorvastatin High Intensity Medications List
High-intensity statin therapy	Rosuvastatin 20-40 mg	Rosuvastatin High Intensity Medications List
High-intensity statin therapy	Simvastatin 80 mg	Simvastatin High Intensity Medications List
High-intensity statin therapy	Ezetimibe-simvastatin 80 mg	Ezetimibe Simvastatin High Intensity Medications List
Moderate-intensity statin therapy	Atorvastatin 10-20 mg	Atorvastatin Moderate Intensity Medications List
Moderate-intensity statin therapy	Amlodipine-atorvastatin 10-20 mg	Amlodipine Atorvastatin Moderate Intensity Medications List
Moderate-intensity statin therapy	Rosuvastatin 5-10 mg	Rosuvastatin Moderate Intensity Medications List
Moderate-intensity statin therapy	Simvastatin 20-40 mg	Simvastatin Moderate Intensity Medications List
Moderate-intensity statin therapy	Ezetimibe-simvastatin 20-40 mg	Ezetimibe Simvastatin Moderate Intensity Medications List
Moderate-intensity statin therapy	Pravastatin 40-80 mg	Pravastatin Moderate Intensity Medications List
Moderate-intensity statin therapy	Lovastatin 40 mg	Lovastatin Moderate Intensity Medications List
Moderate-intensity statin therapy	Fluvastatin 40-80 mg	Fluvastatin Moderate Intensity Medications List
Moderate-intensity statin therapy	Pitavastatin 1–4 mg	Pitavastatin Moderate Intensity Medications List
Low-intensity statin therapy	Ezetimibe-simvastatin 10 mg	Ezetimibe Simvastatin Low Intensity Medications List
Low-intensity statin therapy	Fluvastatin 20 mg	Fluvastatin Low Intensity Medications List
Low-intensity statin therapy	Lovastatin 10-20 mg	Lovastatin Low Intensity Medications List
Low-intensity statin therapy	Pravastatin 10–20 mg	Pravastatin Low Intensity Medications List
Low-intensity statin therapy	Simvastatin 5-10 mg	Simvastatin Low Intensity Medications List

Eligible Population: *Rate 2*—Statin Adherence 80%

Product lines	Commercial, Medicaid, Medicare (report each product line separately).
Age	40–75 years as of December 31 of the measurement year.
Continuous enrollment	The measurement year and the year prior to the measurement year.
Allowable gap	No more than one gap in enrollment of up to 45 days during the measurement year. To determine continuous enrollment for a Medicaid beneficiary for whom enrollment is verified monthly, the member may not have more than a 1-month gap in coverage (e.g., a member whose coverage lapses for 2 months [60 days] is not considered continuously enrolled).
Anchor date	December 31 of the measurement year.
Benefit	Medical. Pharmacy during the measurement year.
Event/diagnosis	All members who meet the numerator criteria for Rate 1.

Administrative Specification: *Rate 2*—Statin Adherence 80%

Denominator The Rate 2 eligible population.

Numerator The number of members who achieved a PDC of at least 80% during the treatment period.

Follow the steps below to identify numerator compliance.

Step 1 Identify the IPSD. The IPSD is the earliest dispensing event for any high-intensity, moderate-intensity or low-intensity statin medication during the measurement year. Use all the medication lists above to identify statin medication dispensing events.

Step 2 To determine the treatment period, calculate the number of days beginning on the IPSD through the end of the measurement year.

Step 3 Count the days covered by at least one prescription for any high-intensity, moderate-intensity or low-intensity statin medication during the treatment period. To ensure the measure does not give credit for supply that extends beyond the measurement year, subtract any days supply that extends beyond December 31 of the measurement year.

Step 4 Calculate the member's PDC using the following equation. Multiply the equation by 100 and round (using the .5 rule) to the nearest whole number. For example, if a member has 291 total days covered by a medication during a 365-day treatment period, this calculates to 0.7972. Multiply this number by 100, convert it to 79.72% and round it to 80%, the nearest whole number.

$$\frac{\text{Total Days Covered by a Statin Medication in the Treatment Period (step 3)}}{\text{Total Days in Treatment Period (step 2)}}$$

Step 5 Sum the number of members whose PDC is ≥80% for the treatment period.

Note

- *All members who are numerator compliant for Rate 1 must be used as the eligible population for Rate 2 (regardless of the data source used to capture the Rate 1 numerator). For example, if supplemental data were used to identify compliance for the Rate 1 numerator, then supplemental data will be included in identifying the Rate 2 eligible population.*

Data Elements for Reporting

Organizations that submit HEDIS data to NCQA must provide the following data elements.

Table SPD-1/2/3: Data Elements for Statin Therapy for Patients With Diabetes

Metric	Data Element	Reporting Instructions
ReceivedTherapy	Benefit	Metadata
Adherence	EligiblePopulation	For each Metric
	ExclusionAdminRequired	Only for ReceivedTherapy Metric
	NumeratorByAdmin	For each Metric
	NumeratorBySupplemental	For each Metric
	Rate	(Percent)

Rules for Allowable Adjustments of HEDIS

The "Rules for Allowable Adjustments of HEDIS" (the "Rules") describe how NCQA's HEDIS measure specifications can be adjusted for other populations, if applicable. The Rules, reviewed and approved by NCQA measure experts, provide for expanded use of HEDIS measures without changing their clinical intent.

Adjusted HEDIS measures *may not* be used for HEDIS health plan reporting.

Rules for Allowable Adjustments of Statin Therapy for Patients With Diabetes

NONCLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Product lines	Yes	Organizations are not required to use product line criteria; product lines may be combined and all (or no) product line criteria may be used.
Ages	Yes, with limits	Age determination dates may be changed (e.g., select, "age as of June 30"). Changing the denominator age range is allowed within the specified age range (ages 40–75 years). The denominator age may not be expanded.
Continuous enrollment, allowable gap, anchor date	Yes	Organizations are not required to use enrollment criteria; adjustments are allowed.
Benefits	Yes	Organizations are not required to use a benefit; adjustments are allowed.
Other	Yes	Organizations may use additional eligible population criteria to focus on an area of interest defined by gender, race, ethnicity, socioeconomic or sociodemographic characteristics, geographic region or another characteristic.
CLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Event/diagnosis	No	Only events or diagnoses that contain (or map to) codes in the medication lists and value sets may be used to identify visits. Medication lists, value sets and logic may not be changed.
Denominator Exclusions	**Adjustments Allowed (Yes/No)**	**Notes**
Required Exclusions	Yes, with limits	Apply required exclusions according to specified medication lists and value sets. The hospice, deceased member and palliative care exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments.*
Exclusions: I-SNP, LTI, frailty or advanced illness	Yes	These exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments*.

Numerator Criteria	Adjustments Allowed (Yes/No)	Notes
• Rate 1: Received Statin Therapy • Rate 2: Statin Adherence 80%	No	Medication lists, value sets and logic may not be changed.

Musculoskeletal Conditions

Osteoporosis Management in Women Who Had a Fracture (OMW)

SUMMARY OF CHANGES TO HEDIS MY 2023

- Added a required exclusion for members who died during the measurement year.
- Added a direct reference code for palliative care.
- Updated the number of occurrences required for the frailty cross-cutting exclusion.
- Revised the "Other" criteria in the Nonclinical Components table under *Rules for Allowable Adjustments of HEDIS.*
- Revised the "Required exclusions" criteria in the Clinical Components table under *Rules for Allowable Adjustments of HEDIS*.

Description

The percentage of women 67–85 years of age who suffered a fracture and who had either a bone mineral density (BMD) test or prescription for a drug to treat osteoporosis in the six months after the fracture.

Definitions

Intake period

A 12-month (1 year) window that begins on July 1 of the year prior to the measurement year and ends on June 30 of the measurement year. The intake period is used to capture the first fracture.

Episode date

The date of service for an eligible encounter during the intake period with a diagnosis of fracture.

For an outpatient, observation or ED visit, *the episode date is the date of service.*

For an inpatient stay, *the episode date is the date of discharge.*

For direct transfers, *the episode date is the discharge date from the last admission.*

IESD

Index episode start date. The earliest episode date during the intake period that meets all eligible population criteria.

Direct transfer

A **direct transfer** is when the discharge date from the first inpatient setting precedes the admission date to a second inpatient setting by one calendar day or less. For example:

- An inpatient discharge on June 1, followed by an admission to another inpatient setting on June 1, *is a direct transfer.*
- An inpatient discharge on June 1, followed by an admission to an inpatient setting on June 2, *is a direct transfer.*
- An inpatient discharge on June 1, followed by an admission to another inpatient setting on June 3, *is not a direct transfer;* these are two distinct inpatient stays.

Use the following method to identify admissions to and discharges from inpatient settings.

1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
2. Identify the admission and discharge dates for the stay.

Note: *The direct transfer does not require a fracture diagnosis.*

Active prescription A prescription is considered active if the "days supply" indicated on the date when the member was dispensed the prescription is the number of days or more between that date and the relevant service date.

Eligible Population

Product line Medicare.

Age Women 67–85 years as of December 31 of the measurement year.

Continuous enrollment 12 months (1 year) before the episode date through 180 days (6 months) after the episode date.

Allowable gap No more than one gap in enrollment of up to 45 days during the continuous enrollment period.

Anchor date Episode date.

Benefits Medical and pharmacy.

Event/diagnosis Follow the steps below to identify the eligible population.

Step 1 Identify all members who had either of the following during the intake period.

- An outpatient visit (Outpatient Value Set), an observation visit (Observation Value Set) or an ED visit (ED Value Set), for a fracture (Fractures Value Set) ***without*** (Telehealth Modifier Value Set; Telehealth POS Value Set).
  - Do not include visits that result in an inpatient stay (Inpatient Stay Value Set).
- An acute or nonacute inpatient discharge with a fracture (Fractures Value Set) on the discharge claim. To identify acute and nonacute inpatient discharges:
  1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
  2. Identify the discharge date for the stay.

If the member had more than one fracture, identify all fractures and assess eligibility in steps 2–4.

Step 2 Test for negative diagnosis history. Remove fractures where either of the following occurred during the 60-day (2 months) period prior to the episode date:

- An outpatient visit (Outpatient Value Set), a telephone visit (Telephone Visits Value Set), an e-visit or virtual check-in (Online Assessments Value Set), an observation visit (Observation Value Set) or an ED visit (ED Value Set) for a fracture (Fractures Value Set).

  - Do not include visits that result in an inpatient stay (Inpatient Stay Value Set).
- An acute or nonacute inpatient discharge with a fracture (Fractures Value Set) on the discharge claim. To identify acute and nonacute inpatient discharges:
  1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
  2. Identify the discharge date for the stay.

For an acute or nonacute inpatient episode, use the date of admission to determine the 60-day period.

For episodes that were direct transfers, use the first admission to determine the negative diagnosis history.

For inpatient stay episodes that were a result of an outpatient, ED or observation visit, use the date of the outpatient, ED or observation visit to determine negative diagnosis history.

Step 3 Calculate continuous enrollment. Members must be continuously enrolled during the 12 months prior to the episode date through 180 days (6 months) post-episode date.

Step 4: Required exclusions Exclude episode dates where any of the following are met:

- Members who had a BMD test (Bone Mineral Density Tests Value Set) during the 730 days (24 months) prior to the episode date.
- Members who had a claim/encounter for osteoporosis therapy (Osteoporosis Medication Therapy Value Set) during the 365 days (12 months) prior to the episode date.
- Members who received a dispensed prescription or had an active prescription to treat osteoporosis (Osteoporosis Medications List) during the 365 days (12 months) prior to the episode date.
- Members in hospice or using hospice services any time during the measurement year. Refer to *General Guideline 15: Members in Hospice*.
- Members who died any time during the measurement year. Refer to *General Guideline 16: Deceased Members.*
- Members who received palliative care (Palliative Care Assessment Value Set; Palliative Care Encounter Value Set; Palliative Care Intervention Value Set; ICD-10-CM code Z51.5) any time during the intake period through the end of the measurement year.

For an acute or nonacute inpatient event, use the date of admission to identify the days prior to the episode date.

For direct transfers, use the first admission date to identify the days prior to the episode date.

For outpatient, ED and observation visits that result in an inpatient stay, use the date of the outpatient, ED or observation visit to identify the days prior to the episode Date.

Step 5 Select the IESD. The measure examines the earliest eligible episode per member that meets the criteria above.

Step 6: Exclusions

Exclude members who meet any of the following criteria:

Note: *Supplemental and medical record data may not be used for these exclusions.*

- Members 67 years of age and older as of December 31 of the measurement year who meet either of the following:
  - Enrolled in an Institutional SNP (I-SNP) any time during the intake period through the end of the measurement year.
  - Living long-term in an institution any time during the intake period through the end of the measurement year as identified by the LTI flag in the Monthly Membership Detail Data File. Use the run date of the file to determine if a member had an LTI flag during the intake period through the end of the measurement year.
- Members 67–80 years of age as of December 31 of the measurement year (all product lines) with frailty ***and*** advanced illness. Members must meet ***BOTH*** of the following frailty and advanced illness criteria to be excluded:
  1. At least two indications of frailty (Frailty Device Value Set; Frailty Diagnosis Value Set; Frailty Encounter Value Set; Frailty Symptom Value Set) with different dates of service during the intake period through the end of the measurement year.
  2. Any of the following during the measurement year or the year prior to the measurement year (count services that occur over both years):
     - At least two outpatient visits (Outpatient Value Set), observation visits (Observation Value Set), ED visits (ED Value Set), telephone visits (Telephone Visits Value Set), e-visits or virtual check-ins (Online Assessments Value Set), nonacute inpatient encounters (Nonacute Inpatient Value Set) or nonacute inpatient discharges (instructions below; the diagnosis must be on the discharge claim) on different dates of service, with an advanced illness diagnosis (Advanced Illness Value Set). Visit type need not be the same for the two visits. To identify a nonacute inpatient discharge:
       1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
       2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set) on the claim.
       3. Identify the discharge date for the stay.
     - At least one acute inpatient encounter (Acute Inpatient Value Set) with an advanced illness diagnosis (Advanced Illness Value Set).
     - At least one acute inpatient discharge with an advanced illness diagnosis (Advanced Illness Value Set) on the discharge claim. To identify an acute inpatient discharge:
       1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
       2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
       3. Identify the discharge date for the stay.
     - A dispensed dementia medication (Dementia Medications List).

- Members 81 years of age and older as of December 31 of the measurement year (all product lines) with at least two indications of frailty (Frailty Device Value Set; Frailty Diagnosis Value Set; Frailty Encounter Value Set; Frailty Symptom Value Set) with different dates of service during the intake period through the end of the measurement year.

Dementia Medications

Description	Prescription
Cholinesterase inhibitors	• Donepezil • Galantamine • Rivastigmine
Miscellaneous central nervous system agents	• Memantine
Dementia combinations	• Donepezil-memantine

Administrative Specification

Denominator The eligible population.

Numerator Appropriate testing or treatment for osteoporosis after the fracture defined by any of the following criteria:

- A BMD test (Bone Mineral Density Tests Value Set), in any setting, on the IESD or in the 180-day (6-month) period after the IESD.
- If the IESD was an inpatient stay, a BMD test (Bone Mineral Density Tests Value Set) during the inpatient stay.
- Osteoporosis therapy (Osteoporosis Medication Therapy Value Set) on the IESD or in the 180-day (6-month) period after the IESD.
- If the IESD was an inpatient stay, long-acting osteoporosis therapy (Long-Acting Osteoporosis Medications Value Set) during the inpatient stay.
- A dispensed prescription to treat osteoporosis (Osteoporosis Medications List) on the IESD or in the 180-day (6-month) period after the IESD.

Osteoporosis Medications

Description	Prescription
Bisphosphonates	• Alendronate • Alendronate-cholecalciferol • Ibandronate • Risedronate • Zoledronic acid
Other agents	• Abaloparatide • Denosumab • Raloxifene • Romosozumab • Teriparatide

Note

- *Fractures of finger, toe, face and skull are not included in this measure.*

Data Elements for Reporting

Organizations that submit HEDIS data to NCQA must provide the following data elements.

Table OMW-3: Data Elements for Osteoporosis Management in Women Who Had a Fracture

Metric	Data Element	Reporting Instructions
OsteoporosisManagementWomen	Benefit	Metadata
	EligiblePopulation	Report once
	ExclusionAdminRequired	Report once
	NumeratorByAdmin	Report once
	NumeratorBySupplemental	Report once
	Rate	(Percent)

Rules for Allowable Adjustments of HEDIS

The "Rules for Allowable Adjustments of HEDIS" (the "Rules") describe how NCQA's HEDIS measure specifications can be adjusted for other populations, if applicable. The Rules, reviewed and approved by NCQA measure experts, provide for expanded use of HEDIS measures without changing their clinical intent.

Adjusted HEDIS measures *may not* be used for HEDIS health plan reporting.

Rules for Allowable Adjustments of Osteoporosis Management in Women Who Had a Fracture

NONCLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Product lines	Yes	Organizations are not required to use product line criteria; product lines may be combined and all (or no) product line criteria may be used.
Ages	Yes, with limits	The age determination dates may be changed (e.g., select, "age as of June 30"). Changing the denominator age range is allowed between 50-85 years of age.
Continuous enrollment, allowable gap, anchor date	Yes	Organizations are not required to use enrollment criteria; adjustments are allowed.
Benefits	Yes	Organizations are not required to use a benefit; adjustments are allowed.
Other	Yes	Organizations may use additional eligible population criteria to focus on an area of interest defined by gender, race, ethnicity, socioeconomic or sociodemographic characteristics, geographic region or another characteristic.
CLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Event/diagnosis	No	Only events or diagnoses that contain (or map to) codes in the value sets may be used to identify visits. Value sets and logic may not be changed.
Denominator Exclusions	**Adjustments Allowed (Yes/No)**	**Notes**
Required exclusions	Yes, with limits	Apply required exclusions according to specified medication lists and value sets. The palliative care, deceased member and hospice exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments.*
Exclusions: I-SNP, LTI, frailty or advanced illness	Yes	These exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments*.
Numerator Criteria	**Adjustments Allowed (Yes/No)**	**Notes**
Appropriate Testing or Treatment for Osteoporosis	No	Medication lists, value sets and logic may not be changed.

Care Coordination

Transitions of Care (TRC)

SUMMARY OF CHANGES TO HEDIS MY 2023

- Added a required exclusion for members who died during the measurement year.
- Revised the "Other" criteria in the Nonclinical Components table under *Rules for Allowable Adjustments of HEDIS.*
- Revised the "Required exclusions" criteria in the Clinical Components table under *Rules for Allowable Adjustments of HEDIS*.

Description

The percentage of discharges for members 18 years of age and older who had each of the following. Four rates are reported:

- *Notification of Inpatient Admission*. Documentation of receipt of notification of inpatient admission on the day of admission through 2 days after the admission (3 total days).
- *Receipt of Discharge Information.* Documentation of receipt of discharge information on the day of discharge through 2 days after the discharge (3 total days).
- *Patient Engagement After Inpatient Discharge*. Documentation of patient engagement (e.g., office visits, visits to the home, telehealth) provided within 30 days after discharge.
- *Medication Reconciliation Post-Discharge*. Documentation of medication reconciliation on the date of discharge through 30 days after discharge (31 total days).

Definitions

Medication reconciliation	A type of review in which the discharge medications are reconciled with the most recent medication list in the outpatient medical record.
Medication list	A list of medications in the medical record. The medication list may include medication names only or may include medication names, dosages and frequency, over-the-counter (OTC) medications and herbal or supplemental therapies.

Eligible Population

Product lines	Medicare.
Ages	18 years and older as of December 31 of the measurement year. Report two age stratifications and a total rate: • 18–64 years. • 65 years and older. • Total.
Continuous enrollment	The date of discharge through 30 days after discharge (31 total days).
Allowable gap	None.

Anchor date None.

Benefit Medical.

Event/diagnosis An acute or nonacute inpatient discharge on or between January 1 and December 1 of the measurement year. To identify acute and nonacute inpatient discharges:

1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
2. Identify the discharge date for the stay.

The denominator for this measure is based on discharges, not on members. If members have more than one discharge, include all discharges on or between January 1 and December 1 of the measurement year.

Observation visits that precede the inpatient stay Do not adjust the admit date if the discharge is preceded by an observation stay; use the admit date from the acute or nonacute inpatient stay.

Readmission or direct transfer If the discharge is followed by a readmission or direct transfer to an acute or nonacute inpatient care setting on the date of discharge through 30 days after discharge (31 days total), use the admit date from the first admission and the discharge date from the last discharge. To identify readmissions and direct transfers during the 31-day period:

1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
2. Identify the admission date for the stay (the admission date must occur during the 31-day period).
3. Identify the discharge date for the stay (the discharge date is the event date).

Exclude both the initial and the readmission/direct transfer discharge if the last discharge occurs after December 1 of the measurement year.

If the admission date and the discharge date for an acute inpatient stay occur between the admission and discharge dates for a nonacute inpatient stay, include only the nonacute inpatient discharge. To identify acute inpatient discharges:

1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
3. Identify the admission date for the stay.
4. Identify the discharge date for the stay.

To identify nonacute inpatient discharges:

1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set).
3. Identify the admission date for the stay.
4. Identify the discharge date for the stay.

Note: *If a member remains in an acute or nonacute facility through December 1 of the measurement year, a discharge is not included in the measure for this member, but the organization must have a method for identifying the member's status for the remainder of the measurement year, and may not assume the member remained admitted based only on the absence of a discharge before December 1.*

If the organization is unable to confirm the member remained in the acute or nonacute care setting through December 1, disregard the readmission or direct transfer and use the initial discharge date.

Required exclusions

Members who meet either of the following criteria:

- Members in hospice or using hospice services any time during the measurement year. Refer to *General Guideline 15: Members in Hospice*.
- Members who died any time during the measurement year. Refer to *General Guideline 16: Deceased Members.*

Administrative Specification

Denominator

The eligible population.

Numerators

Notification of Inpatient Admission

Administrative reporting is not available for this indicator.

Receipt of Discharge Information

Administrative reporting is not available for this indicator.

Patient Engagement After Inpatient Discharge

Patient engagement provided within 30 days after discharge. Do not include patient engagement that occurs on the date of discharge. The following meet criteria for patient engagement:

- An outpatient visit (Outpatient Value Set).
- A telephone visit (Telephone Visits Value Set).
- Transitional care management services (Transitional Care Management Services Value Set).
- An e-visit or virtual check-in (Online Assessments Value Set).

Medication Reconciliation Post-Discharge

Medication reconciliation (Medication Reconciliation Encounter Value Set; Medication Reconciliation Intervention Value Set) conducted by a prescribing practitioner, clinical pharmacist, physician assistant or registered nurse on the date of discharge through 30 days after discharge (31 total days).

Hybrid Specification

Denominator

A systematic sample drawn from the eligible population.

The denominator is based on discharges, not on members. Members may appear more than once in the sample.

Organizations may reduce the sample size based only on the prior year's audited, product line-specific rate for the lowest rate of all TRC indicators.

Refer to the *Guidelines for Calculations and Sampling* for information on reducing the sample size.

Identifying the medical record

Documentation in any outpatient medical record that is accessible to the PCP or ongoing care provider is eligible for use in reporting.

Numerators

Notification of Inpatient Admission

Documentation of receipt of notification of inpatient admission on the day of admission or on the day of admission through 2 days after the admission (3 total days).

Administrative

Administrative reporting is not available for this indicator.

Medical record

Documentation in the outpatient medical record must include evidence of receipt of notification of inpatient admission on the day of admission through 2 days after the admission (3 total days).

Documentation in the outpatient medical record must include evidence of receipt of notification of inpatient admission that includes evidence of the date when the documentation was received. Any of the following examples meet criteria:

- Communication between inpatient providers or staff and the member's PCP or ongoing care provider (e.g., phone call, email, fax).
- Communication about admission between emergency department and the member's PCP or ongoing care provider (e.g., phone call, email, fax).
- Communication about admission to the member's PCP or ongoing care provider through a health information exchange; an automated admission, or discharge and transfer (ADT) alert system.
- Communication about admission with the member's PCP or ongoing care provider through a shared electronic medical record (EMR) system. When using a shared EMR system, documentation of a "received date" is not required to meet criteria. Evidence that the information was filed in the EMR and is accessible to the PCP or ongoing care provider on the day of admission through 2 days after the admission (3 total days) meets criteria.
- Communication about admission to the member's PCP or ongoing care provider from the member's health plan.
- Indication that the member's PCP or ongoing care provider admitted the member to the hospital.
- Indication that a specialist admitted the member to the hospital and notified the member's PCP or ongoing care provider.
- Indication that the PCP or ongoing care provider placed orders for tests and treatments any time during the member's inpatient stay.
- Documentation that the PCP or ongoing care provider performed a preadmission exam or received communication about a planned inpatient admission. The time frame that the planned inpatient admission must be communicated is not limited to the day of admission through 2 days after the admission (3 total days); documentation that the PCP or ongoing care provider performed a preadmission exam or received notification of a planned admission prior to the admit date also meets criteria. The planned admission documentation or preadmission exam must clearly pertain to the denominator event.

Note: *When an ED visit results in an inpatient admission, notification that a provider sent the member to the ED does not meet criteria. Evidence that the PCP or ongoing care provider communicated with the ED about the admission meets criteria.*

Receipt of Discharge Information

Documentation of receipt of discharge information on the day of discharge through 2 days after the discharge (3 total days).

Administrative

Administrative reporting is not available for this indicator.

Medical record

Documentation in the outpatient medical record must include evidence of receipt of discharge information on the day of discharge through 2 days after the discharge (3 total days) with evidence of the date when the documentation was received. Discharge information may be included in, but not limited to, a discharge summary or summary of care record or be located in structured fields in an EHR. At a minimum, the discharge information must include all of the following:

- The practitioner responsible for the member's care during the inpatient stay.
- Procedures or treatment provided.
- Diagnoses at discharge.
- Current medication list.
- Testing results, or documentation of pending tests or no tests pending.
- Instructions for patient care post-discharge.

Note: *If the PCP or ongoing care provider is the discharging provider, the discharge information must be documented in the medical record on the day of discharge through 2 days after the discharge (3 total days).*

When using a shared EMR system, documentation of a "received date" in the EMR is not required to meet criteria. Evidence that the information was filed in the EMR and is accessible to the PCP or ongoing care provider on the day of discharge through 2 days after the discharge (3 total days) meets criteria.

Patient Engagement After Inpatient Discharge

Documentation of patient engagement (e.g., office visits, visits to the home, or telehealth) provided within 30 days after discharge. Do not include patient engagement that occurs on the date of discharge.

Administrative

Refer to *Administrative Specification* to identify positive numerator hits from administrative data.

Medical record

Documentation in the outpatient medical record must include evidence of patient engagement within 30 days after discharge. Any of the following meet criteria:

- An outpatient visit, including office visits and home visits.
- A telephone visit.
- A synchronous telehealth visit where real-time interaction occurred between the member and provider using audio and video communication.
- An e-visit or virtual check-in (asynchronous telehealth where two-way interaction, which was not real-time, occurred between the member and provider).

Note: *If the member is unable to communicate with the provider, interaction between the member's caregiver and the provider meets criteria.*

Medication Reconciliation Post-Discharge Medication reconciliation conducted by a prescribing practitioner, clinical pharmacist, physician assistant or registered nurse, as documented through either administrative data or medical record review on the date of discharge through 30 days after discharge (31 total days).

Administrative Refer to *Administrative Specification* to identify positive numerator hits from administrative data.

Medical record Documentation in the outpatient medical record must include evidence of medication reconciliation and the date when it was performed. Any of the following meet criteria:

- Documentation of the current medications with a notation that the provider reconciled the current and discharge medications.
- Documentation of the current medications with a notation that references the discharge medications (e.g., no changes in medications since discharge, same medications at discharge, discontinue all discharge medications).
- Documentation of the member's current medications with a notation that the discharge medications were reviewed.
- Documentation of a current medication list, a discharge medication list and notation that both lists were reviewed on the same date of service.
- Documentation of the current medications with evidence that the member was seen for post-discharge hospital follow-up with evidence of medication reconciliation or review. Evidence that the member was seen for post-discharge hospital follow-up requires documentation that indicates the provider was aware of the member's hospitalization or discharge.
- Documentation in the discharge summary that the discharge medications were reconciled with the most recent medication list in the outpatient medical record. There must be evidence that the discharge summary was filed in the outpatient chart on the date of discharge through 30 days after discharge (31 total days).
- Notation that no medications were prescribed or ordered upon discharge.

Note

- *The following notations or examples of documentation do not count as numerator compliant:*
  - *Notification of Inpatient Admission and Notification of Inpatient Discharge:*
    - *Documentation that the member or the member's family notified the member's PCP or ongoing care provider of the admission or discharge.*
    - *Documentation of notification that does not include a time frame or date when the documentation was received.*
  - *Medication Reconciliation Post-Discharge:*
    - *Documentation of "post-op/surgery follow-up" without a reference to "hospitalization," "admission" or "inpatient stay" does not imply a hospitalization and is not considered evidence that the provider was aware of a hospitalization.*
- *The Medication Reconciliation Post-Discharge numerator assesses whether medication reconciliation occurred. It does not attempt to assess the quality of the medication list documented in the medical record or the process used to document the most recent medication list in the medical record.*

- *The denominator is based on the discharge date found in administrative/claims data, but organizations may use other systems (including data found during medical record review) to identify data errors and make corrections.*
  - *If a different discharge date is found in the medical record, and the organization chooses to use that date, the organization must assess all indicators using the updated discharge date, including those that were previously compliant based on administrative data.*
- *Organizations may have different methods for billing intensive outpatient visits and partial hospitalizations. Some methods may be comparable to outpatient billing, with separate claims for each date of service; others may be comparable to inpatient billing, with an admission date, a discharge date and units of service. Organizations whose billing methods are comparable to inpatient billing may count each unit of service as an individual visit. The unit of service must have occurred during the required period for the rate (i.e., within 30 days after discharge).*
- *Refer to Appendix 3 for the definition of* PCP *and* ongoing care provider*.*
- *A medication reconciliation performed without the member present meets criteria.*

Data Elements for Reporting

Organizations that submit HEDIS data to NCQA must provide the following data elements.

Table TRC-3: Data Elements for Transitions of Care

Metric	Age	Data Element	Reporting Instructions	A
MedicationReconciliationPostDischarge	18-64	CollectionMethod	For each Metric, repeat per Stratification	✓
PatientEngagementAfterInpatientDischarge	65+	EligiblePopulation*	For each Metric and Stratification	✓
NotificationInpatientAdmission	Total	ExclusionAdminRequired*†	For each Metric and Stratification	✓
ReceiptDischargeInformation		NumeratorByAdminElig†	For each Metric and Stratification	
		CYAR†	Only for Total (Percent)	
		MinReqSampleSize	For each Metric, repeat per Stratification	
		OversampleRate	For each Metric, repeat per Stratification	
		OversampleRecordsNumber	(Count)	
		ExclusionValidDataErrors	For each Metric, repeat per Stratification	
		ExclusionEmployeeOrDep	For each Metric, repeat per Stratification	
		OversampleRecsAdded	For each Metric, repeat per Stratification	
		Denominator	For each Stratification, repeat per Metric	
		NumeratorByAdmin†	For each Metric and Stratification	✓
		NumeratorByMedicalRecords	For each Metric and Stratification	
		NumeratorBySupplemental	For each Metric and Stratification	✓
		Rate	(Percent)	✓

*Repeat the EligiblePopulation and ExclusionAdminRequired values for metrics using the administrative method.

†These data elements are only reported or calculated for the MedicationReconciliationPostDischarge and PatientEngagementAfterInpatientDischarge Metrics.

Rules for Allowable Adjustments of HEDIS

The "Rules for Allowable Adjustments of HEDIS" (the "Rules") describe how NCQA's HEDIS measure specifications can be adjusted for other populations, if applicable. The Rules, reviewed and approved by NCQA measure experts, provide for expanded use of HEDIS measures without changing their clinical intent.

Adjusted HEDIS measures *may not* be used for HEDIS health plan reporting.

Rules for Allowable Adjustments of Transitions of Care

NONCLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Product lines	Yes	Organizations are not required to use product line criteria; product lines may be combined and all (or no) product line criteria may be used.
Ages	Yes	Age determination dates may be changed (e.g., select, "age as of June 30"). Changing the denominator age range is allowed.
Continuous enrollment, allowable gap, anchor date	Yes	Organizations are not required to use enrollment criteria; adjustments are allowed.
Benefits	Yes	Organizations are not required to use a benefit; adjustments are allowed.
Other	Yes	Organizations may use additional eligible population criteria to focus on an area of interest defined by gender, race, ethnicity, socioeconomic or sociodemographic characteristics, geographic region or another characteristic.
CLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Event/diagnosis	Yes, with limits	Only events that contain (or map to) codes in the value sets may be used to identify the eligible population for each rate. The value sets and logic may not be changed. ***Note:*** *Organizations may choose alternate measurement-period date ranges.* *Organizations may assess at the member level (vs. discharge level) by applying measure logic appropriately (i.e., percentage of members with documentation of medication reconciliation after each discharge).*
Denominator Exclusions	**Adjustments Allowed (Yes/No)**	**Notes**
Required exclusions	Yes	The hospice and deceased member exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments*.

Numerator Criteria	Adjustments Allowed (Yes/No)	Notes
• Notification of Inpatient Admission • Receipt of Discharge Information	No	Allowable adjustments are not permitted for these components of the Transitions of Care measure.
• Patient Engagement After Inpatient Discharge • Medication Reconciliation Post-Discharge	No	Value sets and logic may not be changed.

Follow-Up After Emergency Department Visit for People With Multiple High-Risk Chronic Conditions (FMC)

SUMMARY OF CHANGES FOR HEDIS MY 2023

- Added a required exclusion for members who died during the measurement year.
- Added domiciliary/rest home visits to the numerator.
- Revised the "Other" criteria in the Nonclinical Components table under *Rules for Allowable Adjustments of HEDIS.*
- Revised the "Required exclusions" criteria in the Clinical Components table under *Rules for Allowable Adjustments of HEDIS*.

Description

The percentage of emergency department (ED) visits for members 18 years of age and older who have multiple high-risk chronic conditions who had a follow-up service within 7 days of the ED visit.

Eligible Population

Product lines Medicare.

Ages 18 years and older as of the ED visit. Report two age stratifications and a total rate:

- 18–64 years.
- 65 years and older.
- Total.

Continuous enrollment 365 days prior to the ED visit through 7 days after the ED visit.

Allowable gap No more than one gap in enrollment of up to 45 days during the 365 days prior to the ED visit and no gap during the 7 days following the ED visit.

Anchor date None.

Benefits Medical.

Event/diagnosis Follow the steps below to identify the eligible population.

Step 1 An ED visit (ED Value Set) on or between January 1 and December 24 of the measurement year where the member was 18 years or older on the date of the visit.

The denominator for this measure is based on ED visits, not on members. If a member has more than one ED visit, identify all ED visits between January 1 and December 24 of the measurement year.

Step 2: ED visits resulting in inpatient stay

Exclude ED visits that result in an inpatient stay. Exclude ED visits followed by admission to an acute or nonacute inpatient care setting on the date of the ED visit or within 7 days after the ED visit, regardless of the principal diagnosis for admission. To identify admissions to an acute or nonacute inpatient care setting:

1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
2. Identify the admission date for the stay.

These events are excluded from the measure because admission to an acute or nonacute setting may prevent an outpatient follow-up visit from taking place.

Step 3: Eligible chronic condition diagnoses

Identify ED visits where the member had a chronic condition prior to the ED visit.

The following are eligible chronic condition diagnoses. Each bullet indicates an eligible chronic condition (for example, COPD and asthma are considered the same chronic condition):

- COPD and asthma (COPD Diagnosis Value Set; Asthma Diagnosis Value Set; Unspecified Bronchitis Value Set).
- Alzheimer's disease and related disorders (Dementia Value Set; Frontotemporal Dementia Value Set).
- Chronic kidney disease (Chronic Kidney Disease Value Set).
- Depression (Major Depression Value Set; Dysthymic Disorder Value Set).
- Heart failure (Chronic Heart Failure Value Set; Heart Failure Diagnosis Value Set).
- Acute myocardial infarction (MI Value Set; Old Myocardial Infarction Value Set).
- Atrial fibrillation (Atrial Fibrillation Value Set).
- Stroke and transient ischemic attack (Stroke Value Set).
  - Remove any visit with a principal diagnosis of encounter for other specified aftercare (Stroke Exclusion Value Set).
  - Remove any visit with any diagnosis of concussion with loss of consciousness or fracture of vault of skull, initial encounter (Other Stroke Exclusions Value Set).

Using the eligible chronic condition diagnoses above, identify members who had any of the following during the measurement year or the year prior to the measurement year, **but prior to the ED visit** (count services that occur over both years):

- At least two outpatient visits (Outpatient Value Set), observation visits (Observation Value Set), ED visits (ED Value Set), telephone visits (Telephone Visits Value Set), e-visits or virtual check-ins (Online Assessments Value Set), nonacute inpatient encounters (Nonacute Inpatient Value Set) or nonacute inpatient discharges (instructions below; the diagnosis must be on the discharge claim) on different dates of service, with an eligible chronic condition. Visit type need not be the same for the two visits, but the visits must be for the same eligible chronic condition. To identity a nonacute inpatient discharge:
  1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).

2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set) on the claim.
3. Identify the discharge date for the stay.

- At least one acute inpatient encounter (Acute Inpatient Value Set) with an eligible chronic condition.
- At least one acute inpatient discharge with an eligible chronic condition on the discharge claim. To identify an acute inpatient discharge:
  1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set).
  2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
  3. Identify the discharge date for the stay.

For each ED visit, identify the total number of chronic conditions the member had prior to the ED visit.

Step 4: Identifying members with multiple chronic conditions

Identify ED visits where the member had **two or more** different chronic conditions prior to the ED visit, that meet the criteria included in step 3. These are eligible ED visits.

Step 5: Multiple visits in 8-day period

If a member has more than one ED visit in an 8-day period, include only the first eligible ED visit. For example, if a member has an eligible ED visit on January 1, include the January 1 visit and do not include ED visits that occur on or between January 2 and January 8. Then, if applicable, include the next eligible ED visit that occurs on or after January 9. Identify visits chronologically, including only one visit per 8-day period.

Required exclusions

Exclude members who meet either of the following criteria:

- Members in hospice or using hospice services any time during the measurement year. Refer to *General Guideline 15: Members in Hospice*.
- Members who died any time during the measurement year. Refer to *General Guideline 16: Deceased Members.*

Administrative Specification

Denominator

The eligible population.

Numerator

7-Day Follow-Up

A follow-up service within 7 days after the ED visit (8 total days). Include visits that occur on the date of the ED visit. The following meet criteria for follow-up:

- An outpatient visit (Outpatient Value Set).
- A telephone visit (Telephone Visits Value Set).
- Transitional care management services (Transitional Care Management Services Value Set).
- Case management visits (Case Management Encounter Value Set).
- Complex Care Management Services (Complex Care Management Services Value Set).

- An outpatient or telehealth behavioral health visit (Visit Setting Unspecified Value Set ***with*** Outpatient POS Value Set).
- An outpatient or telehealth behavioral health visit (BH Outpatient Value Set).
- An intensive outpatient encounter or partial hospitalization (Visit Setting Unspecified Value Set ***with*** Partial Hospitalization POS Value Set).
- An intensive outpatient encounter or partial hospitalization (Partial Hospitalization or Intensive Outpatient Value Set).
- A community mental health center visit (Visit Setting Unspecified Value Set ***with*** Community Mental Health Center POS Value Set).
- Electroconvulsive therapy (Electroconvulsive Therapy Value Set) ***with*** (Ambulatory Surgical Center POS Value Set; Community Mental Health Center POS Value Set; Outpatient POS Value Set; Partial Hospitalization POS Value Set).
- A telehealth visit (Visit Setting Unspecified Value Set ***with*** Telehealth POS Value Set).
- An observation visit (Observation Value Set).
- A substance use disorder service (Substance Use Disorder Services Value Set).
- An e-visit or virtual check-in (Online Assessments Value Set).
- A domiciliary or rest home visit (Domiciliary or Rest Home Visit Value Set).

Note

- *Organizations may have different methods for billing intensive outpatient visits and partial hospitalizations. Some methods may be comparable to outpatient billing, with separate claims for each date of service; others may be comparable to inpatient billing, with an admission date, a discharge date and units of service. Organizations whose billing methods are comparable to inpatient billing may count each unit of service as an individual visit. The unit of service must have occurred during the required period for the rate (within 7 days after the ED visit).*

Data Elements for Reporting

Organizations that submit HEDIS data to NCQA must provide the following data elements.

Table FMC-3: Data Elements for Follow-Up After Emergency Department Visit for People With High-Risk Multiple Chronic Conditions

Metric	Age	Data Element	Reporting Instructions
FollowUp7Day	16-64	EligiblePopulation	For each Stratification
	65+	ExclusionAdminRequired	For each Stratification
	Total	NumeratorByAdmin	For each Stratification
		NumeratorBySupplemental	For each Stratification
		Rate	(Percent)

Rules for Allowable Adjustments of HEDIS

The "Rules for Allowable Adjustments of HEDIS" (the "Rules") describe how NCQA's HEDIS measure specifications can be adjusted for other populations, if applicable. The Rules, reviewed and approved by NCQA measure experts, provide for expanded use of HEDIS measures without changing their clinical intent.

Adjusted HEDIS measures *may not* be used for HEDIS health plan reporting.

Rules for Allowable Adjustments of Follow-Up After Emergency Department Visit for People With High-Risk Multiple Chronic Conditions

NONCLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Product lines	Yes	Organizations are not required to use product line criteria; product lines may be combined and all (or no) product line criteria may be used.
Ages	Yes	The age determination dates may be changed (e.g., select, "age as of June 30"). Expanding the denominator age range is allowed.
Continuous enrollment, allowable gap, anchor date	Yes	Organizations are not required to use enrollment criteria; adjustments are allowed.
Benefits	Yes	Organizations are not required to use a benefit; adjustments are allowed.
Other	Yes	Organizations may use additional eligible population criteria to focus on an area of interest defined by gender, race, ethnicity, socioeconomic or sociodemographic characteristics, geographic region or another characteristic.
CLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Event/diagnosis	Yes, with limits	Only events or diagnoses that contain (or map to) codes in the value sets may be used to identify visits with a diagnosis. The value sets and logic may not be changed. ***Note:*** *Organizations may assess at the member level by applying measure logic appropriately (i.e., percentage of members with documentation of an emergency department visit with multiple high-risk chronic conditions, who had a follow-up visit within 7 days).*
Denominator Exclusions	**Adjustments Allowed (Yes/No)**	**Notes**
Exclusions	No	These exclusions are part of the eligible population criteria.
Required exclusions	Yes	The hospice and deceased member exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments*.
Numerator Criteria	**Adjustments Allowed (Yes/No)**	**Notes**
7-Day Follow-Up	No	Value sets and logic may not be changed.

Measures Collected Through the Medicare Health Outcomes Survey

Medicare Health Outcomes Survey (HOS)

Detailed specifications and summary of changes are contained in *HEDIS Volume 6: Specifications for the Medicare Health Outcomes Survey.*

HEDIS MY 2023 Volume 6: Specifications for the Medicare Health Outcomes Survey will be published prior to survey administration in May 2024.

Description

This measure provides a general indication of how well a Medicare Advantage Organization (MAO) manages the physical and mental health of its members. The survey measures physical and mental health status at the beginning of a 2-year period and again at the end of the 2-year period, when a change score is calculated. Each member's health status is categorized as "better than expected," "the same as expected," or "worse than expected," accounting for death and risk-adjustment factors. MAO-specific results are assigned as percentages of members whose health status was better, the same or worse than expected.

Fall Risk Management (FRM)

This measure is collected using the Medicare Health Outcomes Survey (HOS). Detailed specifications and summary of changes are contained in *HEDIS Volume 6: Specifications for the Medicare Health Outcomes Survey.*

HEDIS MY 2023 Volume 6: Specifications for the Medicare Health Outcomes Survey will be published prior to survey administration in May 2024.

Description

The two components of this measure assess different facets of fall risk management.

- *Discussing Fall Risk.* The percentage of Medicare members 65 years of age and older who were seen by a practitioner in the past 12 months and who discussed falls or problems with balance or walking with their current practitioner.
- *Managing Fall Risk.* The percentage of Medicare members 65 years of age and older who had a fall or had problems with balance or walking in the past 12 months, who were seen by a practitioner in the past 12 months and who received a recommendation for how to prevent falls or treat problems with balance or walking from their current practitioner.

Eligible Population

Product line	Medicare.
Age	65 and older as of December 31 of the measurement year.
Exclusion	Evidence from CMS administrative records of a hospice start date.

Protocol and Survey Instrument

Medicare	Collected using the HOS. Refer to *HEDIS Volume 6: Specifications for the Medicare Health Outcomes Survey* for the HOS questionnaires and data collection protocols.

Management of Urinary Incontinence in Older Adults (MUI)

This measure is collected using the HOS. Detailed specifications and summary of changes are contained in *HEDIS Volume 6: Specifications for the Medicare Health Outcomes Survey.*

HEDIS MY 2023 Volume 6: Specifications for the Medicare Health Outcomes Survey will be published prior to survey administration in May 2024.

Description

The following components of this measure assess the management of urinary incontinence in older adults.

- *Discussing Urinary Incontinence.* The percentage of Medicare members 65 years of age and older who reported having urine leakage in the past 6 months and who discussed their urinary leakage problem with a health care provider.
- *Discussing Treatment of Urinary Incontinence.* The percentage of Medicare members 65 years of age and older who reported having urine leakage in the past 6 months and who discussed treatment options for their current urine leakage problem.
- *Impact of Urinary Incontinence.* The percentage of Medicare members 65 years of age and older who reported having urine leakage in the past 6 months and who reported that urine leakage made them change their daily activities or interfered with their sleep a lot.

  Note: *A lower rate indicates better performance for this indicator.*

Eligible Population

Product line	Medicare.
Age	65 and older as of December 31 of the measurement year.
Exclusion	Evidence from CMS administrative records of a hospice start date.

Protocol and Survey Instrument

Medicare	Collected using the HOS. Refer to *HEDIS Volume 6: Specifications for the Medicare Health Outcomes Survey* for the HOS questionnaires and data collection protocols.

Physical Activity in Older Adults (PAO)

This measure is collected using the HOS. Detailed specifications and summary of changes are contained in *HEDIS Volume 6: Specifications for the Medicare Health Outcomes Survey.*

HEDIS MY 2023 Volume 6: Specifications for the Medicare Health Outcomes Survey will be published prior to survey administration in May 2024.

Description

The two components of this measure assess different facets of promoting physical activity in older adults.

- *Discussing Physical Activity.* The percentage of Medicare members 65 years of age and older who had a doctor's visit in the past 12 months and who spoke with a doctor or other health provider about their level of exercise or physical activity.
- *Advising Physical Activity.* The percentage of Medicare members 65 years of age and older who had a doctor's visit in the past 12 months and who received advice to start, increase or maintain their level exercise or physical activity.

Eligible Population

Product line	Medicare.
Age	65 and older as of December 31 of the measurement year.
Exclusion	Evidence from CMS administrative records of a hospice start date.

Protocol and Survey Instrument

Medicare	Collected using the HOS. Refer to *HEDIS Volume 6: Specifications for the Medicare Health Outcomes Survey* for the HOS questionnaires and data collection protocols.

Risk Adjusted Utilization

Guidelines for Risk Adjusted Utilization Measures

SUMMARY OF CHANGES TO HEDIS MY 2023

- Removed *Guideline 7: SES Stratification;* this text has been moved to the General Guidelines for Data Collection and Reporting as *General Guideline 32: Medicare Socioeconomic Status Stratification*.

Guidelines

1. **Which services count?** Include all services, whether or not the organization paid for them or expects to pay for them (include denied claims) when applying risk adjustment in the Risk Adjusted Utilization measures. *Do not include* denied services (only include paid services and services expected to be paid) when identifying all other events (e.g., the IHS in the PCR measure or observed events in the other risk adjusted utilization measures).

When confirming that an ED visit does not result in an inpatient or observation stay, all inpatient and observation stays must be considered, regardless of payment status (paid, suspended, pending, denied). For example, if an ED visit is paid but an inpatient stay is denied, the ED visit resulted in an inpatient stay and is not included in the Emergency Department Utilization measure when identifying observed ED visits.

The organization may have:

- Covered the full amount.
- Paid only a portion of the amount (e.g., 80%).
- Paid nothing because the member covered the entire amount to meet a deductible.
- Paid nothing because the service was covered as part of a PMPM payment.
- Denied the service.

Count the service as paid or expected to be paid if:

- The organization paid the full amount ***or*** a portion of the amount (e.g., 80%).
- The member paid for the service as part of the benefit offering (e.g., to meet a deductible), ***or***
- The service was covered under a PMPM payment.

Count the service as denied if:

- The organization denied the service for any reason, unless the member paid for the service as part of the benefit offering (e.g., to meet a deductible), ***or***
- The claim for the service was rejected because it was missing information or was invalid for another reason.

2. **Risk adjustment.** Organizations may not use supplemental data sources when applying the risk adjustment methodology.

Organizations may not use risk assessment protocols to supplement diagnoses for calculation of the risk adjustment scores for these measures. The measurement model was developed and tested using only claims-based diagnoses and diagnoses from additional data sources would affect the validity of the models as they are current implemented in the specification.

3. **Counting transfers.** Unless otherwise specified in the measure, treat transfers *between* institutions as separate admissions. Base transfer reports *within* an institution on the type and level of services provided. Report separate admissions when the transfer is between acute and nonacute levels of service or between mental health/chemical dependency services and non-mental health/chemical dependency services.

   Count only one admission when the transfer takes place within the same service category but to a different level of care; for example, from intensive care to a lesser level of care or from a lesser level of care to intensive care.

4. **Mental health and chemical dependency transfers.** Unless otherwise specified in the measure, count as a separate admission a transfer within the same institution but to a different level of care (e.g., a transfer between inpatient and residential care). Each level must appropriately include discharges and length of stay (count inpatient days under inpatient; count residential days under residential).

5. **Observation stays without an admission and/or discharge date.** For observation stays (Observation Stay Value Set) that do not have a recorded admission or discharge date, set the admission date to the earliest date of service on the claim and set the discharge date to the last date of service on the claim.

6. **Direct transfers.** A direct transfer is when the discharge date from the initial stay precedes the admission date to a subsequent stay by one calendar day or less. For example:
   - A discharge on June 1, followed by a subsequent admission on June 1, *is a direct transfer.*
   - A discharge on June 1, followed by a subsequent admission on June 2, *is a direct transfer.*
   - A discharge on June 1, followed by a subsequent admission on June 3, *is not a direct transfer;* these are two distinct stays.
   - A discharge on June 1, followed by a subsequent admission on June 2 (with discharge on June 3), followed by a subsequent admission on June 4, *is a direct transfer.*

   Direct transfers may occur from and between different facilities and/or different service levels. Refer to individual measure specifications for details.

Risk Adjustment Comorbidity Category Determination

Step 1 Identify all diagnoses for encounters during the classification period for each denominator unit of the measure (i.e., denominator event or member). Include the following when identifying encounters:
- Outpatient visits (Outpatient Value Set).
- Telephone visits (Telephone Visits Value Set).
- Observation visits (Observation Value Set).
- ED visits (ED Value Set).
- Inpatient events:
  - Nonacute inpatient encounters (Nonacute Inpatient Value Set).
  - Acute inpatient encounters (Acute Inpatient Value Set).
  - Acute and nonacute inpatient discharges (Inpatient Stay Value Set).

Use the date of service for outpatient, observation and ED visits. Use the discharge date for inpatient events.

For PCR, exclude the principal discharge diagnosis on the IHS. For the HFS measure, exclude the primary discharge diagnosis on the skilled nursing facility discharge (SND) to the community.

Step 2 Assign each diagnosis to a comorbid Clinical Condition (CC) category using Table CC—Mapping. If the code appears more than once in Table CC—Mapping, it is assigned to multiple CCs.

Exclude all diagnoses that cannot be assigned to a comorbid CC category. For members with no qualifying diagnoses from face-to-face encounters, skip to the *Risk Adjustment Weighting* section.

All digits must match exactly when mapping diagnosis codes to the comorbid CCs.

Step 3 Determine HCCs for each comorbid CC identified. Refer to Table HCC—Rank.

For each denominator unit's comorbid CC list, match the comorbid CC code to the comorbid CC code in the table, and assign:

- The ranking group.
- The rank.
- The HCC.

For comorbid CCs that do not match to Table HCC—Rank, use the comorbid CC as the HCC and assign a rank of 1.

Note: *One comorbid CC can map to multiple HCCs; each HCC can have one or more comorbid CCs.*

Step 4 Assess each ranking group separately and select only the highest ranked HCC in each ranking group using the "Rank" column (1 is the highest rank possible).

Drop all other HCCs in each ranking group, and de-duplicate the HCC list if necessary.

Example *Assume a denominator unit with the following comorbid CCs*: CC-85, CC-17 and CC-19 (assume no other CCs).

- CC-85 does not have a map to the ranking table and becomes HCC-85.
- HCC-17 and HCC-19 are part of Diabetes Ranking Group 1. Because CC-17 is ranked higher than CC-19 in Ranking Group Diabetes 1, the comorbidity is assigned as HCC-17 for Ranking Group 1.
- The final comorbidities for this denominator unit are HCC-17 and HCC-85.

Example: Table HCC—Rank

Ranking Group	CC	Description	Rank	HCC
NA	CC-85	Congestive Heart Failure	NA	HCC-85
Diabetes 1	CC-17	Diabetes With Acute Complications	1	HCC-17
	CC-18	Diabetes With Chronic Complications	2	HCC-18
	CC-19	Diabetes Without Complications	3	HCC-19

Step 5 Identify combination HCCs listed in Table HCC—Comb.

Some combinations suggest a greater amount of risk when observed together. For example, when diabetes *and* CHF are present, an increased amount of risk is evident. Additional HCCs are selected to account for these relationships.

Compare each denominator unit's list of unique HCCs to those in the *Comorbid HCC* columns in Table HCC—Comb and assign any additional HCC conditions.

If there are fully nested combinations, use only the more comprehensive pattern. For example, if the diabetes/CHF combination is nested in the diabetes/CHF/renal combination, count only the diabetes/CHF/renal combination.

If there are overlapping combinations, use both sets of combinations. Based on the combinations, a denominator unit can have none, one or more of these added HCCs.

Example For a denominator unit with comorbidities HCC-17 and HCC-85 (assume no other HCCs), assign HCC-901 in addition to HCC-17 and HCC-85. This *does not* replace HCC-17 and HCC-85.

Example: Table HCC—Comb

Comorbid HCC 1	Comorbid HCC 2	Comorbid HCC 3	HCC-Combination	HCC-Comb Description
HCC-17	HCC-85	NA	HCC-901	Combination: Diabetes and CHF
HCC-18	HCC-85	NA	HCC-901	Combination: Diabetes and CHF
HCC-19	HCC-85	NA	HCC-901	Combination: Diabetes and CHF

Note

For the PCR Medicaid product line only, *organizations must use the "RAU Table – PCR Medicaid" risk adjustment table when assigning comorbid and discharge conditions. For all other measures and the commercial and Medicare PCR product lines, organizations must use the "RAU Shared Tables" risk adjustment table when assigning comorbid and discharge conditions.*

Plan All-Cause Readmissions (PCR)

SUMMARY OF CHANGES TO HEDIS MY 2023

- Replaced "female members" with "members" in the pregnancy exclusion.
- Clarified truncating and rounding rules in steps 6 and 8 of the *Risk Adjustment Weighting* section.
- Revised the "Other" criteria in the Nonclinical Components table under *Rules for Allowable Adjustments of HEDIS* for Observed Measurement.

Description

For members 18 years of age and older, the number of acute inpatient and observation stays during the measurement year that were followed by an unplanned acute readmission for any diagnosis within 30 days and the predicted probability of an acute readmission.

Note: *For commercial and Medicaid, report only members 18–64 years of age.*

Definitions

IHS	Index hospital stay. An acute inpatient or observation stay with a discharge on or between January 1 and December 1 of the measurement year, as identified in the denominator.
Index Admission Date	The IHS admission date.
Index Discharge Date	The IHS discharge date. The index discharge date must occur on or between January 1 and December 1 of the measurement year.
Index Readmission Stay	An acute inpatient or observation stay for any diagnosis with an admission date within 30 days of a previous Index Discharge Date.
Index Readmission Date	The admission date associated with the Index Readmission Stay.
Planned hospital stay	A hospital stay is considered planned if it meets criteria as described in step 3 (required exclusions) of the numerator.
Plan population	Members in the eligible population prior to exclusion of outliers (denominator steps 1–5). The plan population is only used as a denominator for the Outlier Rate. Members must be 18 and older as of the earliest Index Discharge Date. The plan population is based on members, not discharges. Count members only once in the plan population. Assign members to the product/product line in which they are enrolled at the start of the continuous enrollment period of their earliest IHS. If the member has a gap at the beginning of this continuous enrollment period, assign the member

	to the product/product line in which they were enrolled as of their first enrollment segment during this continuous enrollment period.
Outlier	Medicaid and Medicare members in the eligible population with four or more IHS between January 1 and December 1 of the measurement year. Commercial members in the eligible population with three or more IHS between January 1 and December 1 of the measurement year. Assign members to the product/product line in which they are enrolled at the start of the continuous enrollment period of their earliest IHS. If the member has a gap at the beginning of this continuous enrollment period, assign the member to the product/product line in which they were enrolled as of their first enrollment segment during the continuous enrollment period.
Nonoutlier	Members in the eligible population who are not considered outliers.
Classification period	365 days prior to and including Index Discharge Date.

Eligible Population

Product line	Commercial, Medicare, Medicaid (report each product line separately).
Stratification	For only Medicare IHS, report the following SES stratifications and total: • Non-LIS/DE, Nondisability. • LIS/DE. • Disability. • LIS/DE and Disability. • Other. • Unknown. • Total Medicare. ***Note:*** *The stratifications are mutually exclusive and the sum of all six stratifications is the Total population.*
Ages	*For commercial,* 18–64 years as of the Index Discharge Date. *For Medicare,* 18 years and older as of the Index Discharge Date. *For Medicaid,* 18–64 years as of the Index Discharge Date.
Continuous enrollment	365 days prior to the Index Discharge Date through 30 days after the Index Discharge Date.
Allowable gap	No more than one gap in enrollment of up to 45 days during the 365 days prior to the Index Discharge Date and no gap during the 30 days following the Index Discharge Date.
Anchor date	Index Discharge Date.
Benefit	Medical.

Event/diagnosis An acute inpatient or observation stay discharge on or between January 1 and December 1 of the measurement year.

The denominator for this measure is based on discharges, not members. Include all acute inpatient or observation stay discharges for nonoutlier members who had one or more discharges on or between January 1 and December 1 of the measurement year.

Follow the steps below to identify acute inpatient and observation stays.

Required exclusions Members in hospice or using hospice services any time during the measurement year. Refer to *General Guideline 15: Members in Hospice.*

Administrative Specification

Denominator The eligible population.

Step 1 Identify all acute inpatient and observation stay discharges on or between January 1 and December 1 of the measurement year. To identify acute inpatient and observation stay discharges:

1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set) and observation stays (Observation Stay Value Set).
2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
3. Identify the discharge date for the stay.

Inpatient and observation stays where the discharge date from the first setting and the admission date to the second setting are 2 or more calendar days apart must be considered distinct stays.

The measure includes acute discharges from any type of facility (including behavioral healthcare facilities).

Step 2 *Direct transfers:* For discharges with one or more direct transfers, use the last discharge.

Using the discharges identified in step 1, identify direct transfers between acute inpatient and observation or between observation and acute inpatient using the definition found in the *Guidelines for Risk Adjusted Utilization Measures.*

Exclude the hospital stay if the direct transfer's discharge date occurs after December 1 of the measurement year.

Step 3 Exclude hospital stays where the Index Admission Date is the same as the Index Discharge Date.

Step 4 Exclude hospital stays for the following reasons:

- The member died during the stay.
- Members with a principal diagnosis of pregnancy (Pregnancy Value Set) on the discharge claim.
- A principal diagnosis of a condition originating in the perinatal period (Perinatal Conditions Value Set) on the discharge claim.

Note: *For hospital stays where there was a direct transfer (identified in step 2), use the original stay and any direct transfer stays to identify exclusions in this step.*

Step 5 Calculate continuous enrollment.

Step 6 Remove hospital stays for outlier members and report these members as outliers in Tables PCR-A-1/2 and PCR-A-3.

Note: *Count discharges with one or more direct transfers (identified in step 2) as one discharge when identifying outlier members.*

Step 7 Assign each remaining acute inpatient or observation stay to an age and stratification category using the reporting instructions below.

Risk Adjustment Determination

For each IHS among nonoutlier members, use the following steps to identify risk adjustment categories based on presence of observation stay status at discharge, surgeries, discharge condition, comorbidity, age and gender.

Observation Stay Determine if the IHS at discharge was an observation stay (Observation Stay Value Set). For direct transfers, determine the hospitalization status using the last discharge.

Surgeries Determine if the member underwent surgery during the stay (Surgery Procedure Value Set). Consider an IHS to include a surgery if at least one procedure code is present from any provider between the admission and discharge dates.

Discharge Condition Assign a discharge Clinical Condition (CC) category code or codes to the IHS based on its principal discharge diagnosis, using Table CC-Mapping. For direct transfers, use the principal discharge diagnosis from the last discharge.

Exclude diagnoses that cannot be mapped to Table CC-Mapping.

Comorbidities Refer to the *Risk Adjustment Comorbidity Category Determination* in the *Guidelines for Risk Adjusted Utilization Measures.*

Risk Adjustment Weighting

For each IHS among nonoutliers, use the following steps to identify risk adjustment weights based on observation stays status at discharge, surgeries, discharge condition, comorbidity, age and gender. Weights are specific to product line (Medicare Under 65, Medicare 65+, commercial, Medicaid). Refer to the reporting indicator column in the risk adjustment tables to ensure that weights are linked appropriately.

Note: *For Medicare product lines, IHS that are discharged or transferred to skilled nursing care should be assigned two sets of risk adjustment weights; the skilled nursing care risk weights for reporting in Table PCR-C-3 and the standard set of risk weights for reporting in Table PCR-A-3 and Table PCR-B-3. For reporting IHS that are discharged or transferred to skilled nursing care, do not assign the skilled nursing care risk weights for the stays when reporting in Table PCR-A-3 and Table PCR-B-3 and do not assign the standard set or risk weights for the stays when reporting in Table PCR-C-3.*

Step 1 For each IHS discharge that is an observation stay, link the observation stay IHS weight.

Step 2 For each IHS with a surgery, link the surgery weight.

Step 3 For each IHS with a discharge CC Category, link the primary discharge weights.

Step 4 For each IHS with a comorbidity HCC Category, link the comorbidity weights.

Step 5 Link the age and gender weights for each IHS.

Step 6 Sum all weights associated with the IHS (i.e., observation stay, presence of surgery, principal discharge diagnosis, comorbidities, age and gender) and use the formula below to calculate the Estimated Readmission Risk for each IHS:

Estimated Readmission Risk = $\frac{e^{(\sum \text{WeightsForIHS})}}{1+e^{(\sum \text{WeightsForIHS})}}$

OR

Estimated Readmission Risk = [exp (sum of weights for IHS)] / [1 + exp (sum of weights for IHS)]

Note: *"Exp" refers to the exponential or antilog function.*

Truncate the estimated readmission risk *for each IHS* to 10 decimal places. Do not truncate or round in previous steps.

Step 7 Calculate the Count of Expected Readmissions for each age and stratification category. The Count of Expected Readmissions is the sum of the Estimated Readmission Risk calculated in step 6 for each IHS in each age and stratification category.

$$Count\ of\ Expected\ Readmissions = \sum(\text{Estimated Readmission Risk})$$

Step 8 Use the formula below and the Estimated Readmission Risk calculated in step 6 to calculate the variance for each IHS.

Variance = Estimated Readmission Risk x (1 – Estimated Readmission Risk)

Truncate the variance *for each IHS* to 10 decimal places.

For example: If the Estimated Readmission Risk is 0.1518450741 for an IHS, then the variance for this IHS is 0.1518450741 x 0.8481549259 = 0.1287881475.

Note: *Organizations must sum the variances for each stratification and age when populating the Variance cells in the reporting tables. When reporting, round the variance to 4 decimal places using the .5 rule.*

Numerator At least one acute readmission for any diagnosis within 30 days of the Index Discharge Date.

Step 1 Identify all acute inpatient and observation stays with an admission date on or between January 3 and December 31 of the measurement year. To identify acute inpatient and observation admissions:

1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set) and observation stays (Observation Stay Value Set).
2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set).
3. Identify the admission date for the stay.

Step 2 *Direct transfers:* For discharges with one or more direct transfers, use the last discharge.

Using the discharges identified in step 1, identify direct transfers between acute inpatient and observation or between observation and acute inpatient using the definition found in the *Guidelines for Risk Adjusted Utilization Measures.*

Step 3 Exclude acute hospitalizations with any of the following criteria on the discharge claim:

- Members with a principal diagnosis of pregnancy (Pregnancy Value Set).
- A principal diagnosis for a condition originating in the perinatal period (Perinatal Conditions Value Set).
- A planned hospital stay using any of the following:
  - A principal diagnosis of maintenance chemotherapy (Chemotherapy Encounter Value Set).
  - A principal diagnosis of rehabilitation (Rehabilitation Value Set).
  - An organ transplant (Kidney Transplant Value Set, Bone Marrow Transplant Value Set, Organ Transplant Other Than Kidney Value Set, Introduction of Autologous Pancreatic Cells Value Set).
  - A potentially planned procedure (Potentially Planned Procedures Value Set) without a principal acute diagnosis (Acute Condition Value Set).

Note: *For hospital stays where there was a direct transfer (identified in step 2), use the original stay and any direct transfer stays to identify exclusions in this step.*

Step 4 For each IHS identified in the denominator, determine if any of the acute inpatient and observation stays identified in the numerator have an admission date within 30 days after the Index Discharge Date.

Note: *Count each acute hospitalization only once toward the numerator for the last denominator event.*

If a single numerator event meets criteria for multiple denominator events, only count the last denominator event. For example, consider the following events:

- *Acute inpatient stay 1: May 1–10.*
- *Acute inpatient stay 2: May 15–25 (principal diagnosis of maintenance chemotherapy).*
- *Acute inpatient stay 3: May 30–June 5.*

All three acute inpatient stays are included as denominator events. Stay 2 is excluded from the numerator because it is a planned hospitalization. Stay 3 is within 30 days of Stay 1 and Stay 2. Count Stay 3 as a numerator event only toward the last denominator event (Stay 2, May 15–25).

Reporting: Number of Members in Plan Population

Step 1 Determine the member's age as of the earliest Index Discharge Date.

Step 2 Report the count of members in the plan population for each age group as the MemberCount.

Reporting: Number of Outliers

Step 1 Determine the member's age as of the earliest Index Discharge Date.

Step 2 Report the count of outlier members for each age group as the OutlierMemberCount.

Calculated: Outlier Rate

The number of outlier members (OutlierMemberCount) divided by the number of members in the plan population (MemberCount), displayed as a permillage (multiplied by 1,000), for each age group and totals. Calculated by IDSS as the OutlierRate.

Reporting: Denominator

Count the number of IHS among nonoutlier members for each age group. Report these values as the Denominator.

Reporting: SES Stratification (Medicare only)

Step 1 Determine the member's SES stratifications as of the end of the continuous enrollment period for each Medicare discharge:

- *Non-LIS/DE, Nondisability:* Member is eligible for Medicare due to age only (does not receive LIS, is not DE for Medicaid, does not have disability status).
- *LIS/DE:* Member is eligible for Medicare due to age and receives LIS (includes members eligible for Medicare due to DE), does not have disability status.
- *Disability*: Member is eligible for Medicare due to disability status only.
- *LIS/DE and Disability:* Member is eligible for Medicare, receives LIS and has disability status.
- *Other:* Member has ESRD-only status or is assigned "9—none of the above."
- *Unknown:* Member's SES is unknown.
- *Total Medicare:* Total of all categories.

Step 2 Report Medicare discharges based on the SES stratification assigned for each Medicare index stay in Table PCR-B-3.

Reporting: Skilled Nursing Care Stratification (Medicare 65+ only)

Step 1 For Medicare nonoutlier members 65 years of age and older, determine if the IHS was discharged or transferred to skilled nursing care (Skilled Nursing Stay Value Set).

An index stay is discharged or transferred to skilled nursing care when the discharge date from the acute inpatient or observation stay precedes the admission date for skilled nursing care by one calendar day or less. For example:

- An index stay discharge on June 1, followed by an admission to a skilled nursing setting on June 1, *is an IHS discharged or transferred to skilled nursing care.*
- An index stay discharge on June 1, followed by an admission to a skilled nursing setting on June 2, *is an IHS discharged or transferred to skilled nursing care.*
- An index stay discharge on June 1, followed by an admission to a skilled nursing setting on June 3, *is not an IHS discharged or transferred to skilled nursing care*.

Step 2 Report Medicare discharges for each IHS discharged or transferred to skilled nursing care to an age group in Table PCR-C-3.

Reporting: Numerator

Count the number of observed IHS among nonoutlier members with a readmission within 30 days of discharge for each age group and report these values as the ObservedCount.

Calculated: Observed Readmission Rate

The Count of Observed 30-Day Readmissions (ObservedCount) divided by the Count of Index Stays (Denominator) for each age group and totals. Calculated by IDSS as the ObservedRate.

Reporting: Count of Expected 30-Day Readmissions

Step 1 Calculate the Count of Expected Readmissions among nonoutlier members for each age group.

Step 2 Round to 4 decimal places using the .5 rule and report these values as the ExpectedCount.

Calculated: Expected Readmission Rate

The Count of Expected 30-Day Readmissions (ExpectedCount) divided by the Count of Index Stays (Denominator) for each age group and totals. Calculated by IDSS as the ExpectedRate.

Reporting: Variance

Step 1 Calculate the total (sum) variance for each SES stratification (Medicare only), skilled nursing stratification (Medicare only) and age group.

Step 2 Round to 4 decimal places using the .5 rule and report these values as the CountVariance.

Calculated: O/E Ratio

The Count of Observed 30-Day Readmissions (ObservedCount) divided by the Count of Expected 30-Day Readmissions (ExpectedCount) for each age group and totals. Calculated by IDSS as the OE. The O/E Ratio is not calculated for SES stratifications.

Note

- *Supplemental data may not be used for this measure.*

Table PCR-A-1/2: Data Element for Plan All-Cause Readmissions

Metric	Age	Data Element	Reporting Instructions
PlanAllCauseReadmissions	18-44	MemberCount	For each Stratification
	45-54	OutlierMemberCount	For each Stratification
	55-64	OutlierRate	OutlierMemberCount / MemberCount (Permille)
	18-64	Denominator	For each Stratification
		ObservedCount	For each Stratification
		ObservedRate	ObservedCount / Denominator (Percent)
		ExpectedCount	For each Stratification
		ExpectedRate	ExpectedCount / Denominator (Percent)
		CountVariance	For each Stratification
		OE	ObservedCount / ExpectedCount

Table PCR-A-3: Data Elements for Plan All-Cause Readmissions

Metric	Age	Data Element	Reporting Instructions
PlanAllCauseReadmissions	18-44	MemberCount	For each Stratification
	45-54	OutlierMemberCount	For each Stratification
	55-64	OutlierRate	OutlierMemberCount / MemberCount (Permille)
	18-64	Denominator	For each Stratification
	65-74	ObservedCount	For each Stratification
	75-84	ObservedRate	ObservedCount / Denominator (Percent)
	85+	ExpectedCount	For each Stratification
	65+	ExpectedRate	ExpectedCount / Denominator (Percent)
		CountVariance	For each Stratification
		OE	ObservedCount / ExpectedCount

Table PCR-B-3: Data Elements for Plan All-Cause Readmissions by SES Stratification

Metric	SES Stratification	Age	Data Element	Reporting Instructions
PlanAllCauseReadmissions	NonLisDeNondisability	18-64	Denominator	For each Stratification
	LisDe	65+	ObservedCount	For each Stratification
	Disability		ObservedRate	ObservedCount / Denominator (Percent)
	LisDeAndDisability		ExpectedCount	For each Stratification
	Other		ExpectedRate	ExpectedCount / Denominator (Percent)
	Unknown		CountVariance	For each Stratification

Table PCR-C-3: Data Elements for Plan All-Cause Readmissions for Skilled Nursing Care Stratification

Metric	Age	Data Element	Reporting Instructions
SkilledNursingCare	65-74	Denominator	For each Stratification
	75-84	ObservedCount	For each Stratification
	85+	ObservedRate	ObservedCount / Denominator (Percent)
	65+	ExpectedCount	For each Stratification
		ExpectedRate	ExpectedCount / Denominator (Percent)
		CountVariance	For each Stratification
		OE	ObservedCount / ExpectedCount

Rules for Allowable Adjustments of HEDIS

The "Rules for Allowable Adjustments of HEDIS" (the "Rules") describe how NCQA's HEDIS measure specifications can be adjusted for other populations, if applicable. The Rules, reviewed and approved by NCQA measure experts, provide for expanded use of HEDIS measures without changing their clinical intent.

Adjusted HEDIS measures *may not* be used for HEDIS health plan reporting.

The following table is for the Rules for Allowable Adjustments for **Risk-Adjusted Measurement** of the Plan All-Cause Readmissions measure (Count of Index Stays, Count of Observed 30-Day Readmissions, Observed Readmission Rate, Risk Adjustment Determination, Risk Adjustment Weighting, Count of Expected 30-Day Readmissions, Observed to Expected).

Eligible Population	Adjustments Allowed (Yes/No)	Notes
NONCLINICAL COMPONENTS		
Product lines	No	Organizations may not adjust product lines.
Ages	No	The age determination dates may not be changed. ***Note:*** *The denominator age may not be expanded. The ages for the risk weights may not be changed.*
Continuous enrollment, allowable gap, anchor date	No	For risk adjusted rates organizations are required to use enrollment criteria; adjustments are not allowed.
Benefits	Yes	Organizations are not required to use a benefit; adjustments are allowed.
Other	Yes, with limits	Organizations may only adjust additional eligible population within the eligible population to focus on gender, sociodemographic characteristics or geographical region. ***Note:*** *NCQA recommends evaluating risk model performance and validity within adjusted populations.* *Organizations may not adjust for a clinical subpopulation (e.g., members with a diabetes diagnosis).*
Plan population	Yes	Organizations are not required to used plan population to identify outlier rates.
CLINICAL COMPONENTS		
Stratifications	**Adjustments Allowed (Yes/No)**	**Notes**
• SES Stratification • Skilled Nursing Care Stratification	No, if applied	Stratifications not required, but if they are used the value sets, logic and product lines may not be changed.
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Event/diagnosis	Yes, with limits	Only events or diagnoses that contain (or map to) codes in value sets may be used to identify visits. The value sets and logic may not be changed.

Eligible Population	Adjustments Allowed (Yes/No)	Notes
		Note: *Organizations may include denied claims to calculate the denominator.*
Outlier	Yes, with limits	Organizations may not adjust the outlier logic. ***Note:*** *Organizations may include denied claims to calculate these events.*
Denominator Exclusions	**Adjustments Allowed (Yes/No)**	**Notes**
Required exclusions	No	The hospice exclusion is required. The value sets and logic may not be changed.
Risk Adjustment and Calculation of Expected Events	**Adjust Adjustments Allowed (Yes/No)**	**Notes**
• Risk Adjustment Determination • Risk Adjustment Weighting • Expected Readmissions • Variance	Yes, with limits	Risk adjustment determinations, weighting and calculations of expected events logic may not be changed. ***Note:*** *Organizations may include denied claims to calculate these events.*
Numerator Criteria	**Adjustments Allowed (Yes/No)**	**Notes**
Unplanned Acute Readmission	Yes, with limits	Value sets and logic may not be changed. ***Note:*** *Organizations may include denied claims to calculate the numerator.*

Rules for Allowable Adjustments of HEDIS

The "Rules for Allowable Adjustments of HEDIS" (the "Rules") describe how NCQA's HEDIS measure specifications can be adjusted for other populations, if applicable. The Rules, reviewed and approved by NCQA measure experts, provide for expanded use of HEDIS measures without changing their clinical intent.

Adjusted HEDIS measures *may not* be used for HEDIS health plan reporting.

The following table is for the Rules for Allowable Adjustments for **Observed Measurement** of the Plan All-Cause Readmissions Observed Events measure (Count of Index Stays, Count of Observed 30-Day Readmissions, Observed Readmission Rate).

NONCLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Product lines	Yes	When adjusting this measure to assess for observed events only, organizations are not required to use product line criteria; product lines may be combined and all (or no) product line criteria may be used.
Ages	Yes, with limits	The age determination dates may be changed (e.g., select, "age 50 months as of June 30"). ***Note:*** *The denominator age may not be expanded.*
Continuous enrollment, allowable gap, anchor date	Yes	Organizations are not required to use enrollment criteria; adjustments are allowed.
Benefits	Yes	Organizations are not required to use a benefit; adjustments are allowed.
Other	Yes	Organizations may use additional eligible population criteria to focus on an area of interest defined by gender, race, ethnicity, socioeconomic or sociodemographic characteristics, geographic region or another characteristic.
Plan population	Yes	Organizations are not required to used plan population to identify outlier rates.
CLINICAL COMPONENTS		
Stratifications	**Adjustments Allowed (Yes/No)**	**Notes**
• SES Stratification • Skilled Nursing Care Stratification	No, if applied	Stratifications are not required, but if they are used, the value sets, logic and product lines may not be changed.
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Event/diagnosis	Yes, with limits	Only events or diagnoses that contain (or map to) codes in value sets may be used to identify visits. The value sets and logic may not be changed. ***Note:*** *Organizations may include denied claims to calculate the denominator.*

Eligible Population	Adjustments Allowed (Yes/No)	Notes
Outlier	Yes, with limits	Organizations may not adjust the outlier logic. ***Note:*** *Organizations may include denied claims to calculate these events.*
Denominator Exclusions	**Adjustments Allowed (Yes/No)**	**Notes**
Required exclusions	Yes	The hospice exclusion is not required. Refer to Exclusions in the *Guidelines for the Rules for Allowable Adjustments*.
Numerator Criteria	**Adjustments Allowed (Yes/No)**	**Notes**
Unplanned Acute Readmission	Yes, with limits	Value sets and logic may not be changed. ***Note:*** *Organizations may include denied claims to calculate the numerator.*

Measures Reported Using Electronic Clinical Data Systems

Guidelines for Measures Reported Using Electronic Clinical Data Systems (ECDS)

SUMMARY OF CHANGES TO HEDIS MY 2023

Updated the disclaimer under the *Description*.

Removed the definitions of "QDE" and "audit requirements" from *Guideline 2: HEDIS Definitions and Requirements for ECDS Reporting*.

Added definitions for "dQM," "ELM" and "denominator" to Guideline 2.

Combined the definitions for "FHIR" and "resources" in Guideline 2.

Updated the definition of "CQL" in Guideline 2.

Removed definitions for "structured data" and "semistructured data" from *Guideline 4: Types of ECDS Data*.

Updated *Guideline 6: HEDIS Digital Measure Format*.

Description

HEDIS quality measures reported using ECDS inspire innovative use of electronic clinical data to document high-quality patient care that demonstrates commitment to evidence-based practices. Organizations that report HEDIS using ECDS encourage exchange of the information needed to provide high-quality services, ensuring that the information reaches the right people at the right time.

The ECDS reporting standard represents a step forward in adapting HEDIS to accommodate the expansive information available in electronic clinical datasets used for patient care and quality improvement.

ECDS are the network of data containing a plan member's personal health information and records of their experiences within the health care system. They may also support other care-related activities directly or indirectly, including evidence-based decision support, quality management and outcome reporting. Data in these systems are structured such that automated quality measurement queries can be consistently and reliably executed, providing results quickly and efficiently to the team responsible for the care of health plan members.

Health plans that establish an enterprise network of interoperable electronic data systems will foster a member-centered, team-based approach to improving health care quality and better communication across health care service providers. Visit www.ncqa.org/ecds for more information and FAQs about ECDS reporting.

Disclaimer

The specifications included in this publication provide a summary of the measures that may be reported using ECDS. The specifications are for *reference purposes only* and should not be used for programming the measures. HEDIS digital measure specifications for programming purposes are available in the NCQA Store as NCQA's Digital Measure Packages. Organizations are accountable for all potential updates included in the Digital Measure Packages, including those published with the Volume 2 *Technical Update.*

Guidelines

HEDIS measures reported using ECDS follow the *General Guidelines for Data Collection and Reporting,* unless there is an ECDS-specific guideline listed below that overrides those rules. This hierarchy only exists in specific cases where there is an ECDS guideline listed. If no ECDS-specific guideline is listed, then organizations should default to those found in *General Guidelines for Data Collection and Reporting.*

1. Initial Population

The initial population for any HEDIS measure reported using ECDS includes all members who satisfy criteria, including age and participation criteria. Refer to the logic in the digital measure package for criteria used to define the initial population.

2. HEDIS Definitions and Requirements for ECDS Reporting

Master data management	Organizational policies and procedures for governance of quality measure data; specifically, defining creation of a master data file for quality reporting. An organization must use a master data management process to verify data integrity across the multiple data systems used in HEDIS ECDS reporting, ensuring consistent identification and classification of data elements and standardized data reconciliation procedures. Reconciliation procedures include, but are not limited to, validation, deduplication and reliability checks to correct latent data errors related to either the acquisition of encrypted data or ETL (extract, transform and load) integration of data into a master file.
dQM	Digital Quality Measure. A quality measure published as a downloadable package that includes the technical specifications provided as a standard, machine readable, interoperable format. Visit https://www.ncqa.org/dqm for more information.
FHIR	Fast Healthcare Interoperability Resources. A specification standard for exchanging healthcare information electronically that supports the exchange of structured and standardized data.[12] Exchangeable content in FHIR are resources defined and represented in common ways. They are built from data types that define common reusable patterns of elements and share a common set of metadata.
ELM	Expression Logical Model.[13] A Unified Modeling Language™ specification for representing measure logic independent of syntax and special-purpose constructs introduced at the syntactic level. It is intended to enable distribution and sharing of computable quality logic.
CQL	Clinical Quality Language.[14] A Health Level Seven International (HL7) domain-specific language focused on clinical quality and targeted at measure authors. The CQL specification describes a machine-readable canonical representation called ELM designed to enable sharing of clinical knowledge.
Participation	The identifiers and descriptors for each organization's coverage used to define a member's eligibility for measure reporting. Allocation for HEDIS is based on a member's eligibility during the participation period.

[12] https://www.hl7.org/fhir/overview.html
[13] https://cql.hl7.org/13-f-glossary.html
[14] https://cql.hl7.org/

Denominator

What is reported to NCQA for the measure denominator results, defined as “the initial population, minus exclusions” in the narratives for measures reported using ECDS included in this publication and in the human-readable files.

In contrast, digital measure CQL execution defines the denominator as the initial population prior to the removal of members meeting denominator exclusions.

3. Data Collection Methods

Electronic Method

Measures reported using ECDS are specified for the Electronic Method of data collection.

Electronic transactional data may be used to identify the initial population.

To qualify for HEDIS ECDS reporting, data must use standard layouts, meet the measure technical specification requirements and be accessible by the care team upon request. Organizations meet this requirement if they are able to provide the requested information (e.g., phone, secure email, direct feed, provider portal, file request) to providers who are treating their members. Organizations should have documented processes for tracking these requests to be reviewed as part of the HEDIS audit.

Practitioners or practitioner groups that are accountable for clinical services provided to members must not be prevented from accessing any data used by a health plan for quality measure reporting, regardless of the initial Source System of Record (SSoR).

SSoR

HEDIS measure data in ECDS reporting are submitted by each SSoR accessed to produce the measure result. The SSoR is the authoritative dataset containing the standardized elements the organization requires to generate and report digital quality measure results.

Datasets for ECDS reporting may natively contain both standard and nonstandard data. Refer to *General Guideline 29: Supplemental Data* for electronic clinical data proof-of-service and verification requirements. Each electronic data source used for HEDIS ECDS reporting must have:

- Policies and procedures for establishing and maintaining database management systems.
- Standard layout requirements.
- An automated process for extraction, transformation and loading of all data elements to the master file.

Each SSoR is a data repository where semantic differences in non-standard data have been resolved through integrity testing, and the data has been structured so it can be reliably queried by a HEDIS dQM.

Source priority

When quality data elements to support the measure are identified in multiple data sources, a hierarchy is applied.

Each SSoR used for HEDIS ECDS reporting is categorized using the following priority:

1. Electronic health record (EHR)/personal health record (PHR) (the system of data origin such as laboratory, pharmacy, pathology, radiology).
2. Health information exchange (HIE)/clinical registry.
3. Case management registry.
4. Administrative.

Organizations compare the list of all unique systems containing relevant member data and assign members based on the highest-ranked data category in the hierarchy. SSoRs are mapped using the data type that is loaded to the master file that identifies member eligibility for each component of a quality measure. The applied hierarchy does not imply relevance or validity of a data source; rather, it is applied in cases where a member's data are in multiple locations.

Members are assigned to only one SSoR category for each measure element (e.g., initial population, denominator, exclusions, numerator).

For example, if administrative data are used to identify the initial population, the member is assigned to the Administrative cohort for the initial population. If a numerator event is identified through a query of the organization's case management system, the member is assigned to that cohort for the numerator even though that member may have been included in the measure's initial population using administrative data.

Organizations must complete data collection for the SSoRs by the supplemental data collection deadline. Refer to *General Guideline 8: Audit Preparation* for information about the timeline. When appropriate, an SSoR can be refreshed according to the organization's scheduled refreshes and accounted appropriately for the measure. Refer to *General Guideline 30: Obtaining Information for the Systematic Sample*.

4. Types of ECDS Data

Organizations may use several data sources to provide complete information about the quality of health services delivered to its members. Data systems that may be eligible for HEDIS ECDS reporting include, but are not limited to, member eligibility files, EHRs, PHRs, clinical registries, HIEs, administrative claims systems, electronic laboratory reports (ELR), electronic pharmacy systems, immunization information systems (IIS) and disease/case management registries.

The data within these systems come in a variety of formats. The format type determines how the source is audited. Member-reported services are acceptable if the information is recorded, dated and maintained in the member's legal health record. The member-reported data must follow *General Guideline 39: Member-Reported Services and Biometric Values.*

Data sources are categorized using the following criteria:

EHR/PHR

EHRs and PHRs are transactional systems that store clinically relevant information collected directly from or managed by a patient. An EHR contains the medical and treatment histories of patients; a PHR includes both the standard clinical data collected in a provider's office or another care setting, in addition to information curated directly in the PHR by the patient though an application programming interface (API).

This data category includes biometric information and clinical samples obtained directly from a patient as well as clinical findings resulting from samples collected from a patient (e.g., pathology, laboratory and pharmacy records generated from entities not directly connected to the patient's EHR).

HIE/clinical registry

HIEs and clinical registries eligible for this reporting category include state HIEs, IIS, public health agency systems, regional HIEs (RHIO), Patient-Centered Data Homes™ or other registries developed for research or to support quality improvement and patient safety initiatives.

Doctors, nurses, pharmacists, other health care providers and patients can use HIEs to access and share vital medical information, with the goal of creating a complete patient record.[15] HIEs used for ECDS reporting must use standard protocols to ensure security, privacy, data integrity, sender and receiver authentication and confirmation of delivery.

Clinical registries collect information about people with a specific disease or condition, or patients who may be willing to participate in research about a disease. Registries can be sponsored by a government agency, nonprofit organization, health care facility or private company, and decisions regarding use of the data in the registry are the responsibility of the registry's governing committee.[16]

Case management system

A shared database of member information collected through a collaborative process of member assessment, care planning, care coordination or monitoring of a member's functional status and care experience.

Case management systems eligible for this category of ECDS reporting include any system developed to support the organization's case/disease management activities, including activities performed by delegates.

Administrative

Includes data from administrative claims processing systems for all services incurred (paid, suspended, pending and denied) during the period defined by each measure's participation as well as member management files, member eligibility and enrollment files, electronic member rosters, internal audit files, and member call service databases.

5. Member Allocation for HEDIS ECDS Reporting

Member eligibility is determined by participation in the organization during the participation period. Include all eligible members with the measure-specified benefits during the specified participation period. Members must be allocated to a product/product line for HEDIS ECDS reporting. Allocation is based on the following enrollment unless otherwise specified in the guidance section of the measures:

[15] https://www.healthit.gov/providers-professionals/health-information-exchange/what-hie
[16] https://www.nih.gov/health-information/nih-clinical-research-trials-you/list-registries

- For commercial and Medicare, members must be enrolled with no gap of more than 45 days during each year of the specified participation period.
- For Medicaid, member enrollment is verified monthly and the member may not have a gap of more than 30 days during each year of the specified participation period.
- For measures that are episode-based, members must be enrolled on the day of the event. For all other measures, members must be enrolled on the last day of the measurement period.

6. HEDIS Digital Measure Format

HEDIS dQMs are published as a package of files required for the successful implementation of the measures, however there are several sections where reporting organizations must program measure components not included in the digital measure package (e.g., product line stratifications). For each measure-specific instance of this, refer to the human-readable file contained in the measure package for guidance.

HEDIS dQMs are published in a format which allows them to be directly utilized by a reporting organization. Each dQM package includes both the human-readable technical documentation and the machine-readable files necessary for implementation. Each HEDIS dQM package includes the following files:

- **Measure-specific human-readable file (in the "Narrative" folder):** A human-readable document representing the measure narrative in a way that is easy to understand for persons not familiar with computer coded files.
  1. *{measure id}_HEDIS_MY{MY}-{version}.html*: Contains a measure header table with text descriptions of the measure elements (initial population, denominator, exclusions, numerator) and the data element types, and references.

     This file also contains human-readable versions of the measure logic expressed in CQL. This portion of the file outlines the population criteria measure logic definitions, functions and relevant value sets formatted for easy review by measure implementors. Each measure section expressed in CQL corresponds with the descriptive text found in the measure header.
- **Technical Release Notes file (in the "Documents" folder):** Contains a comprehensive list of updates made to measures since their prior release. The *Summary of Changes* section in the *HEDIS Volume 2 Technical Specifications* publication at the beginning of every measure and in Appendix 1 previews the updates.
- **HEDIS Implementation Guide file (in the "Documents" folder):** Contains a guide to assist with implementation of the HEDIS dQMs, including information on available libraries, data-mapping considerations and developer-level documentation that describes the overall approach for calculating quality measures.
- **ELM files (in the "cql," "libraryCql," "elm" and "libraryElm" folders):** The machine-readable representation of the FHIR/CQL that has been designed for implementation of the quality measure. The ELM file provides the semantics necessary to retrieve the correct data from the measure reporting database.
  1. *{library name or measure ID}-{library or measure version}.cql*: A FHIR/CQL representation of all the data and the expressions created in the CQL library.
  2. *{library name or measure ID}-{library or measure version}.xml*: A standardized, machine-friendly representation of the FHIR/CQL in Extensible Markup Language (XML) format.
  3. *{library name or measure ID}-{library or measure version}.json*: A standardized, machine-friendly representation of the FHIR/CQL in JavaScript Object Notation (JSON) format.
- **Value set files (in the "valuesets" folder):** A FHIR representation of the value sets applicable to the measure in JSON format.

- ***Note:*** *NCQA recommends that all files in a measure package be housed in the same place and that naming conventions be preserved because the computer-readable files reference the library files by name.*

Complete measure packages for HEDIS digital measures are released by NCQA in conjunction with this publication and are available for download from the NCQA Store.

7. Presentation of Codes in HEDIS Digital Measures

HEDIS dQMs reference single codes and value sets that must be used for HEDIS reporting.

Value sets

If there is more than one eligible code to identify a data element, a value set is used. Organizations can refer to the Value Set Directory (VSD) for codes in the value sets, which are also in the measure package in JSON format.

Value sets are listed in the *Data criteria (element level)* section of the measure with an accompanying uniform resource identifier (URI). The OID listed in the URI is the key to locating codes in the HEDIS VSD or HEDIS Medication List Directory (MLD). The OID must be used to identify the correct value set to be used for each specific data element in the dQMs for HEDIS reporting.

- ***Note:*** *Value sets specified as a medication resource are found in the MLD.*

Direct reference codes

If only a single code is required to identify a data element, the code is listed in the *Data criteria (element level)* section of the human-readable file under the *Direct Reference Codes and Codesystems* heading. These codes are also included in the Direct Reference Codes spreadsheet of the VSD.

Breast Cancer Screening (BCS-E)

SUMMARY OF CHANGES TO HEDIS MY 2023

- Refer to the Technical Release Notes file in the Digital Measures Package for a comprehensive list of changes.
- Added new data elements tables for race and ethnicity stratification reporting.
- Revised the "other" criteria of the Nonclinical Components in the *Rules for Allowable Adjustments.*

Description	The percentage of women 50–74 years of age who had a mammogram to screen for breast cancer.
Measurement period	January 1–December 31.
Clinical recommendation statement	The U.S. Preventive Services Task Force recommends screening women 50–74 years of age for breast cancer every 2 years. (B recommendation)
Citations	U.S. Preventive Services Task Force. 2016. "Screening for Breast Cancer: U.S. Preventive Services Task Force Recommendation Statement. *Ann Intern Med* 164(4):279–96.
Characteristics	
Scoring **Type** **Stratification**	Proportion. Process. • Breast Cancer Screening. – Product line: ▪ Commercial. ▪ Medicaid. ▪ Medicare. – SES (for Medicare only): ▪ SES—Non-LIS/DE, Nondisability. ▪ SES—LIS/DE. ▪ SES—Disability. ▪ SES—LIS/DE and Disability. ▪ SES—Other. ▪ SES—Unknown. – Race (for each product line): ▪ Race—White. ▪ Race—Black or African American. ▪ Race—American Indian or Alaska Native. ▪ Race—Asian.
	▪ Race—Native Hawaiian or Other Pacific Islander.

	▪ Race—Some Other Race. ▪ Race—Two or More Races. ▪ Race—Asked but No Answer. ▪ Race—Unknown. – Ethnicity (for each product line): ▪ Ethnicity—Hispanic or Latino. ▪ Ethnicity—Not Hispanic or Latino. ▪ Ethnicity—Asked but No Answer. ▪ Ethnicity—Unknown.
Risk adjustment	None.
Improvement notation	A higher rate indicates better performance.
Guidance	• For Medicare plans, I-SNP and LTI exclusions are not included in the measure calculation logic and need to be programmed manually. Administrative data must be used for these exclusions. • Non-administrative data may be used for the frailty and advanced illness exclusion. **Allocation:** The member was enrolled with a medical benefit throughout the participation period. No more than one gap in enrollment of up to 45 days for each full calendar year of the participation period (i.e., the measurement period and the year prior to the measurement period). No gaps in enrollment are allowed from October 1 two years prior to the measurement period through December 31 two years prior to the measurement period. When identifying members in hospice, the requirements described in *General Guideline 15* for identification of hospice members using the monthly membership detail data files are not included in the measure calculation logic and need to be programmed manually. **Reporting:** For Medicare plans, the SES stratifications are mutually exclusive. NCQA calculates a total rate for Medicare plans by adding all six Medicare stratifications. For all plans, the race and ethnicity stratifications are mutually exclusive and the sum of all categories in each stratification is the total population. SES and product line stratifications are not included in the measure calculation logic and need to be programmed manually. The race and ethnicity stratifications are reported by data source—direct or indirect.

Definitions	
Participation	The identifiers and descriptors for each organization's coverage used to define members' eligibility for measure reporting. Allocation for reporting is based on eligibility during the participation period.
Participation period	October 1 two years prior to the measurement period through the end of the measurement period.
Initial population	Women 52–74 years of age by the end of the measurement period who also meet the criteria for participation.
Exclusions	• Members in hospice or using hospice services any time during the measurement period. • Members who had a bilateral mastectomy or both right and left unilateral mastectomies any time during the member's history through the end of the measurement period. Any of the following meet the criteria for bilateral mastectomy: – Bilateral mastectomy (Bilateral Mastectomy Value Set). – Unilateral mastectomy (Unilateral Mastectomy Value Set) with a bilateral modifier (Bilateral Modifier Value Set) (same procedure). – Unilateral mastectomy found in clinical data (Clinical Unilateral Mastectomy Value Set) with a bilateral modifier (Clinical Bilateral Modifier Value Set) (same procedure). ***Note:*** *The "clinical" mastectomy value sets identify mastectomy; the word "clinical" refers to the data source, not to the type of mastectomy.* – History of bilateral mastectomy (History of Bilateral Mastectomy Value Set). • Any combination of codes from the table below that indicate a mastectomy on ***both*** the left ***and*** right side on the same or different dates of service.

Left Mastectomy (any of the following)	**Right Mastectomy (any of the following)**
Unilateral mastectomy (Unilateral Mastectomy Value Set) ***with*** a left-side modifier (Left Modifier Value Set) (same procedure)	Unilateral mastectomy (Unilateral Mastectomy Value Set) ***with*** a right-side modifier (Right Modifier Value Set) (same procedure)
Unilateral mastectomy found in clinical data (Clinical Unilateral Mastectomy Value Set) ***with*** a left-side modifier (Clinical Left Modifier Value Set) (same procedure)	Unilateral mastectomy found in clinical data (Clinical Unilateral Mastectomy Value Set) ***with*** a right-side modifier (Clinical Right Modifier Value Set) (same procedure)
Absence of the left breast (Absence of Left Breast Value Set)	Absence of the right breast (Absence of Right Breast Value Set)
Left unilateral mastectomy (Unilateral Mastectomy Left Value Set)	Right unilateral mastectomy (Unilateral Mastectomy Right Value Set)

	• Medicare members 66 years of age and older by the end of the measurement period who meet either of the following: – Enrolled in an Institutional SNP (I-SNP) any time during the measurement period. – Living long-term in an institution any time during the measurement period, as identified by the LTI flag in the monthly membership detail data file. Use the run date of the file to determine if a member had an LTI flag during the measurement period. • Members 66 years of age and older by the end of the measurement period, with frailty and advanced illness. Members must meet BOTH of the following frailty and advanced illness criteria to be excluded: – At least two indications of frailty (Frailty Device Value Set; Frailty Diagnosis Value Set; Frailty Encounter Value Set; Frailty Symptom Value Set) with different dates of service during the measurement period. – Any of the following during the measurement period or the year prior to the measurement period (count services that occur over both years): ▪ At least two outpatient visits (Outpatient Value Set), observation visits (Observation Value Set), ED visits (ED Value Set), telephone visits (Telephone Visits Value Set), e-visits or virtual check-ins (Online Assessments Value Set), nonacute inpatient encounters (Nonacute Inpatient Value Set) or nonacute inpatient discharges (instructions below; the diagnosis must be on the discharge claim) on different dates of service, with an advanced illness diagnosis (Advanced Illness Value Set). Visit type need not be the same for the two visits. To identify a nonacute inpatient discharge: 1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set). 2. Confirm the stay was for nonacute care based on the presence of a nonacute code (Nonacute Inpatient Stay Value Set) on the claim. 3. Identify the discharge date for the stay. ▪ At least one acute inpatient encounter (Acute Inpatient Value Set) with an advanced illness diagnosis (Advanced Illness Value Set). ▪ At least one acute inpatient discharge with an advanced illness diagnosis (Advanced Illness Value Set) on the discharge claim. To identify an acute inpatient discharge: 1. Identify all acute and nonacute inpatient stays (Inpatient Stay Value Set). 2. Exclude nonacute inpatient stays (Nonacute Inpatient Stay Value Set). 3. Identify the discharge date for the stay. ▪ A dispensed dementia medication (Dementia Medications List). • Members receiving palliative care (Palliative Care Assessment Value Set; Palliative Care Encounter Value Set; Palliative Care Intervention Value Set; ICD-10-CM code Z51.5) any time during the measurement period.
Denominator	The initial population, minus exclusions.

Numerator	One or more mammograms (Mammography Value Set) any time on or between October 1 two years prior to the measurement period and the end of the measurement period.

Data criteria (element level)

Value Sets:

- **BCSE_HEDIS_MY2023-2.0.0**
  - Absence of Left Breast (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1329)
  - Absence of Right Breast (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1330)
  - Bilateral Mastectomy (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1042)
  - Bilateral Modifier (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1043)
  - Clinical Bilateral Modifier (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1951)
  - Clinical Left Modifier (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1949)
  - Clinical Right Modifier (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1950)
  - Clinical Unilateral Mastectomy (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1948)
  - History of Bilateral Mastectomy (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1331)
  - Left Modifier (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1148)
  - Mammography (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1168)
  - Right Modifier (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1230)
  - Unilateral Mastectomy (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1256)
  - Unilateral Mastectomy Left (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1334)
  - Unilateral Mastectomy Right (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1335)
- **NCQA_AdvancedIllnessandFrailty-2.0.0**
  - Acute Inpatient (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1810)
  - Advanced Illness (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1465)
  - Dementia Medications (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1729)
  - ED (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1086)
  - Frailty Device (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1530)
  - Frailty Diagnosis (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1531)
  - Frailty Encounter (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1532)
  - Frailty Symptom (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1533)
  - Nonacute Inpatient (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1189)
  - Observation (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1191)
  - Online Assessments (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1446)
  - Outpatient (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1202)
  - Telephone Visits (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1246)

- **NCQA_Claims-2.0.0**
  - Inpatient Stay (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1395)
  - Nonacute Inpatient Stay (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1398)
- **NCQA_Hospice-2.0.0**
  - Hospice Encounter (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1761)
  - Hospice Intervention (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1762)
- **NCQA_PalliativeCare-2.0.0**
  - Palliative Care Assessment (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.2225)
  - Palliative Care Encounter (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1450)
  - Palliative Care Intervention (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.2224)
- **NCQA_Stratification-1.0.0**
  - American Indian or Alaska Native Detailed Race (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.2365)
  - Asian Detailed Race (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.2366)
  - Black or African American Detailed Race (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.2367)
  - Hispanic or Latino Detailed Ethnicity (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.2368)
  - Native Hawaiian or Other Pacific Islander Detailed Race (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.2369)
  - White Detailed Race (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.2370)

Direct reference codes and codesystems:

- **NCQA_PalliativeCare-2.0.0**
  - codesystem "ICD-10-CM": 'http://hl7.org/fhir/sid/icd-10-cm'
  - code "Encounter for palliative care": 'Z51.5' from "ICD-10-CM" display 'Encounter for palliative care'
- **NCQA_Terminology-2.0.0**
  - codesystem "ActCode": 'http://terminology.hl7.org/CodeSystem/v3-ActCode'
  - codesystem "ClaimTypeCodes": 'http://terminology.hl7.org/CodeSystem/claim-type'
  - codesystem "ConditionClinicalStatusCodes": 'http://terminology.hl7.org/CodeSystem/condition-clinical'
  - codesystem "NullFlavor": 'http://terminology.hl7.org/CodeSystem/v3-NullFlavor'
  - codesystem "RaceAndEthnicityCDC": 'https://www.hl7.org/fhir/us/core/CodeSystem-cdcrec'
  - code "active": 'active' from "ConditionClinicalStatusCodes"
  - code "American Indian or Alaska Native": '1002-5' from "RaceAndEthnicityCDC" display 'American Indian or Alaska Native'
  - code "Asian": '2028-9' from "RaceAndEthnicityCDC" display 'Asian'
  - code "Asked but no answer": 'ASKU' from "NullFlavor" display 'Asked but no answer'
  - code "Black or African American": '2054-5' from "RaceAndEthnicityCDC" display 'Black or African American'
  - code "Hispanic or Latino": '2135-2' from "RaceAndEthnicityCDC" display 'Hispanic or Latino'

- code "Institutional": 'institutional' from "ClaimTypeCodes"
- code "managed care policy": 'MCPOL' from "ActCode"
- code "Native Hawaiian or Other Pacific Islander": '2076-8' from "RaceAndEthnicityCDC" display 'Native Hawaiian or Other Pacific Islander'
- code "Non Hispanic or Latino": '2186-5' from "RaceAndEthnicityCDC" display 'Non Hispanic or Latino'
- code "Other": 'OTH' from "NullFlavor" display 'Other'
- code "Pharmacy": 'pharmacy' from "ClaimTypeCodes"
- code "Professional": 'professional' from "ClaimTypeCodes"
- code "retiree health program": 'RETIRE' from "ActCode"
- code "subsidized health program": 'SUBSIDIZ' from "ActCode"
- code "Unknown": 'UNK' from "NullFlavor" display 'Unknown'
- code "White": '2106-3' from "RaceAndEthnicityCDC" display 'White'

Data Elements for Reporting

Organizations that submit data to NCQA must provide the following data elements in a specified file.

Table BCS-E-A-1/2: Data Elements for Breast Cancer Screening

Metric	Data Element	Reporting Instructions
BreastCancerScreening	InitialPopulation	Report once
	ExclusionsByEHR	Report once
	ExclusionsByCaseManagement	Report once
	ExclusionsByHIERegistry	Report once
	ExclusionsByAdmin	Report once
	Exclusions	(Sum over SSoRs)
	Denominator	Report once
	NumeratorByEHR	Report once
	NumeratorByCaseManagement	Report once
	NumeratorByHIERegistry	Report once
	NumeratorByAdmin	Report once
	Numerator	(Sum over SSoRs)
	Rate	(Percent)

Table BCS-E-A-3: Data Elements for Breast Cancer Screening

Metric	SES Stratification	Data Element	Reporting Instructions
BreastCancerScreening	NonLisDeNondisability	InitialPopulation	For each Stratification
	LisDe	ExclusionsByEHR	For each Stratification
	Disability	ExclusionsByCaseManagement	For each Stratification
	LisDeAndDisability	ExclusionsByHIERegistry	For each Stratification
	Other	ExclusionsByAdmin	For each Stratification
	Unknown	Exclusions	(Sum over SSoRs)
	Total	Denominator	For each Stratification
		NumeratorByEHR	For each Stratification
		NumeratorByCaseManagement	For each Stratification
		NumeratorByHIERegistry	For each Stratification
		NumeratorByAdmin	For each Stratification
		Numerator	(Sum over SSoRs)
		Rate	(Percent)

Table BCS-E-B-1/2/3: Data Elements for Breast Cancer Screening: Stratifications by Race

Metric
BreastCancerScreening

Race	Source	Data Element	Reporting Instructions
White	Direct	InitialPopulation	For each Stratification
BlackOrAfricanAmerican	Indirect	Exclusions	For each Stratification
AmericanIndianOrAlaskaNative	Total	Denominator	For each Stratification
Asian		Numerator	For each Stratification
NativeHawaiianOrOtherPacificIslander		Rate	(Percent)
SomeOtherRace			
TwoOrMoreRaces			
AskedButNoAnswer*			
Unknown**			

Table BCS-E-C-1/2/3: Data Elements for Breast Cancer Screening: Stratifications by Ethnicity

Metric
BreastCancerScreening

Ethnicity	Source	Data Element	Reporting Instructions
HispanicOrLatino	Direct	InitialPopulation	For each Stratification
NotHispanicOrLatino	Indirect	Exclusions	For each Stratification
AskedButNoAnswer*	Total	Denominator	For each Stratification
Unknown**		Numerator	For each Stratification
		Rate	(Percent)

*AskedButNoAnswer is only reported for Source='Direct.'
**Unknown is only reported for Source=’Indirect.’

Rules for Allowable Adjustments of HEDIS

The "Rules for Allowable Adjustments of HEDIS" (the "Rules") describe how NCQA's HEDIS measure specifications can be adjusted for other populations, if applicable. The Rules, reviewed and approved by NCQA measure experts, provide for expanded use of HEDIS measures without changing their clinical intent.

Adjusted HEDIS measures *may not* be used for HEDIS health plan reporting.

Rules for Allowable Adjustments of Breast Cancer Screening—ECDS

NONCLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Product lines	Yes	Organizations are not required to use product line criteria; product lines may be combined and all (or no) product line criteria may be used.
Ages	Yes, with limits	Age determination dates may be changed (e.g., select, "age as of June 30"). The denominator age range may be expanded to 40–74 years.
Allocation	Yes	Organizations are not required to use enrollment criteria; adjustments are allowed.
Benefit	Yes	Organizations are not required to use a benefit; adjustments are allowed.
Other	Yes	Organizations may use additional eligible population criteria to focus on a population of interest such as gender, race and ethnicity, socioeconomic, sociodemographic characteristic or geographic region.
CLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Event/diagnosis	NA	There is no event/diagnosis for this measure.
Denominator Exclusions	**Adjustments Allowed (Yes/No)**	**Notes**
Exclusions	No	Only specified exclusions may be applied. Value sets may not be changed.
Exclusions: Hospice, palliative care, I-SNP, LTI, frailty or advanced illness	Yes	These exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments.*
Numerator Criteria	**Adjustments Allowed (Yes/No)**	**Notes**
Mammogram	No	Value sets and logic may not be changed.

Colorectal Cancer Screening (COL-E)

SUMMARY OF CHANGES TO HEDIS MY 2023

- Refer to the Technical Release Notes file in the Digital Measures Package for a comprehensive list of changes.
- Revised the "Other" criteria in the Nonclinical Components table under *Rules for Allowable Adjustments of HEDIS.*
- Revised the "Exclusions" criteria in the Clinical Components table under *Rules for Allowable Adjustments of HEDIS*.

Description	The percentage of members 45–75 years of age who had appropriate screening for colorectal cancer.
Measurement period	January 1–December 31.
Clinical recommendation statement	The U.S. Preventive Services Task Force "recommends screening for colorectal cancer in all adults aged 50 to 75 years (A recommendation) and all adults aged 45 to 49 years (B recommendation)." Potential screening methods include an annual guaiac-based fecal occult blood test (gFOBT), annual fecal immunochemical test (FIT), multitargeted stool DNA with FIT test (sDNA FIT) every 3 years, colonoscopy every 10 years, CT colonography every 5 years, flexible sigmoidoscopy every 5 years or flexible sigmoidoscopy every 10 years, with FIT every year.
Citations	U.S. Preventive Services Task Force. 2021. "Screening for Colorectal Cancer: U.S. Preventive Services Task Force Recommendation Statement." JAMA 325(19):1965–1977. doi:10.1001/jama.2021.6238
Characteristics	
Scoring	Proportion.
Type	Process.
Stratification	• Colorectal Cancer Screening. – Product line: ▪ Commercial. ▪ Medicaid. ▪ Medicare. – Age (for each product line): ▪ 46–49 years. ▪ 50–75 years.

	– SES (for Medicare only): ▪ SES—Non-LIS/DE, Nondisability. ▪ SES—LIS/DE. ▪ SES—Disability. ▪ SES—LIS/DE and Disability. ▪ SES—Other. ▪ SES—Unknown. – Race (for each product line): ▪ Race—White. ▪ Race—Black or African American. ▪ Race—American Indian or Alaska Native. ▪ Race—Asian. ▪ Race—Native Hawaiian or Other Pacific Islander. ▪ Race—Some Other Race. ▪ Race—Two or More Races. ▪ Race—Asked but No Answer. ▪ Race—Unknown. – Ethnicity (for each product line): ▪ Ethnicity—Hispanic or Latino. ▪ Ethnicity—Not Hispanic or Latino. ▪ Ethnicity—Asked but No Answer. ▪ Ethnicity—Unknown.
Risk adjustment	None.
Improvement notation	A higher rate indicates better performance.
Guidance	• For Medicare plans, I-SNP and LTI exclusions are not included in the measure calculation logic and need to be programmed manually. Administrative data must be used for these exclusions. • Non-administrative data may be used for the frailty and advanced illness exclusion. **Allocation:** The member was enrolled with a medical benefit throughout the participation period. When identifying members in hospice, the requirements described in *General Guideline 15* for identification of hospice members using the monthly membership detail data files are not included in the measure calculation logic and need to be programmed manually. **Reporting:** For Medicare plans, the SES stratifications are mutually exclusive and the sum of all six stratifications is the total population. For all plans, the race and ethnicity stratifications are mutually exclusive and the sum of all categories in each stratification is the total population.

	SES and product line stratifications are not included in the measure calculation logic and need to be programmed manually. The race and ethnicity stratifications are reported by data source—direct or indirect.
Definitions	
Participation **Participation period**	The identifiers and descriptors for each organization's coverage used to define members' eligibility for measure reporting. Allocation for reporting is based on eligibility during the participation period. The measurement period and the year prior to the measurement period.
Initial population	Members 46–75 years as of the end of the measurement period who also meet the criteria for participation.
Exclusions	• Members in hospice or using hospice services any time during the measurement period. • Members with colorectal cancer or a total colectomy any time during the member's history through the end of the measurement period. • Medicare members 66 years of age and older by the end of the measurement period who meet either of the following: – Enrolled in an Institutional SNP (I-SNP) any time during the measurement period. – Living long-term in an institution any time during the measurement period, as identified by the LTI flag in the monthly membership detail data file. Use the run date of the file to determine if a member had an LTI flag during the measurement period. • Members 66 years of age and older by the end of the measurement period, with frailty and advanced illness. • Members receiving palliative care any time during the measurement period.
Denominator	The initial population, minus exclusions.
Numerator	Members with one or more screenings for colorectal cancer. Any of the following meet criteria: • Fecal occult blood test during the measurement period. • Flexible sigmoidoscopy during the measurement period or the 4 years prior to the measurement period. • Colonoscopy during the measurement period or the 9 years prior to the measurement period. • CT colonography during the measurement period or the 4 years prior to the measurement period. • Stool DNA (sDNA) with FIT test during the measurement period or the 2 years prior to the measurement period.

Data criteria (element level)
Value Sets: • **COLE_HEDIS_MY2023-2.0.0** – Colonoscopy (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1064) – Colorectal Cancer (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1065) – CT Colonography (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1421) – Flexible Sigmoidoscopy (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1102) – FOBT Lab Test (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1959) – FOBT Test Result or Finding (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1960) – History of Colonoscopy (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1910) – History of Flexible Sigmoidoscopy (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1911) – History of Total Colectomy (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1912) – sDNA FIT Lab Test (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1749) – sDNA FIT Test Result or Finding (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1750) – Total Colectomy (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1250) • **NCQA_AdvancedIllnessandFrailty-2.0.0** – Acute Inpatient (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1810) – Advanced Illness (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1465) – Dementia Medications (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1729) – ED (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1086) – Frailty Device (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1530) – Frailty Diagnosis (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1531) – Frailty Encounter (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1532) – Frailty Symptom (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1533) – Nonacute Inpatient (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1189) – Observation (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1191) – Online Assessments (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1446) – Outpatient (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1202) – Telephone Visits (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1246) • **NCQA_Claims-2.0.0** – Inpatient Stay (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1395) – Nonacute Inpatient Stay (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1398) • **NCQA_Hospice-2.0.0** – Hospice Encounter (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1761) – Hospice Intervention (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1762) • **NCQA_PalliativeCare-2.0.0** – Palliative Care Assessment (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.2225)

- Palliative Care Encounter (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.1450)
- Palliative Care Intervention (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.2224)

- **NCQA_Stratification-1.0.0**
  - American Indian or Alaska Native Detailed Race (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.2365)
  - Asian Detailed Race (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.2366)
  - Black or African American Detailed Race (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.2367)
  - Hispanic or Latino Detailed Ethnicity (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.2368)
  - Native Hawaiian or Other Pacific Islander Detailed Race (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.2369)
  - White Detailed Race (https://www.ncqa.org/fhir/valueset/2.16.840.1.113883.3.464.1004.2370)

Direct reference codes and codesystems:

- NCQA_PalliativeCare-2.0.0
  - codesystem "ICD-10-CM": 'http://hl7.org/fhir/sid/icd-10-cm'
  - code "Encounter for palliative care": 'Z51.5' from "ICD-10-CM" display 'Encounter for palliative care'
- NCQA_Terminology-2.0.0
  - codesystem "ActCode": 'http://terminology.hl7.org/CodeSystem/v3-ActCode'
  - codesystem "ClaimTypeCodes": 'http://terminology.hl7.org/CodeSystem/claim-type'
  - codesystem "ConditionClinicalStatusCodes": 'http://terminology.hl7.org/CodeSystem/condition-clinical'
  - codesystem "NullFlavor": 'http://terminology.hl7.org/CodeSystem/v3-NullFlavor'
  - codesystem "RaceAndEthnicityCDC": 'https://www.hl7.org/fhir/us/core/CodeSystem-cdcrec'
  - code "active": 'active' from "ConditionClinicalStatusCodes"
  - code "American Indian or Alaska Native": '1002-5' from "RaceAndEthnicityCDC" display 'American Indian or Alaska Native'
  - code "Asian": '2028-9' from "RaceAndEthnicityCDC" display 'Asian'
  - code "Asked but no answer": 'ASKU' from "NullFlavor" display 'Asked but no answer'
  - code "Black or African American": '2054-5' from "RaceAndEthnicityCDC" display 'Black or African American'
  - code "Hispanic or Latino": '2135-2' from "RaceAndEthnicityCDC" display 'Hispanic or Latino'
  - code "Institutional": 'institutional' from "ClaimTypeCodes"
  - code "managed care policy": 'MCPOL' from "ActCode"
  - code "Native Hawaiian or Other Pacific Islander": '2076-8' from "RaceAndEthnicityCDC" display 'Native Hawaiian or Other Pacific Islander'
  - code "Non Hispanic or Latino": '2186-5' from "RaceAndEthnicityCDC" display 'Non Hispanic or Latino'
  - code "Other": 'OTH' from "NullFlavor" display 'Other'
  - code "Professional": 'professional' from "ClaimTypeCodes"
  - code "retiree health program": 'RETIRE' from "ActCode"

– code "subsidized health program": 'SUBSIDIZ' from "ActCode"
– code "Unknown": 'UNK' from "NullFlavor" display 'Unknown'
– code "White": '2106-3' from "RaceAndEthnicityCDC" display 'White'

Data Elements for Reporting

Organizations that submit data to NCQA must provide the following data elements in a specified file.

Table COL-E-A-1/2: Metadata Elements for Colorectal Cancer Screening

Metric	Age	Data Element	Reporting Instructions
ColorectalCancerScreening	46-49	InitialPopulation	For each Stratification
	50-75	ExclusionsByEHR	For each Stratification
	Total	ExclusionsByCaseManagement	For each Stratification
		ExclusionsByHIERegistry	For each Stratification
		ExclusionsByAdmin	For each Stratification
		Exclusions	(Sum over SSoRs)
		Denominator	For each Stratification
		NumeratorByEHR	For each Stratification
		NumeratorByCaseManagement	For each Stratification
		NumeratorByHIERegistry	For each Stratification
		NumeratorByAdmin	For each Stratification
		Numerator	(Sum over SSoRs)
		Rate	(Percent)

Table COL-E-A-3: Data Elements for Colorectal Cancer Screening

Metric	Age	SES Stratification	Data Element	Reporting Instructions
ColorectalCancerScreening	46-49	NonLisDeNondisability	InitialPopulation	For each Stratification
	50-75	LisDe	ExclusionsByEHR	For each Stratification
	Total	Disability	ExclusionsByCaseManagement	For each Stratification
		LisDeAndDisability	ExclusionsByHIERegistry	For each Stratification
		Other	ExclusionsByAdmin	For each Stratification
		Unknown	Exclusions	(Sum over SSoRs)
		Total	Denominator	For each Stratification
			NumeratorByEHR	For each Stratification
			NumeratorByCaseManagement	For each Stratification
			NumeratorByHIERegistry	For each Stratification
			NumeratorByAdmin	For each Stratification
			Numerator	(Sum over SSoRs)
			Rate	(Percent)

Table COL-E-B 1/2/3: Data Elements for Colorectal Cancer Screening: Stratifications by Race

Metric
ColorectalCancerScreening

Race	Source	Data Element	Reporting Instructions
White	Direct	InitialPopulation	For each Stratification
BlackOrAfricanAmerican	Indirect	Exclusions	For each Stratification
AmericanIndianOrAlaskaNative	Total	Denominator	For each Stratification
Asian		Numerator	For each Stratification
NativeHawaiianOrOtherPacificIslander		Rate	(Percent)
SomeOtherRace			
TwoOrMoreRaces			
AskedButNoAnswer*			
Unknown**			

Table COL-E-C-1/2/3: Data Elements for Colorectal Cancer Screening: Stratifications by Ethnicity

Metric
ColorectalCancerScreening

Ethnicity	Source	Data Element	Reporting Instructions
HispanicOrLatino	Direct	InitialPopulation	For each Stratification
NotHispanicOrLatino	Indirect	Exclusions	For each Stratification
AskedButNoAnswer*	Total	Denominator	For each Stratification
Unknown**		Numerator	For each Stratification
		Rate	(Percent)

*AskedButNoAnswer is only reported for Source='Direct.'
**Unknown is only reported for Source=’Indirect.’

Rules for Allowable Adjustments of HEDIS

The "Rules for Allowable Adjustments of HEDIS" (the "Rules") describe how NCQA's HEDIS measure specifications can be adjusted for other populations, if applicable. The Rules, reviewed and approved by NCQA measure experts, provide for expanded use of HEDIS measures without changing their clinical intent.

Adjusted HEDIS measures *may not* be used for HEDIS health plan reporting.

Rules for Allowable Adjustments of Colorectal Cancer Screening—ECDS

NONCLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Product lines	Yes	Organizations are not required to use product line criteria; product lines may be combined and all (or no) product line criteria may be used.
Ages	Yes, with limits	The age determination dates may be changed (e.g., select, "age as of June 30"). The denominator age may not be expanded.
Allocation	Yes	Organizations are not required to use enrollment criteria; adjustments are allowed.
Benefit	Yes	Organizations are not required to use a benefit; adjustments are allowed.
Other	Yes	Organizations may use additional eligible population criteria to focus on a population of interest such as gender, race and ethnicity, socioeconomic, sociodemographic characteristic or geographic region.
CLINICAL COMPONENTS		
Eligible Population	**Adjustments Allowed (Yes/No)**	**Notes**
Event/diagnosis	NA	There is no event/diagnosis for this measure.
Denominator Exclusions	**Adjustments Allowed (Yes/No)**	**Notes**
Exclusions	No	Only the specified exclusions may be applied. Value sets may not be changed.
Exclusions: Hospice, palliative care, I-SNP, LTI, frailty or advanced illness	Yes	These exclusions are not required. Refer to *Exclusions* in the *Guidelines for the Rules for Allowable Adjustments*.
Numerator Criteria	**Adjustments Allowed (Yes/No)**	**Notes**
Colorectal Cancer Screening	No	The value sets, direct reference codes and logic may not be changed.